



University Narratives

HIV CURRICULUM INTEGRATION

A unique strength in the national HIV response

TABLE OF CONTENTS

Foreword	iii
Introduction	1
Cape Peninsula University of Technology	5
Central University of Technology	10
Durban University of Technology	17
University of Cape Town	24
University of Fort Hare	30
University of Johannesburg	38
North-West University	44
University of South Africa	51
University of Venda	59
University of Limpopo	66
University of the Western Cape	70
Walter Sisulu University	76
University of the Witwatersrand	81
Mangosuthu University of Technology	88
Nelson Mandela University	93
Rhodes University	98
Sefako Makgatho Health Sciences University	103
Tshwane University of Technology	108
University of Mpumalanga	115
University of Pretoria	121
Mapping the way forward	130
Notes	131

FOREWORD

I am delighted to share with you these exciting narratives of our HEAIDS HIV Curriculum Integration and Development Programme that sought to explore how the teaching and learning mandate of higher education could effectively contribute to the country's strategy on turning the tide on the HIV and AIDS epidemic and its impact on our nation and its young people in particular.

When HEAIDS received the grant from the National Skills Fund (NSF) through the Department of Higher Education and Training (DHET), it was clear that academic training and capacity development would be the focus of the grant. The project and its delivery has however far exceeded this expectation. It has certainly met this objective, but what it has also done, is that, it has explored knowledge generation and the creation of a new generation of leaders which is essentially what the role of higher education is in our society.

The project has succeeded in mainstreaming its objectives to the current debate in higher education that speaks to academic development and most significantly curriculum transformation.

It has explored the notions of 'graduateness' and 'citizenship' and it has shown how through HIV and AIDS curriculum integration, fundamental issues of human rights and social justice can be addressed.

It has shown that education brings with it privilege and access and resources and that educated young people can be developed both personally and professionally to become change agents in their families, communities and work spaces.

Through much of the work and the conversations that academics have engaged in on the project, it has become evident that a new generation of academics who are student-centred and self-reflective are key to unlocking this potential of the young people that traverse the higher education space.

This publication is a collection of narrative reflections of the 20 universities that participated in the programme. The narratives were developed by



capturing the voices of the project teams at each of the universities and thus tells their story.

I would like to say a special thank you to Ms Managa Pillay, my Project lead at HEAIDS, who worked long and hard over the past 3 years to make this vision a reality. Together with several technically competent consultants, she has succeeded in laying a good foundation in exploring how the teaching and learning spaces in both the university and TVET Colleges may be used to address societal and social justice issues.

I am excited at the outputs and the contributions that our universities and colleges have made and want to thank each and every academic and project team member for their participation and commitment. I particularly want to thank the Deputy Vice Chancellors (Academic, Research, Teaching and Learning) at the participating institutions for their leadership and oversight at an institutional level, which emerged as a significant contribution to the success of the projects.

Lastly, my thanks to the NSF and the DHET for having the courage and foresight to invest in this innovative and challenging programme and thus contributing to curriculum transformation."

I look forward to the next phase of this work.

DR RAMNEEK AHLUWALIA
CEO: HEAIDS

INTRODUCTION

“Only the light of knowledge about AIDS can rid us from the darkness of fear, misconception and pain.”
– Anonymous

The role of HEAIDS

The Higher Education and Training HIV/AIDS Programme (HEAIDS) is an initiative of the Department of Higher Education and Training (DHET), undertaken by Universities South Africa (USAF), the representative body of South African public universities, and the South African College Principals' Organisation (SACPO). HEAIDS provides assistance to public universities and technical and vocational education and training (TVET) colleges across South Africa in their response to the HIV and TB epidemics. Its programmatic work is closely aligned to successive versions of the five-year South African *National Strategic Plan on HIV, TB and STIs*, which specifies national targets for HIV and TB testing, care and treatment, and which adopts a rights-based approach to the two epidemics. The national approach is aligned with global strategies and best practice advocated by the Joint United Nations Program on HIV/AIDS (UNAIDS) and the World Health Organization (WHO).

HEAIDS has three focus areas in its engagement with the higher education sector, which are as follows:

- Teaching and learning
- Research, innovation and knowledge sharing, and
- Engagement with communities.

Research findings

Phase 2 of the Programme enabled HEAIDS to fund substantial research endeavors in the higher education sector, to ascertain the primary needs in tertiary level teaching and learning spaces regarding meaningful engagement with HIV and AIDS. The research findings were that many lecturers acknowledged that they had a social responsibility to address HIV in some

form or another through their teaching. Academics wanted to integrate learning about HIV meaningfully into curricula, though many believed that they did not have the skills or the specialist knowledge which they believed was necessary to be able to engage in HIV curriculum integration confidently. There was hence a need for further development of curricular resources, for capacitation of teaching staff, and provision of other forms of support for academics in their engagement with HIV.

The research also revealed a general consensus that graduates of higher education were expected to develop a range of professional and personal competencies in relation to HIV and AIDS, particularly in the workplace, by the time that they completed their studies. Furthermore, increasingly higher education is expected to play a leading role in addressing and finding solutions to South Africa's major societal challenges, which include HIV and AIDS.

In accordance with these research findings, HEAIDS chose as its objective for Phase 3, strengthening the capacity of higher education institutions (HEIs) to integrate HIV/AIDS into the curriculum. Capacity development hence emerged as the main thrust of the Phase 3 interventions, making possible an application to the National Skill Development Fund (NSF) for funding of this work.

Invitation for HCI proposals

In 2014, after funding had been secured, HEAIDS sent out a Call to all public HEIs to submit funding proposals for integration of HIV and AIDS into the curriculum. The proposal was to be submitted via the Deputy Vice Chancellor (DVC) (Teaching and Learning) or the DVC (Academic) of each institution¹.

¹ The reason for this was that by working through the DVCs HEAIDS hoped to encourage broader institutional engagement in developing the HCI project proposal. Ideally then HCI projects were not to be located in a single faculty or department, but should enjoy broader academic support and involvement.

A total of 25 HEIs applied for funding, with proposals in some cases building on what had already been achieved in terms of curriculum integration. All proposals which were submitted underwent a thorough formative review by a technical team at HEAIDS. Detailed feedback was provided to the HEIs regarding how proposals could be improved to align to the intention of the grant. It was important for HEIs to firstly understand that this scope of work was directed at the academic staff and not the student and secondly that the outcome was not about HIV testing and condom use and prevention, but rather about using the teaching and learning space to address HIV and more importantly its intersectionality's and drivers such as human rights and social justice amongst others. After up to three rounds of submission, review and proposal amendment, 18 HEIs received approval of their HCI project proposals and were granted funding for implementation. Later two additional HEIs which had not existed at the time of the initial submission date, viz. Mpumalanga University of Technology and Sefako Makgatho University, submitted proposals and were also allocated funding for HIV curriculum integration (HCI) projects, albeit over a shorter time-frame. Hence a total of 20 HEIs contributed to this body of work.

The HCI programme unfolds

The first nine months of HCI engagement focused on an extensive process of lobbying and advocacy at HEIs regarding curriculum integration, as well as exploring with academic staff their understanding of HCI. HEAIDS acknowledged the importance of recognising that some institutions had already engaged in extensive and meaningful HCI, and that this existing body of knowledge needed to be shared. A key guiding principle in the HCI approach was that the academics themselves understood their institutional spaces best, and hence should determine the shape and extent of their HCI projects. They were thus encouraged to take the lead in developing their HEI proposals and implementing them as well.

Once the grants had been awarded, all the institutions which received funding were visited by the HEAIDS project team, to obtain an understanding of what they needed in terms of capacitation. Given the small HEAIDS team that was involved in the HCI initiative, strategic decisions were made to identify

and draw on external expertise which would enhance the HCI processes. Hence, at different stages of the programme's time-frame skilled individuals were drawn into the process, to augment and enhance the projects. Consultant input was particularly important for capacity development (such as development of appropriate curricula and training of HEI lecturers), establishment of effective monitoring and evaluation (M&E) processes, development and piloting of various capacity development workshops that were offered to all academic and project staff working on implementation of the project and finally, for narrative documentation of the individual institutional projects.

Among contributions made by various consultants to the HCI programme, of particular importance was the part played by Dr Paulette Powell, a highly respected educationalist who brought to HCI extensive experience of engagement with, and strengthening of tertiary educational institutions, not only in South Africa but also through a range of other African countries. Dr Powell's main focus within the HEAIDS programme was on technical support and capacitation of the participating HEIs. She achieved this by adopting various distinctive roles: facilitating workshops and providing training and capacity development, offering practical assistance and sharing her expertise in curriculum development, suggesting viable responses when projects faced unexpected challenges, and providing objective feedback on project performance. Her meticulous and context-informed contributions to the HCI programme were widely appreciated by participating institutions.

Further significant support in developing content and facilitating workshops for academics across the participating HEIs included:

- Ms Sianne Abrahams, Ms Lucina Reddy and Ms Cal Volks (UCT) – HIV 102 and Institutional Conversations for HIV curriculum integration.
- Dr Charlene Carbonatto (UP) – Models for HIV curriculum integration
- Professor Lesley Wood (NWU) – Participatory Pedagogy
- Mr Pierre Brouard (UP) – Critical Diversity Literacy
- Ms Pillay and Dr Powell, supported the delivery of these workshops.

Considerable interest was expressed at various institutions in the development of a compulsory HIV 101

module at entry level for all new tertiary level students. Such a course could provide basic up-dated HIV information for academics, who lamented that besides HIV not being their area of expertise, that the information in the field was changing daily and that keeping abreast of this was often a challenge. Individual project reports document progress in development of such module.

Another capacity development focus area was to provide training to academics from participating institutions in participatory pedagogies, which could engage students in thinking critically about HIV, whilst acknowledging the reality of HIV and AIDS fatigue amongst young people. Workshops engaged academics with methodologies focusing on notions of self-reflection, including 'curriculum in the making' and 'humanising pedagogy'. The workshops were highly participatory, encouraging self-reflection and exposing academic staff to innovative teaching methods, including use of drawings and photographs in their work, practising free-writing and development of case-studies. Through such capacity development, academics were challenged not only to address HIV-related issues with students but also to engage them on broader issues that could contribute to their personal well-being, their professional competence and their taking ownership of broader societal responsibilities.

Questions also arose regarding what models of curriculum integration were most appropriate for the HEIs. In response, academics were given an overview of the range of approaches taken for integration (including their relative advantages and shortcomings), so as to better inform their choices.

Regular institutional visits contributed to the success of the programme. They enabled the HEAIDS team to engage with institutional academic leaders (including DVCs, deans and heads of departments), to work directly with academics involved in the institutional projects, to listen and respond to the constraints and challenges facing project teams, and to encourage teams to maintain their project momentum needed to achieve their proposed outcomes. Budgetary issues were also addressed at these meetings.

Major challenges

The main challenge for the HCI programme was the widespread student protests in 2015 and 2016

across the institutions. The protests and disruption began around student funding issues, popularly known as #FeesMustFall. Later protests such as #RhodesMustFall and #AfrikaansMustFall arose in relation to issues such as decolonisation, and the need for curriculum transformation. It was a challenge to maintain the momentum of the programme during these disruptive periods. At some institutions HCI projects were negatively impacted by protest action, but most were able to continue or to adapt to the more volatile institutional context.

As the individual institutional reports that follow will reveal, some institutions had particular internal challenges that impacted negatively on the success of their projects. Common challenges included limited or no project management skills available at some institutions, difficulty in identifying existing HCI champions, changes in senior management during the funding cycle, poor communication, difficult institutional structures, difficulty in meeting the financial reporting requirements of HEAIDS and loss of key project leadership during the programme's life-span.

Highlights of HCI engagement

A number of highlights of the HCI programme are particularly worth noting. One example is that the sheer variety of HEI projects and the diversity of their approaches was both challenging and gratifying as they brought with them a dynamism and energy which ultimately contributed to the success of the programme.

An important benefit was the effectiveness of project interventions in bringing academics together for participative and informative engagement. A few occasions involved representatives of all of the participating institutions gathered in national workshops to share their experiences, insights and lessons learned. Other engagements involved a few institutions engaging in dialogue, sharing good practice and developing capacity. Such engagement took place within a context of collaborative good faith, characterised by a deepening of trust between institutions and with the HEAIDS co-ordinating team. Over time, such structured interactions effectively evolved into a powerful community of practice, with different projects explicitly recognising its deep value.

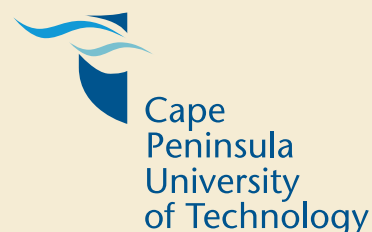
The capacity development workshops highlighted the value of HIV and AIDS integration as a powerful curriculum transformation entry point which could lead to engagement with issues such as good citizenship and social justice, racism, gender roles and sexism, sexuality, and institutional power and oppression. The critical diversity literacy (CDL) workshops, which developed in response to expressed academic need, were particularly powerful, engaging academic staff in an analysis of unequal power relations, leading to an understanding of how inequity is produced by cultural practices. Academic staff were equipped with tools to challenge such inequity. Notions of masculinity and femininity were also critically examined in these workshops, raising issues about harmful stereotypes in the media and in everyday life that perpetuate hierarchies of racial and/or gender ‘superiority’ or privilege. Feedback regarding these workshops was very positive, with participants acknowledging the need for such personal engagement and the importance of confronting uncomfortable issues head-on. Through serendipity the CDL workshops tackled a number of the difficult issues raised in the widespread student protests, including decolonization, critical revision of the current curriculum, the challenges to ‘white privilege’ and to ‘male privilege’, and the importance of tackling the relation of power and privilege. In this way, the CDL workshops provided a vehicle for equipping participating academics with crucial skills which are critical to their teaching effectively within such a volatile tertiary education sector.

Documentation of HCI projects at HEIs

Individual narrative reports which document the activities and experiences of each of the participating HEIs follow this *Introduction*. The individual reports are based on visits to institutions (between December 2016 and June 2017) by Rob Hamilton, a consultant to HEAIDS appointed to undertake this work. Visits included scheduled focus group discussions with HCI team members about their projects². With permission, he audio-recorded all focus group discussions held with HCI team members and then transcribed the recordings. Thereafter transcriptions were closely examined for common themes, institution-specific and unique themes, as well as unanticipated outcomes, and each narrative report drew on key emergent concepts. In the case of a few institutions, it was not possible for him to meet with the HCI team owing to research time constraints or because of competing academic demands at these institutions. In such cases selected individual academics who had participated in the HCI project were interviewed instead, and the narrative report was then based on a summation of their perceptions and views. The contents of each narrative report were checked for veracity against M&E documentation and programmatic reports submitted to HEAIDS by the project concerned. HEIs (project manager and the DVC academic) were also provided with the final draft of the narratives to ensure that the narrative was in fact a true reflection of their project.

2 Rob Hamilton is a clinical psychologist in private practice who specialises in counselling people living with HIV (PLIV) and providing an alternative approach in addressing addiction. He is also an experienced researcher / writer / editor, with more than two decades of experience engaging with HIV/AIDS, gender inequality, sexual health and related social justice issues.

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**“HIV is a disease for humans – it doesn’t choose who to affect, it is all genders, all races.”
– Cape Peninsula University of Technology, Student comment**

Introduction – the institutional context

The Cape Peninsula University of Technology (CPUT) was established in 2005, with the merging of the Cape Technikon and Peninsula Technikon, part of a national transformation process in higher education across South Africa. CPUT is the only university of technology in the Western Cape, and the university in the province with the highest enrolment¹. In 2015 it had a total student enrolment of 32 674.²

The Cape Technikon had its roots in the Cape Technical College, established in 1920 to provide technical education. The Peninsula Technikon developed out of the Peninsula Technical College, which was established in 1962 to provide technical training for people classified as coloured. In 2001 the Boland and Mowbray Education Colleges were incorporated into the Cape Technikon, forming the Faculty of Education at sites in Wellington and Mowbray.

The CPUT’s current vision is ‘to be at the heart of technology education and innovation in Africa’³.

Initiation of the HCI programme

The HIV curriculum integration programme (HCI) at CPUT had the following objectives:



- Develop a participatory pedagogy
- Encourage self-reflection of lecturers in relation to HIV/AIDS
- Ensure alignment with related policy processes and procedures, and promote buy-in from staff
- Strengthen knowledge of HIV/AIDS and related issues
- Facilitate the development and provision of HIV/AIDS learning and teaching materials, and
- Explore and facilitate linkages between the university and the TVET educational subsector.

In terms of HIV/AIDS educational materials development, the CPUT team decided to revise an existing TAFEISA book, entitled *How 2B AIDS Aware*, which had been developed in 2003/2004. The first version of the book had been received very well across the

¹ <https://www.cput.ac.za>

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

³ <https://www.cput.ac.za>

country. It had been used by 15 higher education institutions (including UNISA). The University of the Western Cape had utilised it in its Education programme and it had even obtained international acclaim, being taken up by the Association of African Universities. However, given that the book was now more than ten years old, it was unanimously decided by the CPUT staff involved in the project, that it needed revision in order to update the book and to bring it in line with new developments within the HIV/AIDS field. However, this book was still regarded as a really good basic resource to use because it has been tried and tested. We are very proud of what was produced.

Two areas of study in which the HCI team said particularly good progress was made with regard to HCI were Information Technology Management (IMT) and Office Management Technology (OMT). The IMT lecturers relied on the revised edition of *How 2B AIDS Aware* as their main resource in engaging students to think critically about HIV.

A different approach was taken by OMT, which embraced the skills required for working in office administration or practice management, and being an administrative assistant or a personal assistant in a private sector setting. OMT students engaged with HIV within a course entitled Business Administration, which situated HIV as one of a range of thorny issues in the economic and social environment which businesses had to learn how to manage. The HIV content included a community engagement component, which meant that students had to grapple in a real-life setting with HIV issues.

Involving staff members

According to the HCI team, obtaining buy-in from academic staff was not problematic. Staff members at CPUT were easily convinced that HIV was an important issue that merited attention. However, what was more challenging was arranging workshops and meetings to accommodate everyone who wished to attend:

Everybody here is basically very busy, staff members have many commitments. So it was very challenging to try and find a day and a time when

everybody was available for a meeting ... Not everybody can be fitted in, in the end.

Nevertheless, in June 2015 the CPUT team arranged a workshop with staff members to discuss HIV curriculum integration. The workshop was well received:

Five out of our six faculties sent representatives to the workshop. People spoke frankly and openly at the workshop. Most of the staff don't have any problems with integration ... They just need help with how exactly to do it. We also found out what were their actual needs for training. Academic staff are mostly open to the idea of integration. We were very pleased to see such a positive response.

Some clear messages emerged from discussions at the workshop:

The staff were very comfortable about using an infusion model – they wanted to find their own ways of integrating HIV into their courses, instead of having HIV as a stand-alone option.

Another theme that emerged had to do with how HIV was to be presented:

We said that it is important to use the technologies that students are comfortable and familiar with. Students don't want to read again about HIV in just another book. Then there is definitely a need to include the experiences of people living with HIV in what we teach, to help reduce the stigma ... and because students make bad decisions under the influence of alcohol and drugs, we have to talk about those issues as well.

Challenges

One challenge was talking frankly about the sexual transmission of HIV. Once they had received appropriate training, staff members were generally comfortable with having such discussions. However, the HCI team said, members of the community were not always accepting of such matters being talked out at institutions of higher education:

You do find with some people in the community, especially those with a stronger religious background and older people, they feel uncomfortable

about you talking to their children or grandchildren about HIV and sex. But you have to do that ... and you need to go out into the community sometimes and talk about why it is necessary. The truth is that, without knowing all the facts the younger generation won't be protected against HIV.

One team member spoke of educating community members and engaging with community groups as being for her a real passion. She said:

My experience with the community has really helped me with my work here, to understand what the real issues with HIV are out there.

A challenge in the HCI project at CPUT related to working with consultants. An external consultant was commissioned to develop an additional booklet for students on HIV and wellness. However, the materials development process turned out to be an unfortunate experience:

The quality of the work that the consultant produced for us was not up to standard and in line of what was discussed with him prior to commencement. The team was unhappy with the deliverables and we had to address the issues. Some sections needed to be rewritten, before it was accepted. At completion, the consultant did not want to relinquish copy right to CPUT. After seeking legal advice, the challenge was resolved, the booklet was printed and utilised in OMT department. The consultant was paid and it was decided that CPUT would not utilise this consultant's services in future.

The team admitted, however, that this had been an important learning experience for them:

We learned about the legal aspects, copy right policy and about getting a contract in place before any consultant starts working.

This was a valuable lesson learnt for the entire team, going forward.

Importance of leadership

What emerged clearly from the focus group discussion was that there was widespread support on campus for the HCI process at CPUT. The HCI team

was especially appreciative of the ongoing support for the project provided by the Deputy Vice Chancellor (Academic), Prof. Anthony Staak.

Most academic staff at CPUT showed enthusiasm about HCI. However, it was ultimately a small group of dedicated staff members who were responsible for driving the HCI process. One member of the HCI team commented:

HIV is something that you either have a passion for or you don't. What keeps me involved and committed to this process is I have seen the impact of HIV – how it has devastated families and communities ... and seeing lives being lost because of HIV, I wanted to make a difference. I think it also speaks to wanting to take the lead, not waiting for someone else to do it for you.

Other team members agreed about the need to show leadership:

It is about getting involved ... because if you don't do it, who is going to do it? ... Do we want the next generation to be properly informed about HIV, so they can make the right decisions, or don't we?

Addressing AIDS fatigue

The team was aware of the reality of AIDS fatigue. Some students felt that they had 'heard it all before', having had basic information about HIV given to them in so many levels at school. However,

... what we find when we talk to our students is that they are still interested – there are many questions that they don't have the answers to about HIV. There is a lot that is not covered properly in the schools. The students are not well equipped, and it is especially a problem for the first years, they are vulnerable, so it needs to be in the first-year curriculum.

What was important in engaging students' attention was the mode of delivery used in teaching:

We have to think about the forms of communication that students nowadays prefer. They like You-Tube, Facebook, twitter, Instagram, and they pick up lots of their information from the internet. So we have to use more of those methods of delivery to hold their

attention when we teach about HIV. In that way we can reach them more effectively.

Students who had been through HIV workshops gave feedback afterwards about the experience, and it had been very encouraging for the team. The HCI team mentioned some of the students comments that they had been struck by:

I feel that I can talk more about HIV and can inform others ...

I am left motivated because now I will be careful and play safe.

I can personally left feeling more educated and more knowledgeable on the topic of the HIV and AIDS pandemic in South Africa, and the effect it has on the lives of people globally.

HCI and broader educational transformation

A critical diversity literacy workshop for CPUT staff was held in March 2016. The team thought that those who attended it had found it very valuable:

So it created a safe space ... the lecturers could talk openly about lots of issues that they were uncomfortable about ... not just HIV, also things like gender, LGBTI, race, discrimination and how to address these issues in the classroom.

The workshop showed that:

Transformation is not just a matter for students to think about – it is a burning issue for staff members too. We all have to be aware of the space we create in a learning environment, which can either include people who are different in some way, or exclude them. And we need to create a learning environment where the students can start to look at their own perspectives on the world – are they either including others or excluding them?

In fact, one CPUT team member concluded:

So we can really say that dealing with diversity is a whole learning process for both lecturers and for students alike.

Further material development

Now that two books were developed, the team found that there were certain key elements missing. Those elements included issues of social justice, gender diversity, good citizenship, general men and women's health and softer skills like how to prepare a good cv and preparing for an interview. The team embarked on a writing retreat to cover these important areas. The focus or theme was a roadmap and what students need to equip them for the workplace and how to become a good citizen. Another focus area was to make the book as interactive as possible to create an interest in research. The book titled: *Student Guide. Roadmap to your social, mental and physical well-being* was launched on the same day as the other two books and were very well received. The book will soon be loaded on Blackboard and on the institutional library's site. We were also invited to introduce the book to Life Orientation lecturers at College of Cape Town.

Feedback from Prof Engel-Hills from the Faculty Health & Sciences:

"I had the good fortune to meet Melanie Marais on Thursday last week and she gave me a hard copy of the newly published, Student Guide: Roadmap to your Social, Mental and Physical Wellbeing. I am very proud that 2 academic staff members from our faculty made such a substantial contribution to this excellent publication. It is long overdue and will be of immense value to our students as they face the challenges of higher education and adulthood. I started reading over the week-end and read from cover-to-cover – I know it will be an important read for everyone at CPUT. Having this as an e-resource is excellent and I trust the academic staff will find

"We all have to be aware of the space we create in a learning environment, which can either include people who are different in some way, or exclude them."

– Cape Peninsula University of Technology FGD

ways to integrate the information into our teaching and learning platform so that it is a living document. Please as the Nursing representative to take this to the faculty Teaching and Learning Committee for discussion as the message needs to go out far and wide that we have this excellent resource available for students (and staff). Thank-you sincerely for this contribution that I can see took much time, effort and commitment"-Academic

"I thought the book launch was about books. How wrong was I! It was about a team of dedicated individuals, students and staff alike, committed to improving our humanitarian response to HIV risk among young people: actions by which we will be judged in the future"- Academic

compassionate towards people who are living with HIV ... There is still a lot to do before we can say that HIV is no longer a problem in this country.

The team was optimistic that as a result of the HCI project more staff members were now involved in HCI in one form or another. This process was likely to continue even after the project came to an end. Nevertheless, one element was essential, one team member believed:

If we want this process of HIV integration to continue and to grow, I would say that one thing you really have to have is leadership ... we are lucky we have had wonderful leadership up to now, and that can definitely help the momentum to continue.

Where to next?

The HCI team members were unanimous in their agreement that HCI should continue and expand at CPUT.

However, an issue of concern for the CPUT team was sustainability. Team members felt that the groundwork for HCI had been laid some years before the current project, and the HEAIDS funding had allowed the process to be taken to another level of development. Nevertheless, sustainability was crucial if HCI was to have a real and ongoing impact on students and, ultimately, on communities:

We have a great deal of work around HIV that we are very proud of. But it is so important to carry on doing the work after this project comes to an end ... helping the students to think about the risks they take, and getting students to be more

Conclusion

The steady and continuous progress made with HIV curriculum integration at CPUT represents a remarkable living archive of knowledge and human experience. Other institutions of higher learning could well tap into and learn from the CPUT narrative. Three elements stand out clearly in their story. The first is that CPUT had previously invested energy and resources in aspects of HIV curriculum integration, and in the current HCI project the team was wise to build on what had already been achieved. Secondly, the full commitment of the institution's top leadership to the process seems to have been an important enabling factor. Finally, none of this engagement would have yielded results if it had not been for the enthusiasm, passion and undoubted commitment of the HCI team, who are to be congratulated on their accomplishment. May the momentum at CPUT continue!

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“Hearing from my colleagues, where they had come from ... and how their journey transformed them. Also what their family values were and how their humanity was very similar to what I experienced ... though we appear different, we are still human, we have a lot in common.”
– Central University of Technology FGD

Introduction – the institutional context

Central University of Technology (CUT) is a higher education institution based in the heartland of South Africa, in Bloemfontein, which provides education and training in science, engineering and technology. The institution opened its doors in 1981 as the Technikon Free State, with 285 students enrolled in mainly secretarial, art and design programmes. In 2004, with the restructuring of higher education, it became a university of technology, hence its current name.¹



CUT offers a range of qualifications in its four faculties, which are Engineering and Information Technology, Health and Environmental Sciences, Management Sciences, and Humanities. It has three campuses. The main campus is in Bloemfontein, with smaller satellite campuses in Welkom and Kimberley respectively. In 2015 CUT had a total student enrolment of 14 193.²

Initiation of HCI

The project leader of the HCI initiative at CUT described how the process began:

What happened ... is that there was a call for submissions of projects, around 2014. However, the HEAIDS call for proposals did not arrive directly on my desk. There were a number of delays. But somehow I was able to follow up with the application, and it was eventually submitted.

Thereafter:

Come January 2015, we officially launched the project, at an off-campus venue, Tuscan Rose. In

¹ <https://www.cut.ac.za>

² Department of Higher Education and Training. 2017. *Statistics on Post-School Education and Training in South Africa: 2015*.

order to try and raise awareness and to mobilise, we invited almost everybody, from the faculties, the deans, and the heads of departments to the academic staff. Not all of them could be there, but the bulk attended. I think it was a success.

A broad-based invitation to participate in HCI was sent out:

We did not want to limit it to any particular faculty. So we made it open to whoever was interested. Right at the beginning we had several people who showed an interest, including Human Resources and the Accounting Department, which came up with a brilliant proposal. Some of those individuals just fell by the wayside. It was like a relay race. Welkom campus joined slightly later, but it was then consistently committed to be involved in the process.

A colleague continued the story:

The bulk of the initial phase of the project was capacity building, where workshops were run by either internal or external facilitators. We got someone to run a workshop much like HIV/AIDS101, since we were aware that CUT academics were not necessarily very knowledgeable about HIV.

At a second workshop:

... Staff members from the Faculty of Management Sciences spoke about HIV and the law in the workplace. It was also a success and raised awareness. People came up with so many brilliant ideas.

An important aspect of early engagement with HCI was obtaining appropriate HIV/AIDS educational materials, and making sure that staff who were interested in HCI could access them easily.

Thereafter, the slow process of incorporating HIV into the curriculum began to take shape:

So we carried on, learning how HIV integration could be done and sharing the process. In the case of the Department of Design and Studio Art, the decision was taken to include HIV/AIDS from first year upwards, with the first-year component entitled *The self, being empowered*. At second-year level it became *Me and my community*, and at third year, it was *Me and the workplace*.

Despite the HCI team losing some of its initial members, a number of departments remained clearly committed to the process:

And then we remained with Teacher Education, the Department of Design and Studio Art and someone from Higher Certificate: Community Development Work, a one-year programme with a strong community focus.

A first-year core module was chosen for HCI because so many courses had it as a prerequisite:

We took a module from the core curriculum, Academic Literacy and Communication Studies, specifically from the Community Development Practice programme, to incorporate HIV into. It's about integration of content and language.

Further progress with integration took place on the Welkom campus of CUT. A Welkom lecturer commented:

I fall under the Education Department, and in my Department we have various subjects. So we are integrating HIV and AIDS throughout the Education Department. It seemed relevant to me to integrate HIV into education, because when you educate a teacher, you potentially empower thousands of learners when they go out and teach. Through those learners, you empower communities. This strategy, working through education, seemed to us to be a good strategy because if you educate teachers, you can make a difference. This project is relevant too because it connects to the vision and mission of CUT, which speaks to diversity.

He said that although lecturers expressed reluctance about taking on HCI:

When we looked at different departments, the lecturers were already including HIV, but they were not aware of doing so, or they were not stressing its importance.

He provided details of how integration was taking place:

If you look at Educational Law, they were talking about rights, the rights of the learners. In Business Studies, they were incorporating it through talking

“This work is all about passion for HIV/AIDS. In order to stick it out until the end, and see results, you have to have real passion.”

– Central University of Technology FGD

about social responsibility, and business ploughing back into the community, sick leave, losing people to HIV. In terms of assessment, they were including it through providing a scenario which involved HIV, and asking students questions about the scenario. For example, advertising a post in a company, and having somebody who was HIV positive apply, and then be rejected on the grounds of their HIV status.

Although lecturers might be reluctant to engage in HIV integration, he said that students on the Welkom campus were enthusiastic about inclusion of HIV in the curriculum.

Challenges experienced

Right at the start, there were bureaucratic and administrative challenges at CUT which delayed the initiation of the HCI process. The HCI project leader recalled:

We received the HEAIDS call for proposals very late. The document moved about on campus, until eventually it landed on my table because there was reference to curriculum issues. So I assembled a task team, including curriculum developers. After brainstorming, we put together a proposal, and I assigned one of the curriculum developers to develop it further.

Thereafter it was submitted. However,

it got delayed further, because of other administrative challenges we had along the way ... That was the next hurdle, which showed me that we shouldn't expect easy sailing with this project.

The initial delays meant that the project time-frame shrunk.

Another challenge was the early loss of HCI team members. The HCI project at CUT began with a relatively sizeable and representative HCI team. However, soon participating lecturers began to drop out:

It became tough because we want everybody to meet on a regular basis, but not everybody

was available. We had, for example, somebody from Human Resources, who left in the middle of last year. By virtue of its content, HIV/AIDS had a natural fit within Human Resources, so this was disappointing for us. Then the representative from the Accounting Department also withdrew.

It appeared that CUT staff members were distracted by competing demands, and simply stopped attending meetings.

A Welkom lecturer spoke about the reluctance of staff members on his campus:

The big challenge for staff was that they were complaining about extra work if they had to include HIV, or saying that the existing curriculum was too full already to also add HIV. Or they would say that they were not well trained to include HIV effectively. This was especially a problem with the Accounting Department – they deal only with money, and they like to count.

Learnings

The HCI team felt that an important learning from the HCI project was taking some care in selecting team members:

The establishment of the task team is really important ... The task team should only constitute academics who will be actually doing the work of incorporation of HIV into the curriculum, and are really committed to the process.

The bureaucratic and administrative delays that the project experienced taught them another important lesson:

It is essential to have the support staff at the university understand what we are doing – and give us the right support when we need it. This can take many forms.

One team member argued:

This work is all about caring about HIV/AIDS. In order to stick it out until the end, and see results, you have to really care about the issues.

Another lecturer concurred that persistence was essential if HCI was to succeed:

And once you start something, you finish it.

Students and HCI

A lecturer in the Department of Design and Studio Art said that she had requested a professional psychologist to assess the responses of students at the start and conclusion of a course with an HCI component. She said that the psychologist's feedback was encouraging:

She could really perceive positive shifts with regards to the students' attitudes to HIV and AIDS and to stigma around HIV/AIDS.

She also read some of the feedback students had provided about the course's HIV content:

Some of the experience was horrifying, but it was a great experience to learn more about it.

It was an eye-opener, it totally reduced my fear of getting tested.

It was inspiring, and I have learned a lot about how the virus can spread.

It was mind-opening, I also learned to look at things from an HIV-positive person's perspective.

A lot of the myths around HIV were debunked, and I realised that I didn't know as much as I thought I did.

Innovation

Following a workshop on critical diversity literacy which HEAIDS had facilitated for academic staff at CUT, lecturers in the Department of Design and Studio Art decided to be innovative:

We decided to take the critical diversity framework as a framework to develop the fourth year or

BTech level in our programme. The critical diversity workshop that we attended, that really helped to empower us.

A design and studio art lecturer explained:

The biggest thing is that [the training] really enabled me to open a dialogue with students, and be able to talk about these kinds of issues, including HIV, and change that divide of us and them – that HIV could never happen to me, it only happens to them. It broke down the barriers of us and them, and made it more of a community of all of us, where some members of the community struggle more with HIV. You feel clearer about what HIV/AIDS is, and you can point the students in the right direction. So I think that was very helpful for me.

Her department had taken innovation further:

We also incorporated HIV into our community project, and so one of the themes was to subtly incorporate it into cartoons, based on the theme 'Bloemfontein, My City'. And what was nice was that an article was published in *Die Volksblad*, and the whole article was about HIV/AIDS. So were able to reach into the community. And while we were painting murals in the community, people came to ask, 'What are you doing?' and 'What does this mean?' And that also helped the students who had just had the HIV training, to have a dialogue with people in the community.

Overcoming student reluctance

A Welkom lecturer and member of the HCI team spoke about his experience of student attitudes to HIV integration:

So the thing is when I was introduced HIV, the students were not happy, they had some attitudes, they said they had heard it all in high school. So then I said, 'Let's talk about critical diversity, it's not all about AIDS.' So they commented visually, they didn't write. Then they felt like they could respond. There is a lot of things they go through, that you can't see. I must say that as a lecturer, it wasn't easy. It was a challenge to find a way to incorporate HIV, or gender inequality or racial issues. So I ... got some advice, and tried to incorporate case-studies,

and at the end of the day, I could see, there were good results.

Unexpected benefits

When asked about the benefits that they had derived from the HCI project, the CUT team members were unanimous that they had found it to be of real value. One person highlighted the personal benefits he had derived in terms of community engagement:

I benefited because I am also a member of the community, in terms of self-empowerment, and it was easier to spread the news about HIV to churches and to community organisations in the community that I belong to.

For another academic, the major learning had been with regard to diversity:

... I benefited because I attended the workshops and I was being opened to different views. It created more empathy for different kinds of people, and sensitised me, for example, to LGBTI issues.

His colleague had similar perceptions:

We are able to talk to people about the pains [real-life challenges] that students and members of the community have in life about HIV, understand their story and respond better.

For the design and studio art lecturer, the critical diversity literacy workshop had brought enormous benefits:

I think I benefited most from the critical diversity workshop, and I think it is something everybody should be exposed to, because it is essential, living in a diverse society, and also being in a powerful position to influence others through our academic teaching. As my colleague said, hearing the stories of people from different backgrounds, it opened me up and helped me to become softer.

Linking HCI with the higher education research agenda

The team members had been able to write some research papers based on their experience of HCI, for which they were very grateful. The team leader remarked:

I don't see myself really as a traditional academic. But we facilitated a three-day workshop in December – a writing retreat – and we had to rely only on ourselves, and out of this process, I can see that we can get three articles at least for publication. The beauty for me – one has been published already – is that we had to rely on ourselves, and work together. Then let us make this an annual event – a writing retreat – and get to publish more.

Another academic agreed with her:

It was awesome, because out of it I could see that I could tailor a training and development programme, where you could take people through this process of how you could ultimately get your article published – the writing right from conceptualisation. Even before the actual writing retreat, people had specific small tasks to do – write an abstract, follow the following guidelines, identify a journal that you want to publish in, and start structuring the body of your article ... I really felt humbled, when I saw what came out of it.

HCI and broader educational transformation

For the CUT team, the critical diversity literacy workshop had been perhaps the most useful part of their HCI journey. One academic commented:

The thing about the critical diversity workshop was that it helped me look at how I evaluated people on first meeting, whether they are black, or gay, or from a different socio-economic class. We form

“It's about constructive engagement where we break these unnecessary walls, and admit there is diversity. HIV/AIDS will probably be just one of those issues that need to be addressed.”
– Central University of Technology FGD

an opinion about who someone is based on first impressions. The workshop challenged that.

The team leader said:

So as a result of the critical diversity literacy workshop the Design and Studio Art Department felt that they could incorporate that into their advanced level course, so there was that progressive change. People were intrigued, they wanted to know who came up with this idea of critical diversity.

Her colleague said that critical diversity literacy had been really useful:

If you are not a psychologist, it is difficult to talk to students about issues of LGBTI or issues of sexuality, if you come from a more conservative background. If you come from a more conservative religious background, you don't talk about such things! So I think those were some of the challenges that we had to confront and deal with. But I think to do the training and that almost destigmatised it, and just took the edge off all of these concepts, and it helped to approach it more, almost to the point where you can feel more enabled to talk about these kinds of issues, and to create a safe environment for your students.

Where to next for the HEIs?

The HCI team appeared to have been invigorated and energised by their involvement in the HCI process. They were adamant that the project shouldn't come to an abrupt end:

So right now I can see light at the end of the tunnel – that people don't see the project just coming to an end, but they can see a phase 2 of the project.

One lecturer pointed to the urgency of writing up some of the research findings:

We need more writing retreats for research, because there is a lot of rich data that has already been generated out of this.

A student who was present for part of the discussion said that in addition to HCI, a fine art competition

could be used as a vehicle for HIV/AIDS messaging at CUT:

I would like a competition for the arts involving HIV. Many of the students after graduation are unemployed, even though they are truly talented ... A competition involving HIV could make their work known. And it would also benefit the student community and the bigger community by educating about HIV.

A lecturer agreed:

Yes, because visual images are understandable even for people with lower levels of literacy. It can take the images into the communities, and can be a platform to make students' work more widely known.

Team members thought that it was important to develop a distance learning component for their programmes:

Because of the *#FeesMustFall* and decolonisation processes last year, we have come to see that this process of students sitting in a classroom doesn't work when there is a strike or disruption, so we have to look at online delivery of education. We want ... to think about incorporating HIV/AIDS and diversity, but using a different delivery mode, rather than using only contact learning.

The project leader said:

I propose the establishment of a Unit of Excellence at CUT, where whatever has been accomplished can be taken forward. Fortunately, there was a unit that was established just last week called Unite, it is about transformation, it's about constructive engagement. I cannot say it was an effect of the HIV/AIDS project, but to a certain extent it can address the sustainability of this. It's about constructive engagement where we break these unnecessary walls, and admit there is diversity. HIV/AIDS will probably be just one of those issues to be addressed, we need to see it going through into that.

A lecturer shared his thoughts:

We need a unit. And we need co-ordinators in every department to make sure that the HIV/AIDS issue is covered. We also need to think about HIV as a stand-alone subject, even at school level.

An exciting option that the lecturers wanted to explore was having a closer working relationship with other institutions of higher education:

There is also the possibility of partnership, like we have already started with Wits and the University of Johannesburg, thirdly with Unisa with online delivery. There are many exciting possibilities that could develop out of collaboration.

Conclusion

The experience of the Central University of Technology shows that challenges early in the process of curriculum integration need not derail the HCI process. On the contrary, the combination of consistent and dedicated leadership and a team which feels passionate about HCI and committed to transformation can steer the process in an exciting and innovative direction. CUT deserves commendation for its zealous perseverance, and support in the more complex processes of transformation that lie ahead.

DURBAN UNIVERSITY OF TECHNOLOGY



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“I have enjoyed learning about the dynamic and changing impact of HIV/AIDS, especially because we work in the worst affected province. That has been a big selling point for the project.”
– Durban University of Technology FGD



Teaching is spread across five campuses in Durban and two more in Pietermaritzburg¹. In 2015, the latest year for which figures are available, DUT had a total student enrolment of 27 023².

Initiating the project

The co-ordinator of the HCI project at DUT began by explaining how the HCI project had been initiated at the institution:

... The Deputy Vice Chancellor (DVC) asked me to run with the project, because my role is that I am the advisor for special projects. At the time I was involved with a curriculum renewal project across the institution, and we had brought in what we called a General Education component, like a grounding programme, but not just at first-year level, we are taking it up to other levels as well. We were looking at active citizenship and critical citizenship, and global issues.

When the DVC and I discussed it, we decided that the HEAIDS concept fitted into my work, so I was tasked with writing the proposal. We got buy-in from the Deans, and ... we identified three faculties to get involved in it: the Faculty of Applied Sciences, Health Sciences and Management Sciences.

The institutional context

The Durban University of Technology (DUT) was formed in 2002 with the merging of two technikons in KwaZulu-Natal, namely the ML Sultan Technikon and Technikon Natal. However, the institution has much older roots, with the forbears of what would later become the Technikon Natal and ML Sultan Technikon respectively having been founded in 1907, more than a century ago. Technikon Natal focus was on providing technical training for whites, while the ML Sultan Technikon was founded to meet the urgent and unmet need for technical education for Indian people who had settled in the province. DUT's vision is to be a preferred university for developing leadership in technology and productive citizenship.

¹ https://en.wikipedia.org/wiki/Durban_University_of_Technology

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

Enablers of HCI

The project co-ordinator said that there were several criteria in place from the start to ensure that academics engaged meaningfully in HCI. These were crucial in ensuring the success of the project:

We had the Dean's buy-in, which I think was very good for the project. The three faculties each had to identify two staff members from their faculties who would be interested in the project. The criteria, built into the proposal, included who is enthusiastic, who is already doing something, and who has a passion for it – for innovation or for HIV.

Other group members identified a number of factors that had assisted the HCI project to flourish. One was that:

HCI wasn't seen as a stand-alone project, it was seen as fitting into an existing programme, and I think that gave it some *gravitas* in the institution.

One of the incentives offered to DUT staff members to engage in HCI was the provision of human resources, to provide relief from part of their workload, so that some of their time would be freed up to develop HCI initiatives:

The way we did it was that we paid so that the lecturer could get someone in to help them, to relieve them. I got excellent commitment from staff, because it was a lever. We said to the HODs, 'We are giving you some time, but we want something in return, in other words some of the lecturers' time to work on HCI'.

This had turned out to be a key enabling factor for the project at DUT:

If I reflect on the project now, this offer was a key factor in getting things moving ... we had something to give ... that we were offering staff support, and wanted something in return – a win-win situation.

A further factor which contributed to the success of HCI was being able to drawing on the existing knowledge and previous experience of the HIV/AIDS Unit at DUT:

When I did my proposal, I also drew on the expertise of a staff member who is in the HIV/AIDS Unit.

She started in 2008 with curriculum integration. And I journeyed with her a bit at that particular time. So when this project came up, she and I sat together to write the proposal. I said to her, 'Tell me what worked or what didn't work in what you did before with integration.'

Innovation

At the time of developing the HCI proposal for HEAIDS, it hadn't yet been decided whether or not DUT's first-year Cornerstone Module should have an HCI component. The plan was that this Cornerstone Module would eventually become a compulsory course for all new students. When the HCI proposal was being drafted the module was still in the process of development. Fortuitously, the group said:

A bonus when the project came about was that we added the Cornerstone Module to our HCI plan. What is exciting for us is that *every* student will be doing this module by 2020, which makes the project have incredible reach.

A male member of the teaching staff explained why he saw the Cornerstone Module as innovative:

For me it is a very interesting module because it touches on a number of important social issues – all the social issues affecting South African society. It is about HIV/AIDS, but it also addresses gender violence, the bill of rights, HIV stigma, and the approach is more about trying to engage the students in a discussion, not using the traditional way of teaching.

He said further that an unusual feature of the module was that it required reciprocity from the teaching staff, in that it obliged them to not only teach students so as to inform the students' understanding of HIV/AIDS, but also expected the staff to listen attentively to what students had to say about HIV and related issues:

We want to know what they have learned about these issues, what their own challenges are, and in the discussion we help them to overcome their biases. It gives them the background that these issues are the things affecting South African society, and HIV is then very important. For me it is interesting because you hear what the students know, and also when they bring up the myths around HIV.

An exciting innovation for DUT was developing HCI within the Department of Sports Science, a somewhat unusual home for it. A female lecturer in Sports Science admitted that she had initially been reluctant to become involved in HCI:

My Dean sent me an email, saying you need to join this [HCI] group, which I knew nothing about. (Laughter) So I contacted D, panicking ... so then we had a meeting, and I learned ... what my role would be. From then on we had other workshops which helped us to develop our curriculum integration, and also to support us in that. I felt easier then. After that it was just about deciding which module to choose to integrate HIV into.

Once she understood the rationale behind integration, she admitted, she became excited about the creative potential of HCI within Sports Science:

We wanted to be doing something different in terms of integration. So what we did is we used sport as a platform to communicate about HIV and awareness, and we left it to the students to decide what particular project they would develop for their sports body. They had various alternatives from which they could pick.

The lecturer noticed some unexpected spin-offs of HCI:

What is very unusual is that our students in different years started talking to each other about HIV – first years to second years, second years to third years. That was really strange for us. Normally our students see their studies almost as competitive. But now we had third years gathering information [about HIV] and giving it to the first years, and the first years in turn used that information to create projects.

In the end the Department of Sports Science was able to find at least one module across all three levels of study into which HIV could be meaningfully integrated. The Sports Science lecturer related that to her astonishment:

The students really seemed to enjoy it. Initially I was apprehensive, because I thought, they won't take well to having to do something on HIV. But they were very enthusiastic, they did something different.

Students identified creative and unusual opportunities for insertion of IEC messaging about HIV:

... a lot of them used sports events. Like our annual sports day, soccer, netball, indigenous games. So students used various sporting codes to do their projects.

With regard to Health Sciences HIV was integrated into Anatomy and Physiology. Here a creative and innovative strategy was identified to assist students to learn about HIV, and to have fun in the process:

The lecturers got the students to develop board-games, mostly around their health sciences knowledge of HIV/AIDS, but also linked to some social, political and economic issues too. The students developed some very nice games to play and also got to learn through having fun.

Given the HCI focus on building academic capacity, an important component of the HCI project at DUT was arranging and hosting workshops to build the capacity of staff members. The first workshop addressed an unusual issue: academic staff and ethics. When asked why ethics had been chosen as the very first area of focus, the reason given for this decision was:

We wanted to address how staff members interact with students, for example, how to deal with a student who discloses their HIV-positive status to you? We also looked at issues like: how ethical are you actually in your day-to-day practice with students?

The workshop on ethics was well received by staff members:

There was a very interactive session on ethics, and in fact it was so popular that we called the facilitator back for another session.

Another important issue addressed in staff training was thinking more critically about student assessment:

What is it we measure when we assess students' performance? This issue we picked up in the Cornerstone Module, where we found that students were still coming with the attitude from school that they are assessed on the content of their answers. So, for example, if my story about HIV is more traumatic than yours, will I get higher marks?

Assessment was not a trivial issue:

The students need to understand, and in fact so do the staff, what is it that is being assessed. Are you actually assessing what you say you will be assessing? And how ethical is your assessing and your practice?

Challenges

Implementing HCI at DUT did not take place without some challenges being experienced. The Cornerstone Module initially presented the biggest headache for the team:

Last year [implementing HCI in the Cornerstone Module] was a challenge because of the large number of students. Challenges were especially with monitoring and evaluation. Now at least we have had the training. It is a very sensitive course, because many of the students want to share their personal experiences of HIV during the course.

As the HCI project life-cycle came to an end, a major consideration at DUT was the sustainability of HCI, the team members said:

Lecturer relief was a good incentive to get staff involved in the project, but what do we now have to do to keep people on board?

With Cornerstone it has been easier because we have been able to keep the same lecturers in the module. But with Sports Science and other areas, it is more challenging as lecturers get moved within a department.

It was felt that the Sports Science lecturer who had initiated HCI would nevertheless remain a long-term asset regardless of where she taught:

In terms of sustainability she is a champion, because wherever she goes, she brings in some of her new ideas.

Collaboration

At DUT a number of opportunities for collaboration presented themselves. As mentioned previously, the HIV/AIDS Unit provided invaluable assistance in the development of the HCI proposal, having had

previous experience of, and expert knowledge about curriculum integration.

Another example of collaboration was with regard to teaching:

[A]nother module that was developed was HIV and Communicable Diseases in KwaZulu-Natal. And a person from the HIV/AIDS Unit, Melusi Dlamini, is actually teaching that module. So we are getting a crossing over of someone who is working in a support unit, now having the opportunity to teach in an academic unit. And that for me is really good to see. One of the spin-offs of that project.

An opportunity for HCI arose in the Faculty of Applied Sciences:

... through a community project, called Biotechnology, where they teach the students about [HIV]. But then each student must go and identify a local community, and they must present to that local community, they use posters, they use skits. And they are also assessed on that. So one academic member of staff has looked at the ways in which students share their information in the community, and what kind of information they actually share ... how do students get the message across to the community, talk to the community, in a way and at a level that the community understand?

In the Faculty of Management Sciences the law modules provided an opening for HCI:

The General Education module, Law for Life, and there was another law module too including HIV ... certain legal issues or questions around HIV/AIDS, and so that was what integration was based on – including workplace and employment issues. So very relevant to students going into management. More or less an infusion process was followed there.

Student response

Positive student responses to HCI occurred in a number of ways at DUT. For example, in the Cornerstone Module, first-year students were given the option of choosing between a range of topics for one assignment, with possible choices including the Bill of

“What is exciting about HCI is that we see students in different years of study sharing knowledge, rather than just keeping it to themselves. Students are enhancing each other’s lives.”
– Durban University of Technology FGD

Rights, gender-based violence and HIV/AIDS. What was surprising, the HCI team said, was that:

Quite a few of them made videos, quite a few made posters, songs, poems, and many of them were about HIV/AIDS. The highest percentage in fact chose HIV/AIDS more than any other topic.

Students’ willingness to engage meaningfully with HIV was reflected in the complexity of the issues that they chose for their assignments:

Some chose to discuss stigma, some discussed the myths, also interesting is that certain people chose to look at the possible origins of HIV/AIDS ... the challenges faced by the people who are most affected by it, the challenges that South African communities face ... how to combat stigma through education.

The focus group was asked whether there had been any uncomfortable experiences at DUT in teaching students using HCI. An HCI team member said that on reflection that this had definitely not been the case:

No. In fact, it was surprising to me how much students wanted to learn. Especially first-years, they would ask a lot of questions. But apart from their texts, they had third-year students available, who were doing a lot of [HIV] research, so they could go up to them and ask questions.

A male staff member at DUT concurred that he had had similarly positive experiences:

For me, it was like the students were quite inquisitive, they asked lots of questions, like: ‘Where did this disease come from?’ And also some students in class were bold to tell us they are HIV positive. It was a shock.

Other academics said that that they had also been the recipients of students’ confidences:

Students do come and talk to me more, I am more like the middle person. They do confide in me about their personal issues, not just HIV, also issues like housing and finance. And then I will go to my HOD and find out if there is anything we can do for that student.

The students’ awareness of the HIV/AIDS Unit was reflected in the making of a student video:

One of the videos that the students made was about the work done by our HIV/AIDS Unit. What really spoke to me of the strength of the project was the degree of integration between the HIV/AIDS Unit on campus, which does the ‘First Things First Campaign’ promotions, and addresses all other health aspects of HIV. The students actually even included those linkages in their video.

Unforeseen benefits

When asked whether the HCI project had brought her unexpected benefits, the lecturer in the Department of Sports Science responded by saying:

For me, it was the first project that I was involved in, so for me, it was about developing myself. And learning, together with the students, that was a major benefit. It was at first uncomfortable, always talking about HIV, HIV, but then you get used to it. And because it was integrated into so many modules, it became the norm. And we also learned about giving the students a safe space.

Staff members learned the importance of maintaining confidentiality when students talked about HIV:

“To what extent can we develop academics as reflective practitioners? That for me was a critical component.”
– Durban University of Technology FGD

Students knew that whatever they talked about, it was kept in that space, it wasn't talked about outside that context.

The co-ordinator of the HCI project felt that the experience had been 'a stretch' for her:

For me it was about managing a project that has so many requirements that you have to satisfy. That was huge! Even just the reporting requirements were complex. The learning for me too was that actually staff are willing to participate in projects, as long as you can help them develop a passion for it. That was key. And also giving them the safe space to grow.

On reflection, it seemed that the notion of a safe space was equally important for both students and academics, to enable them to develop.

Another benefit that the HCI engagement provided was:

... learning about the dynamic nature of the changing impact of HIV/AIDS. And we are also one of the greatest affected areas of the country. That has been for me part of the selling point of the project. Because we as KwaZulu-Natal are notorious for high HIV prevalence, and I am asking: 'So what are we doing about it?'

The team reflected that giving students and academic staff opportunities to learn without being unduly prescriptive had been crucial for the project's success:

The other learning that was good for me was allowing people freedom. Not being prescriptive, for me that was a good learning as well. A project can go well, without having to micro-manage it. Give people that safe space, and they can move.

Another team member admitted that what he had learned from HCI was how to deal more comfortably with diversity:

For me, it was about learning ... how to teach students from a wide range of backgrounds. And being surprised by how much some students responded, the intense extra effort they put in.

The HCI team members appreciated the opportunity they had been given to become learners again:

For me it was great having the three months' training, and being a student, going through all the issues in class that you would go through with the students later.

Another academic felt that her experience of HCI had been transformative

It changed my teaching approach. I learned a lot from reflection on my own teaching practices.

The co-ordinator said that she particularly enjoyed one aspect. This was:

... the capacity development that we built in. It was partly about, to what extent can we develop academics as reflective practitioners? That for me was a critical component. Cultural and environmental sensitivity was also important. We hooked it onto the graduate attributes we are aiming to develop at DUT.

Research opportunities

As at other institutions, the opportunity to engage in research presented itself during the course of the HCI project:

One of the other things we did through this project was we decided that we wanted to do action research. So ... we arranged a group skype workshop with one of the action research experts from the UK, Jack Whitehead. Staff members put their questions together, and we sent the questions to him, and then we had a skype workshop with him.

Team members agreed that the workshop had been valuable:

It showed us how action research is so interesting and important. Someone coming in and tying all the loose ends together about action research, that was what it was about.

A male staff member added that he hoped to complete and publish a research paper arising out of his involvement in HCI:

I am hopefully going to publish some research. We interviewed lecturers and staff about the experience

of Cornerstone last year, and why students chose HIV to work on in the second semester, what the challenges were. We have the raw data to analyse now.

Where to next?

Team members were excited about the possibilities that HCI had opened up for them. The academic member of the Department of Sports Sciences commented:

We are already committed to doing HIV in other ways at second year and higher levels. It is going to be within health issues in sport, but also in employment and management.

Sustainability was nevertheless a concern at DUT:

In terms of sustainability, that little booklet we developed is an example of something we produced that we hope to share with staff and which will help sustain interest in HCI. But integration is now also moving on to other issues, like entrepreneurship. Some of the processes we introduced with HCI can be used for other curriculum integration issues, so sustainability is broader than just HIV issues.

The project had encouraged academics to ask searching questions about what they were doing. This mode of critical enquiry was likely to continue:

In the Cornerstone Module, we will be asking, 'So what are we teaching? How are we going to teach it? And where does HIV/AIDS fit? Where are the shifts?'

Conclusion

The DUT experience illustrates the easy movement of an HCI project where enabling factors smoothed the way, and where there was consistent and determined leadership of the HCI initiative through the lifespan of the project. Long-term sustainability of HCI is clearly a concern, given that there are so many competing priorities for transformation at an institution of this size. Yet the enthusiasm and competence of the HCI team members in their engagement with HIV suggests grounds for optimism: that this initiative will leave an important legacy at DUT, and that the questions regarding transformation that it has raised will have knock-on effects and benefits for the University beyond the lifetime of the funded activities.



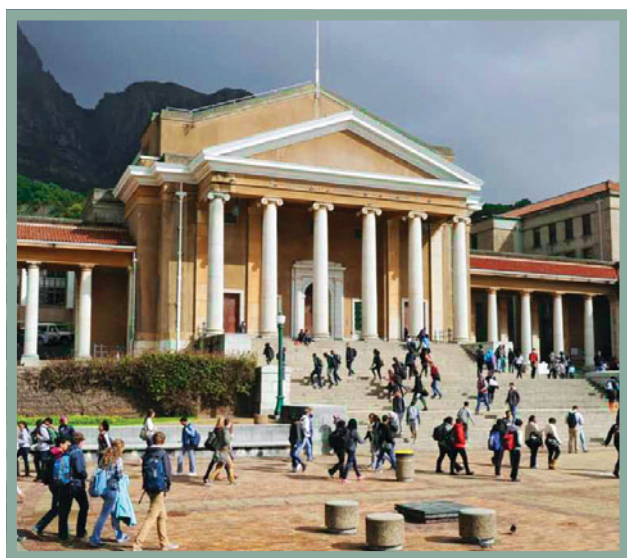
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Introduction – the institutional context



The University of Cape Town (UCT) is the oldest university in South Africa. Its roots date back to 1829, when the South African College was founded as a high school for boys, later also developing a facility for tertiary education. In 1887 female students were admitted for the first time. The formal establishment of the University took place in 1918, utilising a bequest from Alfred Beit and substantial gifts from mining magnates Julius Wernher and Otto Beit. Ten years later the university move to its current location at Groote Schuur, on land bequeathed by the politician and British colonialist Cecil John Rhodes. Black students were first admitted in the 1920s. Previous UCT alumni include three Nobel laureates: Sir Aaron Klug, Professor Alan MacLeod Cormack and JM Coetzee¹.

Today UCT has a diverse student population in terms of race and gender. Its current vision is to redress past injustices, promote equal opportunity, safeguard human rights and ensure that its teaching, learning and research uphold the inherent dignity of all and help to address South Africa's development needs².

The University has six faculties: Commerce, Engineering and the Built Environment, Law, Health Sciences, Humanities and Science. In 2015 UCT had a student enrolment of 27 809³.

HIV/AIDS, Inclusivity and Change Unit (HAICU)

The HIV/AIDS, Inclusivity and Change Unit (HAICU) was established at UCT in 1994, initially as the HIV/AIDS Unit, with the aim of providing leadership in the University's response to HIV/AIDS. HAICU was one of the first inclusive responses to the HIV/AIDS epidemic at an educational institution at tertiary level. It is based in the Office of the Vice Chancellor of UCT. HAICU's primary purpose evolved over a period of time into supporting the development of an AIDS-competent university community (including both students and staff members) (see Campbell *et al.*, 2005)⁴. The HAICU vision aligned with UCT's commitment to ensuring that its graduates develop a range of key competencies; amongst others, this included the capacity to address HIV/AIDS both personally and professionally. Furthermore, since

1 <http://www.uct.ac.za>

2 <http://www.uct.ac.za>

3 Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

4 Campbell, C., Nair, Y., Maimane, S. & Sibiyi, Z. (2005). Building AIDS competent communities. *AIDS Bulletin*, 14(3), 14-19.



2003 HAICU has co-ordinated HIV/AIDS curriculum integration (HCI) at UCT.

Currently HAICU's mission is defined as to co-ordinate a collaborative response that supports broader UCT transformation, and builds student and staff capacity through curriculum, co-curriculum and social responsiveness initiatives.⁵

According to HAICU staff, the *#FeesMustFall* protests catalysed the transformation discourse on campus:

HAICU had been focusing on HIV as a transformation issue since early 2000 as the HIV lens lends itself to viewing intersecting issues of gender, identity, violence and social justice. The HAICU approach focused on capacitating both lecturers and students to lead and discuss the intersections of health, identity and social justice. HAICU's approach in infusing intersectional issues such as HIV, has provided new ways of considering what a decolonised curriculum means to both staff and students.

Initiation of the HCI project

In 2014, in response to the HEAIDS request for proposals from higher education institutions for funding of HCI projects, HAICU submitted a proposal on behalf of UCT, which was approved by the Acting Deputy Vice Chancellor for Transformation, Emeritus Professor Anwar Mall. The HCI proposal had two principal components:

Firstly, an institutional review would be conducted within five of the six faculties at UCT to determine the current levels of HCI, as well as the feasibility of further HIV/AIDS integration. The faculties were: Health Sciences, Commerce, Science, Engineering and the Built Environment, and Humanities.

The second component envisaged strengthening infusion of HIV/AIDS into five courses within the five specified faculties, in collaboration with academic staff members within these faculties.

Overview of current HIV integration

The review of current HCI at UCT was conducted in 2015 and the results were finalised in the following year. The review utilised the following methods to draw together information:

- a desktop scan of faculty handbooks
- HAICU staff members attending faculty meetings to obtain first-hand information from faculty academic staff members
- notifying academic staff about the HCI grant and requesting them to provide any additional information not available elsewhere, and
- collating the information derived from all these sources into a single database, to provide an overview of existing HCI within the University.

The completed institutional review was then submitted to the University Council for adoption.

5 <http://www.haicu.uct.ac.za>

According to HAICU:

The review started out with a focus on what was being taught in terms of HIV/AIDS knowledge. It was important to understand where there were gaps, and what the needs were in each course in order to infuse the relevant content.

The institutional review found that in 2014, 141 courses that were taught at UCT included content related in some way to HIV/AIDS, with a total of some 201.5 notional hours of teaching. A majority of these courses fell within the Faculty of Health Sciences, with a further significant proportion in the Faculty of Humanities (primarily in the Michaelis School of Fine Art). More than two thirds of the notional hours of teaching with an HCI component took place in these two faculties⁶.

The preferred approach at UCT for HCI was clear. According to HAICU, it was using the infusion model, i.e. integrating and blending aspects of HIV and AIDS into disciplinary subject matter in such a way that it was in keeping with the intellectual and professional needs of the discipline, so that students, understood HIV from a lens of graduate competency and management thereof as it pertained to their discipline.

Strengthening of HIV integration

The HCI proposal envisaged strengthening of HIV/AIDS curriculum integration across five faculties at UCT, and the project achieved this within at least one course in each faculty. Three outstanding examples are described in more detail below.

Health Sciences: HIV knowledge contributes to professionalism

In the Faculty of Health Sciences, the HCI project strengthened an existing course at first-year level which benefited all students studying for any of the health profession qualifications. Reflecting on the process, an academic in the Faculty of Health Sciences said:

The HCI workshop is in fact not something that is completely new ... it is an extension of an earlier partnership between HAICU and the School of Public Health. It fits into a first-year course ... *Becoming a Professional*. The workshop is called 'Me and HIV', and it's a compulsory workshop for all first-year students ... whether they are students of medicine, or occupational therapists, or speech therapists, physiotherapists ... It's run over a number of weekends, and is a mandatory part of their degree.

Each weekend workshop had a richly varied programme. It included presentations by guest speakers, facilitator-led small group activities, and a consolidation lecture at the very end. All students completed a pre-evaluation questionnaire at the beginning of the workshop to determine their pre-existing knowledge, behaviour and attitudes in relation to HIV/AIDS. At the end of the workshop a similar post-evaluation questionnaire was administered.

Comparison of the pre-evaluation and post-evaluation results provided a useful indication of changes in students as a result of the workshop.

According to the Health Sciences lecturer, for students an essential component of the learning process was learning in small groups:

The small groups are very important ... no more than 12 or 14 in each group. And it gives students an opportunity to talk about issues, sensitive issues such as sex, that the students can discuss issues that they wouldn't easily be able to talk about in the big group. The facilitators of the small groups are comprised of academic staff from the Faculty of Health Sciences and the peer educators from HAICU (the Agents of Change Educators or ACEs) they're highly skilled ... they make sure the students feel safe and that they can talk without feeling embarrassed.

She felt that what was distinctive about the 'Me and HIV' workshop was that:

It goes beyond just acquiring knowledge ... it gets the students to reflect on their own experiences of

6 HAICU. undated. *Mid-term narrative report*. Cape Town: University of Cape Town.

HIV, and their perceptions ... it brings HIV back to being a personal issue, students also think about their own vulnerability. They end up having very different ideas about HIV compared to when they started.

The post-evaluation feedback about the workshop had been truly heartening:

We get such incredibly positive responses. The students learn so much – about sexuality, also about race, gender – and understanding trauma ... They also learn how all these issues interact. I would say that they definitely do come away changed. It's very important that the workshop teaches them about empathy, especially for people living with HIV... Also about what it means to be a professional.

Engineering: subtle infusion of HIV in a foundation engineering course – Professional Communication

Within the Faculty of Engineering and Built Environment, a different approach was taken for HIV integration: partly because unlike students in the health sciences, engineering students were likely to complain that HIV/AIDS wasn't their issue, or that it wasn't necessary for them to know much about it. Students would lack enthusiasm for a course that addressed HIV/AIDS directly. In response to this challenge, HIV/AIDS was skilfully embedded into a professional communications course for engineering students. According to an engineering lecturer, infusion of HIV had been achieved ingeniously:

The focus was on technical writing, being able to put together a good technical report which includes the use of visual aids like graphs and pie-charts to make a presentation that was effective ... So HIV was brought in as if it was just one of the issues facing somebody running their own company, like an electrical engineer. So for example, the engineering student finds out in the scenario which is given out, that 13% of the company's workforce is known to be HIV positive. Then there are also x per cent of employees whose HIV status is unknown. So in running the company, how does an engineer plan around that, and manage the problem of HIV?

The academic said that HAICU had been very helpful in coming up with a plausible approach for infusing HIV content into the communications course. Students needed to understand the socio-economic context of HIV and how this could impact on the labour force that engineers worked with:

It was a really inspired idea, bringing in HIV as just another aspect of what it means to be a professional engineer today. So the scenario was very convincing, it seemed real.

Engineering lecturers were enthusiastic about having an HIV component in the curriculum, he reported. However, he pointed out one challenge: lecturers felt inadequate about teaching which involved extensive HIV/AIDS content:

Here HAICU was a great help ... there was a good synergy between Electrical Engineering and the HAICU Unit, bringing different strengths to this course. They could teach about the HIV aspects.

What had helped with getting buy-in from students for the course was that professional registration as an engineer required candidates to be able to manage HIV/AIDS effectively in the workplace, including address human resources management issues.

The engineering academic related that she had been struck by how some engineering students were poorly informed about HIV despite the issue having seemingly been addressed *ad nauseam* in schools:

Knowledge levels differed a lot, we found. Students from privileged backgrounds varied in terms of how much they knew [about HIV]. Some even came across as well informed. However, it also concerned me to see that students from the rural areas or from outlying areas still had limited knowledge about HIV.

After completing the communications course, she said, hopefully students could return to their respective departments retaining the core knowledge of HIV they had acquired, and be able to utilise their new understanding across the board.

One challenge relating to the sustainability of the communications course was similar to the issue that had arisen in Health Sciences:

Engineering lecturers are sometimes unwilling to teach students about the essential facts of HIV. But in the future HAICU staff members are not necessarily going to be available to just step in and provide the HIV/AIDS input ... In the long run, engineering lecturers have to develop enough confidence to teach the HIV content themselves. One way to make this happen may be to rotate teaching the different components of the course between different lecturers.

Fine Arts: Socially responsive art engaging with HIV/AIDS

A different approach to HIV integration was taken in the Michaelis School of Fine Art, which fits within the Humanities Faculty at UCT. In the years before the current HCI initiative, fine arts students had engaged with HIV to a limited degree since 2012 – for example, students had been given the task of creating art works relating to HIV prevention. However, according to a fine arts lecturer, the HEAIDS funding had enabled curriculum integration to be taken to new levels. A decision was taken that HIV infusion would take place at two different levels of levels of study:

We have found two different opportunities to get students to reflect on HIV, during their first year and at a third year level. We talk to the students about HIV as well as a whole range of social issues ... such as gender-based violence, racism, xenophobia. The 58 students were divided into eight groups that collaborate on artistic responses to the issues we have discussed in class.

He explained further that the first-year class was only able to provide a brief introduction to HIV/AIDS, because it ran for a brief period (3-4 weeks), and involved more than 90 minute lectures; feedback sessions; discussion sessions and two 45 minute tutorials over and above individual consultations. Clearly students were exposed to the complexities of HIV/AIDS and social justice, and their appetite was whetted to be more meaningfully involved in grappling with these issues.

This process of engaging with HIV/AIDS deepened substantially in the third year of the Fine Arts degree, the lecturer said:

The third-year HIV component, titled as the Public Practice Elective is scheduled in the first two terms, and it's taught over four weeks during the 12 week semester. There are four electives to choose from thus we teach a smaller group of students, in 2016 8 students signed up for the project. Teaching a smaller group allows us to have longer, more engaged discussions with individuals students about their ideas.

A smaller class at this level enabled funds to be set aside to pay for art materials for the students:

What is possible at this level is very exciting, partly because additional funds are available to cover some of the costs of the materials that are needed to create the artworks. This allows students to focus on the develop and implementation of ideas instead of being focussed on generating funds for their projects.

The lecturer said that students were discouraged from responding in a strident and didactic way to HIV/AIDS, or taking a passive approach:

We want them to become more aware of the social justice issues, their role is not necessarily about providing solutions but to think critically about what they can add to the ongoing public dialogue surrounding these issues... At this stage we really want to challenge the students' perceptions of what constitutes art and how that links to the intersecting issues of health and social justice.

An innovative element in the third-year HIV project required students to engage with local community organisations involved in HIV/AIDS work, such as the Desmond Tutu HIV Foundation, to arrange installation of the artwork they created. The lecturer pointed out that the process of community engagement was a logistical challenge for the students:

Commuting between the UCT campus and the community we were collaborating with consumed a fair amount of time, luckily we were in a position to fund the travel to facilitate the project.

However, feedback from third-year students who had completed the HIV component showed their unre-served enthusiasm for taking on its challenges.

Challenges

During the period of the HEAIDS-funded HCI project, UCT faced a number of institutional challenges which affected implementation of HIV integration. The *#FeesMustFall* protests engaged academics at UCT in different ways. For example, a key element of the student occupation of the Fine Art campus in 2017 was a discussion on how to utilise art as a strategy to reflect on the challenges of transition, power and protest. Academics engaged with this concept to focus on how issues of social justice may have a place in core curricula. As a result of student movements, relevant education that addressed issues such as race, privilege, power and violence was sought after by some academics at UCT. In response, HAICU encouraged thinking about HCI in a manner that provided an intersectional perspective, which acknowledged these social justice issues and spoke to how they too exacerbated the HIV/AIDS epidemic.

HAICU stresses the importance of having individuals or units within faculties at UCT who take on particular responsibility for championing HIV curriculum integration. The primary benefit of this is that integration projects are likely to be seen through to their natural conclusion. A further advantage is that HCI is more likely to become a faculty or institutional project, and so its sustainability is not dependant on particular individuals involved in curriculum development or teaching.

HAICU points out that the long-term sustainability of HIV infusion into the curriculum at UCT is a particular challenge. The approach to HCI taken at UCT involved academic staff being able to make autonomous decisions about where in the curriculum HIV infusion should take place. Not all lecturers necessarily have an HIV/AIDS knowledge base sufficient to support informed decisions about where HIV would be a good fit. Another factor influencing curriculum

integration is academic's willingness to pilot infusion into their core curricula. Often, infusion requires input of additional preparation time in terms of training, materials development and adaptation of assessment tools by the HAICU team.

HAICU acknowledges that teaching which is solely inclusive of HIV is not sustainable in the long term. It believes that ultimately a holistic social justice framework for learning about HIV makes much more sense. The notion of intersectionality provides a meaningful contextual frame: examining and learning about HIV within the setting of broader health issues, mental health and general well-being, as well as utilising lenses such as gender, race, class, ethnicity, citizenship and disability to understand its complexities.

Conclusion

The remarkable success stories of HIV curriculum integration at UCT incorporates important lessons which other institutions of higher learning would do well to learn from. Clearly the involvement of HAICU, with its specialist skills and historical knowledge base, has been an important enabler of meaningful engagement in HIV integration. Furthermore, allowing academic staff to make autonomous decisions about where integration needs to take place has played a key role in securing buy-in from staff members. The use of the infusion model of integration has been strategic in nurturing student motivation to learn more about HIV and associated issues. Finally, the location of learning about HIV/AIDS within an intersectional frame that incorporates a range of social justice issues makes a great deal of sense for long-term sustainability of this vital initiative. Long may UCT continue to find ways to produce graduates with a broad spectrum of competencies and a keen passion for social justice.

UNIVERSITY OF FORT HARE



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Institutional context

Founded in 1916, the University of Fort Hare (UFH) was the first institution in sub-Saharan Africa to allow black Africans access to a university education. It has a long and proud history. Previous students include remarkable individuals such as Nelson Mandela and Oliver Tambo of the African National Congress, Robert Sobukwe of the Pan-Africanist Congress, Chief Mangosuthu Buthelezi of the Inkatha Freedom Party, Nobel Peace Prize winner Archbishop Desmond Tutu, Kenneth Kaunda, the first president of Zambia, and Julius Nyerere, the first president of Tanzania. The university's oldest and

more established campus is in Alice, while there are secondary campuses in Bhisho and East London¹. In 2015 UFH's total student enrolment stood at 13 458².

Defining HIV curriculum integration

The UFH proposal for HIV curriculum integration (HCI) defined the principal and long-term objective of HCI at Fort Hare as being to improve the capacity of academic staff to integrate HIV/AIDS and related issues into curricula in all faculties by 2030, i.e. to enhance the development of new curricula which would be inclusive of HIV/AIDS. Achieving this goal was

¹ https://en.wikipedia.org/wiki/University_of_Fort_Hare

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

regarded as likely to make an important contribution to Fort Hare's vision of producing well-rounded and skilled graduates with an awareness of HIV/AIDS and other important social issues.

The pilot project for the HCI process was identified as training and capacitating academics in HIV/AIDS curriculum development and integration in three key academic areas:

- In the Life, Knowledge and Action (LKA) grounding programme in the Transdisciplinary studies under the office of the DVC AA
- In the Faculty of Health Sciences, Department of Nursing Science and
- In the Faculty of Law.

The Guskey model

The HCI team mentioned that Guskey's model of the five stages of professional development³ had been chosen as the most useful framework for appraising changes in academic behaviour with regard to HCI at Fort Hare (see box). What is distinctive about this model is that the last stage emphasise the impact on students as a key variable, which can in turn encourage further academic behaviour change. Hence for example, obtaining meaningful feedback

Guskey: Five stages of professional development for academics

1. Improve the design and delivery of programmes.
2. Validate the relationship between what was intended and what was achieved in terms of change.
3. Ensure organisational support and that organisational culture supports the desired change.
4. Ensure that participants use the new knowledge and skills to implement the new practice.
5. Changes in teaching practices are sustained when academic development and implementation of new skills is reinforced by evidence of improved student learning.

from students about courses informed by HCI that the students have found such courses beneficial is helpful in terms of motivating academics to continue implementing HCI, and is crucial to the sustainability of HCI beyond the funded project's time-frame (see also Student responses).

An early challenge

Right at the start of the HCI project, the University of Fort Hare experienced unexpected difficulties.

One academic commented:

We struggled with starting late with HCI, we also struggled with leadership issues which led to us not spending our budget as much or as quickly as we should have. We are only now involved in catching up.

The initial problems were described in more detail by other group members.

HEAIDS awarded the University of Fort Hare a grant for HCI slightly later than expected, only at the end of 2014, owing to amendments of the original proposal that were required before HEAIDS could agree to fund the project. The leadership issue mentioned related to the question regarding which unit or academic department would be best suited to take the lead role in developing HCI at Fort Hare. The grant was made conditional on the project being led by the University's HIV/AIDS Unit. However, soon after the funding had been approved, it became apparent that such an arrangement did not make institutional sense, as the primary objectives of the HIV/AIDS Unit were not in accord with the HCI grant's focus on curriculum development and, particularly, its stress on enhancing the pedagogical capabilities of academics (rather than focusing on students).

The University of Fort Hare then took a decision to reposition the project so that it would enjoy greater academic leadership, and hence the School of Health Sciences was assigned the lead role. According to a team member who belonged to the School, her academic unit was a more appropriate home for HCI,

3 Guskey, T.R. (2000a). *Evaluating Professional Development*. Thousand Oaks, CA: Corwin.



because of the School's longstanding engagement in addressing the multiple challenges of HIV/AIDS in South Africa, albeit primarily on a biomedical level.

Pilot project experiences

Most often higher education institutions select a standard approach for the implementation and teaching of HCI. In the case of Fort Hare, the three academic departments involved in the pilot project all chose differing approaches for teaching about HIV.

The Department of Nursing Sciences decided to use its traditional problem-based learning approach in conjunction with community-based education to teach about HCI. This had previously worked well, particularly in terms of studying various diseases and their management. In contrast, the Law Faculty opted for a case-study approach as its vehicle for getting students fully engaged with HIV.

In contrast to these choices, the Life, Knowledge and Action (LKA) grounding programme felt that an active learning approach was more feasible for its large classes of first-year undergraduate students, most of whom would be new to university life. Active learning comprised a range of teaching methods, which included showing videos or films to a class to trigger

student reaction and discussion, inviting lecturers from the Department of Nursing Sciences or other departments to visit and be guest speakers, thus providing students with well-informed and relevant information, and finally, employing the small group discussion approach prior to submission of written assignments, so that students would be compelled to engage with each other about HIV/AIDS, hence generating active discussion and debate.

A lecturer in the LKA Department said that personally he felt very comfortable about embracing HCI because of the prior history of his department. He pointed out that in all there were six themes in the LKA grounding programme:

... and one theme deals specifically with HIV. It's a trans-disciplinary course. Our students, of course, come from different disciplines, but all first-year students do this course, and really we want to make a point that they *should* get something out of it about HIV. The course has always been there. So it hasn't been very difficult for us to get on board with the HCI programme.

A Nursing Science academic thought that HCI had been less challenging for her discipline than for the other two academic departments involved in the pilot project:

For us in Nursing Science we had the curriculum already there, it was just evaluating and looking at it, to see: is it still relevant? And if it is relevant, are we teaching it in such a way that the students are coming out afterwards, and being really able to address the current issues in terms of HIV?

She added that she had benefitted personally from HCI in a way that she had not imagined:

It afforded me the opportunity to be able to get to know people from different faculties and it has been wonderful to find a common ground. It doesn't always look like that is possible.

In contrast, a lecturer in the Law Faculty spoke of the challenges of inserting HCI into an already full curriculum:

In the Faculty of Law, we ended up integrating HIV into our existing modules. And it wasn't about

integrating HIV into every module, because in a four-year degree there are over 40 modules ... So what we then did was to select modules at every year level which could more deliberately incorporate HIV – at least one per year. For example, in our first-year course, the Legal Skills Course ... It is designed to inculcate certain skills that lawyers need. We are dealing with case knowledge, dealing with issues of discrimination and so forth.

And the second module that we chose at first year was a substantive course, which was Family Law, particularly issues of domestic violence and so on. At second-year level we chose similarly, a skills course and a substantive course ... Professional Skills and Ethics, and there the students organise mock trials, including a case involving HIV/AIDS. And at third year it was Labour Law, and in fourth year HIV was in the Human Rights course, as well as in the module on Advanced Legal Writing.

In practice what happened in the Law Faculty was that the HCI theme was sustained through the full course of the law degree.

A female colleague in the Law Faculty spoke of her experience:

When it started I was a bit sceptical about it: I worried about information overload, we wouldn't have enough time. But then when we had training I realised that this is *not* what you thought it was. You are not having a focus on HIV/AIDS, you are *touching* on it. Our approach is rooted in dialogue and debate, it is facilitated and not taught. So long as you facilitate properly, these [HIV-related] issues come up naturally.

Colleagues in other faculties agreed with her in this regards. What seemed to make most sense was for HIV to be raised as an issue organically, so that it could appear to students as if it was just an incidental issue, and not necessarily a primary focus.

Enabling factors

Two enabling factors emerged from the focus group discussion as having been key to successful engagement with HCI at Fort Hare. One important contributory factor was that Fort Hare academics

were passionate about expanding their repertoire of skills and they wished to challenge students to think critically about HIV. The other factor was that there was a commitment by Faculty level leadership to the process:

What has helped us in this work is the involvement of management – like our Deputy Dean sits on this committee. Our current Dean used to teach Labour Law, and the Deputy Dean teaches Family Law, so we got buy-in from them. Many projects fail because things are done on the side, without the support of management, so they don't develop traction. Here, it was driven by the leadership.

Challenges

A young academic spoke of a particular challenge which she had first experienced in teaching HIV as part of the LKA grounding course. The challenge was the attitudes of some of the other teaching staff, including some tutors:

All students attend the grounding programme in first year – and HIV is there ... now with us, playing the mentoring role [in engaging tutors and lecturers about HCI], it wasn't easy at all. You try and organise meetings and people think it is a waste of time. All of these things are here and are known, but there wasn't a way of ensuring that anyone who taught it would cover the same ground.

However, these negative attitudes later shifted, she said:

Now it is different, it is really clear.

Another difficulty related to the issue of how HIV knowledge should be assessed:

The challenge of integrating into existing modules was also how to assess for knowledge that is also relevant to this. For example, in Labour Law, HIV might only be 10% or 15% of the exam, so a student might pass the course without even covering HIV issues. So how do you address that? How do you strategically assess so that you are confident that the outcomes of that specific module have been achieved? That is one of the challenges.

But the group also spoke of solutions evolving to meet such challenges:

... what I have noticed from my colleagues, is that they needed a workshop on curriculum development. They are struggling with assessment. It starts with the learning outcomes – to build the content, the skills – and how can you integrate HIV/AIDS without compromising the NQF levels of the students?

A law lecturer commented about the importance of clarifying values in relation to engaging with HIV:

In the workshop that we had, one of the things that we tried to do was to ensure that students not only gain content, but also consider policy issues from a legal perspective. What are the values, and sometimes conflicting values? If they don't see it in context, it doesn't bring change in perceptions. So policy and values, we need to incorporate them into our courses.

Another academic said that engaging with HCI had heightened her awareness that students had widely differing levels of knowledge about HIV. This could present a daunting challenge:

It is difficult because their content of knowledge [of disadvantaged students] is not on the same level as students from more advantaged backgrounds ... when my colleagues were talking about Family Law, although I know nothing about law, I can visualise how that discrimination comes in for HIV and AIDS. It is so clear. And some young people understand this too. But those who do not understand those terms, what happens to them? How do you include them? You can even talk about those people who are excluded from the knowledge content of the very terms themselves.

A colleague in Nursing Science said that HCI had provided new ideas for teaching, but also pointed to current constraints within the University itself:

In our work in HCI in nursing, we have explored the potential for more problem-based learning

– working in small groups – but the constraint is that we require many more teaching venues, which we don't currently have, the cost is prohibitive. It is better if students can discover knowledge for themselves, as they do using this methodology.

Unfortunately we had to retreat to more conventional methods that are still interactive but not quite as experiential, such as case-studies.

The difficulty of measuring the impact of HCI was also highlighted during the discussion:

A lot of impact is not measurable directly – it is actually long term.

Learning new pedagogies

An early benefit arising out of HCI that lecturers appreciated was the pedagogical training provided to tutors in the LKA grounding course:

The training of LKA tutors has definitely made a contribution to human capital development. The tutors have not just been trained, but they have also integrated HIV into their *Umthamo 6* course twice last year already and evaluated it.

A law lecturer said that HCI had raised the bar regarding teaching practice:

We then had to go for training on how to make our pedagogy more interactive. Almost moving towards dialogic and a conversation-oriented approach, putting role-plays, mock trials into the classes. Changing our methodologies of teaching ...

An academic teaching in Nursing Science was impressed with what the new teaching approaches had made possible:

... also for Nursing Science there is definitely a shift, because we had the information, but we were not focusing on the current issues, but once lecturers attended the [pedagogy] workshop, they changed

“Through HCI we are alive to connections across disciplines. As teachers, we can be more reflective and reflexive about what we teach.”

– University of Fort Hare (FGD)

“Students say they enjoy learning about HIV. They particularly become comfortable in talking about HIV to communities.” – University of Fort Hare (FGD)

their ways. You could see in the student dramas that the students developed that we were speaking to the relevant issues. For me that was a clear sign that this is working.

A colleague concurred that new ways of learning had been opened up:

What is more engaging is to come up with scenarios – case-studies – they are more engaging for students. Each group is given a case-study, and some involve a person who is HIV positive. Somehow each student gets involved. That is how it can be taught.

The group was in agreement about the benefits of the new teaching approach:

The [pedagogy] workshop we had last week, the responses show that the staff are really, really happy that it is continuing.

It was exciting to hear how notions of learning had expanded in unexpected ways:

Moving more towards learning-centred approaches this year, one way to work was to get the students to create a reader, based on articles they select. So it is a reflection of their knowledge, sourced by them. It also helps us to identify what issues are important to them.

Student responses

Bringing HIV into the curriculum had raised some unsettling issues for students, one of the lecturers pointed out:

Because some of our children and our students cannot understand the terms [concepts with regard to HIV/AIDS] they are taught, and then when they go home, they cannot articulate any of them. And then people who are there to support them are not able to support them because they cannot link that term with anything. So they need somebody to come in and help them.

She added that there were greater life challenges for students living with HIV:

The support system in the university, I cannot say it is not there, but the students need more support from the university structures. Because some of the students, they still feel they cannot show up, they cannot confide in anyone, and so for example, they have a CD4 count that is very low, and they cannot share that with anyone.

Notions of HIV stigma and taboo continued to be thorny issues, although the group agreed that they could also see shifts in attitudes among students who had attended courses with HIV integration:

For some students, it is still a taboo, and they need that the institution, they can talk about it. So that nobody is ashamed. But everybody is now aware, we can talk about it.

One of the academics commented that students' feedback regarding HIV curriculum integration was truly encouraging:

For the most part, 95% of evaluations from students were positive. There really weren't any negative ones, maybe just one or two. You could definitely tell, it added value to the curriculum.

A new lecturer agreed that students appeared to welcome HCI:

I have just joined the [HCI] project at the end of last year. But when I am looking at the evaluation reports from the students, they enjoy learning about HIV, it confirms what my colleagues have said. Especially in the community projects they are involved in, it made them to be comfortable to talk about HIV in those communities. And they have found more methods, more innovative ways to address HIV and AIDS, demystify those myths that exist, they are able to interact with those communities. Even to the extent that the communities are telling the students, they want them to come back, that they can do more. The students, they learned a new way of engaging, even with people outside the university setting.

Unexpected benefits

A female lecturer said that her established ways of teaching had been challenged by HCI:

I think one thing I have realised is the temptation to be narrow in your focus, and keep only to what is happening in your discipline. So it is very informative to hear what others are experiencing, their challenges, but also their knowledge. We gained a lot of knowledge. And it also helps us with facilitation styles.

Participants in the University of Fort Hare focus group discussion were excited about cross-disciplinary fertilisation:

I think I am now much more aware of what is happening in other disciplines, like LKA and Law. At the last workshop, it was really interesting, because it was like we were not just listeners, we were also facilitators of law! (Laughter) It was so nice, it breaks down the barriers. And now, we are not just colleagues who don't know each other, I am known even with other staff.

A male colleague agreed:

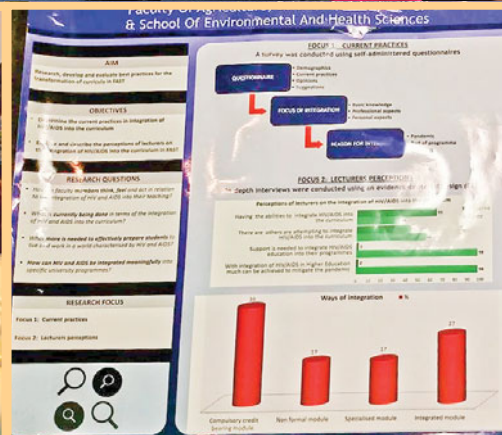
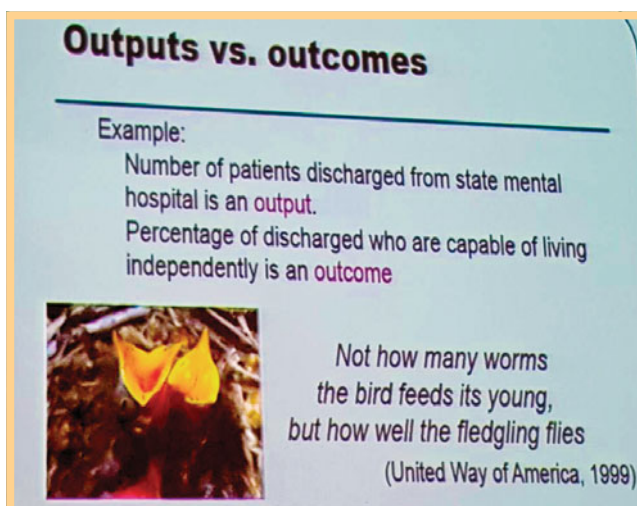
I think we are alive to connections across disciplines, and I think as teachers, we can be more reflective, and more reflexive, about what we teach, even who we teach it to.

The project co-ordinator spoke of how her interpersonal skills had expanded because of the HCI project:

I think also for me, because I have been co-ordinating this project since it started, it has really made me grow as a person. It has forced me to develop skills I didn't have, like if I have to convince people to attend a meeting, I know all the tricks to get them there. I have got to persuade that person that they will be comfortable in that meeting.

A colleague agreed about the building of new connections between lecturers involved in HCI:

I think we have learned to understand each other more, to collaborate more across disciplines, and to assist each other.



The possibilities for research had similarly been expanded:

Because of where we are, and the relationships we have formed, we are more able to visualise the future, and the possibilities, of collaborative research across disciplines, and scholarship.

What was truly intriguing about the process of HCI at the University of Fort Hare was how academic staff had seized the opportunity to build bridges between faculties and disciplines. It was encouraging to hear how much lecturers felt they had been able to learn from each other, in terms of both pedagogical skills and specialist knowledge. The HCI team's sense of camaraderie was tangible. There was no doubt that this could strengthen teaching practice and enrich students learning, because it would enhance the interconnectedness of different bodies of knowledge.

Taking the next steps

The group identified a range of logical steps which they wanted to take when the funding came to an end. One was the issue of developing HCI champions:

Each faculty should identify a champion for the HIV and AIDS curriculum integration outside this funding process.

Another was the expansion of HCI within the university:

I understand also that this is a pilot, so the next thing is a roll-out to other academic departments.

A female lecturer said that HCI could now be expanded more easily beyond the areas chosen for the pilot project because the groundwork had already been done:

What is worth mentioning, is that we had a faculty that was not part of the pilot, but they got so interested, they were talking to us all the time – the Faculty of Management and Commerce. If it expands to all other faculties, I think they will find it

easier, because they will not have to deal with the same starting challenges that we have had.

Others agreed with her:

There is also a fit of HIV with other subjects, like Economics and Development Studies, where the link is not so obvious. We want to expand the opportunities to bring it into the curriculum in indirect ways.

There was unanimous agreement about the importance on following through with research. One comment was:

Our key focus is now on our research activities coming out of HCI.

And yet another HCI team member argued:

We need more publishable research. Developing scholarship is also important for us.

Conclusion

The focus group discussion with staff members involved in HCI at the University of Fort Hare was truly enlightening. Despite the institution facing ongoing challenges in terms of the having limited resources and lecturers being required to teach students with widely differing levels of knowledge and a broad range of capacities for learning, the HCI team appeared to be unfazed. Team members were not only passionate about HCI, but they were also willing to embrace myriad opportunities that HCI had opened up for them – for academic self-development, for more participative pedagogy, to engage in research, and for more meaningful engagement with students. Ultimately, they could contribute significantly to Fort Hare producing high-calibre graduates with the skills and empathy required to contribute meaningfully to the building of a more just society. The stories told by the HCI team members were inspiring and moving. There is definitely much at Fort Hare that is worth sharing with institutions involved in higher education who wish to engage in HIV curriculum integration with true integrity.



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“As a designer, as an artist, one has a social responsibility.”

– University of Johannesburg FGD

Introduction – the institutional context

The University of Johannesburg (UJ) came into existence in 2005 as the result of a merger between Rand Afrikaans University (RAU), the Technikon Witwatersrand and the Soweto and East Rand campuses of Vista University. Prior to the 2005 merger, the Daveyton and Soweto campuses of what was previously Vista University had already been incorporated into RAU. UJ currently has nine faculties and more than 90 academic departments¹. Student enrolment in 2015 was reported to be 49 452, making it the fourth largest tertiary contact learning institution in South Africa.²

Initiating the HCI programme

When HEAIDS sent out a call for proposals for HIV curriculum integration (HCI) at higher education institutions, the Faculty of Art, Design and Architecture (FADA) at UJ undertook to develop a proposal for the University. The involvement of such a faculty in HCI was unusual, since HCI within higher education is most often undertaken in the health sciences, social science or the humanities. Nevertheless, seven of eight FADA departments subsequently chose to participate in the project, viz. Fashion Design, Interior Design, Jewellery Design, Multimedia Design, Visual Art, Graphic Design and Industrial Design.

FADA's proposal was based on Amartya Sen's theory of 'culture as a catalyst for social change' and Pierre Bourdieu's assertion that the cultural field is located 'within the social conditions of their production, circulation and consumption'.³

The proposal outlined the following principles for HCI within FADA:

- Each department within FADA would research and investigate ways in which HIV/AIDS education could be integrated into its undergraduate curriculum and, in some cases, into the BTech curriculum
- Curriculum integration would be linked to art and design methodologies which included social justice, visual thinking strategies, discipline-based art education and design thinking strategies
- Each project would have a community engagement strategy to inculcate social awareness about sexuality, gender inequality and HIV/AIDS, and
- The Visual Art Department proposal envisaged collaboration with internal and external project partners⁴ to explore how art and design methodologies could contribute to active citizenship. Art and design outcomes would also contribute to the *Make a Difference* campaign run by UJ's Institutional Office for HIV and AIDS (IOHA).

Given this overarching framework, the FADA HCI project aimed to:

¹ https://en.wikipedia.org/wiki/University_of_Johannesburg

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

³ Funding application submitted by FADA, University of Johannesburg, to HEAIDS, 2015.

Faculty of Art, Design and Architecture (FADA) – examples of innovation in HIV curriculum integration within the project funded by HEAIDS

Department	Creative outcomes
Visual Art	Screenprint posters on HIV/AIDS awareness. Photovoice. Paper prayers, Mural painting. TVET and Wits collaboration.
Graphic Design	Advertising. Poster design utilising digital technology.
Multimedia Design	Video and performance presentations. ASE collaboration.
Fashion Design	Garments that reflect women's empowerment and the window period of HIV infection.
Jewellery Design and Manufacture	Self-adornment based on IOHA themes. Relation of drug and substance abuse with HIV/AIDS. Gender and sexuality related to HIV/AIDS.
Industrial Design	Life-skills development for homeless minors. Community engagement.
Interior Design	Exhibition stand design / ambient spatial campaigns for HIV/AIDS awareness and promotion of IOHA services.

1. Evaluate the extent to which HIV/AIDS was included in current curricula of participating academic departments
2. Design and implement discipline-specific interventions which integrated HIV/AIDS education in the undergraduate/graduate curriculum in the participating departments
3. Use appropriate visual methodologies and design thinking strategies to engender critical thinking capabilities amongst students with regard to the HIV/AIDS pandemic
4. Inculcate awareness of HIV/AIDS and active citizenship skills amongst FADA students, other students and community members of participating external partners
5. Develop discipline-specific HIV/AIDS, human rights and gender advocacy campaign models which encompassed public implementation and activism, and
6. Foster the generalisability of this curriculum-integration model to other South African tertiary departments of art and design, or to relevant UJ academic departments.

The HCI project team thought that FADA enjoyed distinct advantages in terms of implementing HCI. An example given was that it was not an academic school of art and design, but instead had its origins instead in a technikon setting. Hence the lecturing staff were much more likely to teach about, and to place an emphasis on practical and pragmatic aspects of design, underscored by an equal emphasis on research in a more traditional academic learning environment.

Challenges

One of the HCI team members spoke about the initial difficulty of starting a conversation with students about HIV:

The first challenge is how do we speak about these things in a way that is not overt preaching and secondly, how do we find a visual language for what we want to say? A peer-oriented conversation, or an invitation to a conversation. It isn't easy to start these kinds of conversations.

“The strength of our HCI approach is that it is socially relevant, and that it also connects to students' future career paths.”

– University of Johannesburg FGD

4 These included Artist Proof Studio, a registered NGO and UJ Project Partner; the Tshulu Trust in rural HaMakuya, Limpopo; the Central Johannesburg (FET) College's Art and Design Department students; Wits University's Drama For Life Programme; students from the Association of Sound Engineering (a private registered media education college); the Sonke Gender Justice Network; Lefika la Phodiso (a registered Art Therapy Centre, Johannesburg); and graduate students from the Department of Sociology and the Health Science Faculty at UJ.

A younger academic also talked about her anxiety regarding initiating a dialogue with her students about HIV, and how she had broken the ice:

The best way I could think of to contribute meaningfully to this project was to speak openly and honestly. Not as an uppity white person, talking to a group of black students about safer sex practices. So I had a very chilled conversation, asking about what they did get up to on weekends, and over time we let that develop into what they knew and what they thought they knew about AIDS; a very informal discussion, rather than a lecture. And because it was jewellery ... I let my students come up with their own designs.

She then took the process further:

I also asked them to write an essay as part of the research component of the design process, about what HIV and AIDS meant to them in the context of their families and their social groups. And what they felt needed to be explained more. I got some fascinating essays back. One student, from the Congo, who spent some time in the US, was writing about the spread of HIV in prisons. Another student who sees himself as gender-fluid, spoke of the associated crises in that particular social circle.

The process of dialogue with her students eventually led to an important collaboration:

... I integrated it with another project, called the Thuthuka Project, a nationwide community outreach for jewellery schools; and as a result, there are students from less privileged parts of our country, who are going to see the results of this project, when they come to our awards evening ... it just lets the process spread a little further, and allows our students to show publicly, part of the work they have done – and mingle with industry.

Another lecturer spoke of the challenges of working with students in a rural setting, and designing an appropriate mural with messages related to HIV:

The idea is communication – not being able to find a poster [in such an area with important public health messages]. I took my fourth-year students to a small rural village called HaMakuya. The social workers said that because of the AIDS situation in

that village, young girls who were in child-headed households were being preyed-upon sexually by older men, because the only way that they could get food for their families was by trading sex ... It is a huge problem all over ... what came out of our visit was the need to have access to information – like the contact numbers of the AIDS and Child Helplines. So what we ended up doing was painting a mural on the main wall of the municipality building ... the students put all this information on the wall about HIV, rape, where to get help Interestingly, there wasn't a word for rape in Tshivenda, the language spoken in that area.

A member of the HCI team spoke about her wish to continue to have conversations with her students about HIV, but of her awareness of the necessity of maintaining appropriate boundaries:

I am keen to carry on having casual conversations in class with my students, like asking a questions such as: 'How was your weekend?' It can easily lead us on to discussions about sexual behaviour and risk-taking. In this way I hope that I can reinforce messaging about HIV prevention. But I am concerned that as a young inexperienced lecturer, through having such conversations I am not breaking appropriate boundaries between lecturer and student.

A number of team members spoke about some aspects of the HEAIDS grant having been frustrating for them. For example, one lecturer shared his reflections:

If I was just to have a little critique of the way that budgeting was handled, it was that there were specific budgetary categories that appeared and that we had to apply generically, irrespective of what the project was. What we found was, that it just took a lot of time to massage those details into place, because the different projects are very diverse ... Rather than the budgetary categories being broad enough to accommodate the very diverse running costs that we might have in projects. So if the budgetary and funding process could be more responsive to what we put on the table, it would solve a lot of the problems that we have had, and made the funding process less hard work than it should have been. It is fantastic that we get funding, but when it comes with such a big overhead, it is a bit worrisome.

“Our history of having evolved from a Technikon brings a pragmatic and practical dimension to our HCI work.”
– University of Johannesburg FGD

A colleague concurred:

But I must agree that the budgeting process and the funding implications do get in the way ... It does allow us to employ other people, to run the programme. That is useful. The way that the budget is handled is problematic though.

One of the hurdles in the current year had been changes in finance staff within the University:

We have had problems this year with our finance reporting. Over a period of 12 months, we have experienced attrition in the finance support role in that four finance business partners have come and gone.

Innovation

The FADA team related a series of accounts of creative endeavours with students that spoke of their capacity for innovation. For example, a lecturer spoke about a problem that he had given to his class to address:

This year I tried to include [HCI] in our third year – more than a poster, and appropriate to industrial design, and we thought of a condom carrier or wallet, an actual product that could be developed. Also design the point-of-sale display stand and marketing – i.e. signage or posters. The students had all their pre-conceived ideas broken down by the trainer. They loved the project. It was socially relevant, but also relevant to their future career paths. Very excited to think what other projects we can run, maybe we can partner with a condom distributor next time?

Another team member thought that collaboration was a particularly beneficial option for a subject like Multimedia:

... what we did way back in 2013,(but it needs funding), was to set up a huge collaboration between our Department and Drama for Life at Wits, and we ended up putting on a performance at the Hillbrow

Theatre – it was hugely successful. The problem for us as Multimedia is that a lot of it has a performative element, and we want to get students away from just sitting behind their computer screens.

A colleague agreed that collaboration with other organisations brought particular benefits to students:

We tried to work with NGOs that need visual material for their own education work in schools, for instance. If we can contribute to something that will have a life outside of the classroom project, it would be a little bit more meaningful in terms of impact.

One lecturer felt that opportunities for HIV curriculum integration had not yet been exhausted:

We brought HIV into Design, which is very conceptual, and also brought it into Construction, which is understanding drawings, technical services, so we also used the project in that subject. But there are also other subjects that we can also maybe integrate it into. Maybe the challenge will be greater in the technical subjects?

The team was unanimous that the notion of citizenship was an important consideration that influenced how they engaged with students. One lecturer expressed her thoughts in these words:

And as a designer, as an artist, you now have a social responsibility. In fact from an educational point of view, we are lucky to be existing at a time when there are all these social problems out there, because we feel that design can contribute meaningfully to social issues. We believe that, and it is what we try to instil in our students. And we already have a community engagement ... a working initiative ... that we need to contribute to every year anyway.

An interesting idea which was raised was about teaching problem-solving skills in the FADA courses that would have a much wider application:

We can probably all agree that Design is about problem solving, how to solve a puzzle. Many of my students are on feeding schemes, many on

“The best way I could think of to contribute meaningfully to this project was to speak to students openly and honestly about HIV.”

– University of Johannesburg FGD

bursaries, most are in university accommodation. Many are in Joburg for the first time ... what it became is about making good choices, not just in Design but also making good life choices. I tell them, don't leave that philosophy here, take it home with you, and use it!

Other lecturers agreed with this perspective.

Engaging with students

FADA team members admitted that there were sometimes occasions when students were reluctant to engage with HIV/AIDS or complained that they were bored with such issues. However, the team agreed, the solution was not to avoid talking about HIV, but to bring it up in novel ways:

The approach that we have taken in the Multimedia Department has been quite varied, from getting the students to be quite creative themselves, to somehow crack the AIDS fatigue, which we do find, when we broach the subject, there is literally eyes rolling. But we get students to engage indirectly, like asking, ‘What are the social, economic, structural drivers of HIV, that have landed us with the social problems surrounding the crisis?’

A useful entry point for talking about HIV, and indeed for introducing other social issues, was the notion of citizenship:

[HCI] is going to be much easier now, given what we have already done. Looking at new ways of getting students to interact with the subject of HIV. And I think we have a responsibility to continue developing the idea of citizenship development and

knowledge in our programmes. And this is a really good way of doing it.

One lecturer pointed out that every conversation with a student had a potential multiplier effect. He said:

You look for a peer-oriented conversation, or an invitation to a conversation. The hope is too that if you work with a group, there will be a ripple effect. And that they will have the confidence to tackle these issues going forward. So, for example, we find that if we work with the second-year students, then in the fourth year class they will tackle the issues of gender, sexuality, etc. going forward, in their independent work, without us asking for them to do it.

Research opportunities

A team member spoke of the link she had seen between engaging in HCI and conducting research:

When I think of this kind of collaboration I have been very keen to turn it into research. Because once the project becomes part of the research process, other types of funding become possible, it can lead to higher qualifications for students and for our staff, like a Masters, and I am very keen on that. And it can become a research driver.

Another academic recalled that collection of routine data in class had eventually led to writing a research paper which could be published:

We gathered data in the class, we recorded things, we let it grow as much as it could, but it almost grew quite large, are we going to document two years, or just one year of activity? Eventually we got a paper accepted for publication.

“We can't ignore the fact that there are social needs out there that we need to think about too in our teaching.”

– University of Johannesburg FGD

Where to next?

The FADA team members were keen to find commercial or industrial sources of funding for the HCI work, or perhaps even access other funding within the higher education sector, which could fund the resources needed to sustain their projects. One example given was:

... taking the design for the trendy ARV pills container for youth, and finding a way to go into production, develop a workable marketing strategy (including design of posters, other advertising media), and actually produce it and sell it. This would be highly meaningful to students, and would look good on their CVs – actual experience in design, production and marketing. It could also serve a social good. Conceptualisation of ideas isn't enough.

A lecturer believed strongly that connecting to real-world HIV/AIDS interventions was crucial if the work they did with students was to carry real value:

So how do we get it out into the world, and make a difference to the campaign around HIV? For instance, the South African AIDS Conference was one of the proposals to take some of what we had done and showcase it, but the system didn't really accommodate it very effectively at all ... As opposed to taking some of the incredibly innovative stuff we had done in the faculty, and using it elsewhere. Like I have an invitation, for example: the Social Work Department wants a workshop in January for all the social work students about using visual methodologies. Those kind of linkages could be a lot more effective.

A colleague argued that comprehensive training for FADA lecturers about HIV/AIDS was essential:

I think it is vitally important that we get accredited training, an official training in this stuff, it would be incredibly powerful for us to have, we need to look at medical aspects, and then maybe more socially orientated aspects. So that is something that would be incredibly valuable.

Many lecturers could find space for HIV in their curricula. However, this was not the case in every Department. For example, a lecturer in the Graphic Design Department said:



I am happy to give an occasional glance to HIV, often in an indirect way, but there is no time or space to provide a clear and obvious focus on it, as in jewellery design. The syllabus in graphic design is very tightly filled, so there isn't much flexibility, and so there is little opportunity to insert HIV except in passing.

Lecturers planned to continue with interventions that raised awareness of HIV in different subjects in different ways. Some thought that the work could easily continue, for example in jewellery design. Other departments that looked to engage in community outreach in rural areas would require committed and substantial funding. However, there was a strong desire to continue to find innovative and creative ways to include HIV in teaching, as well as to address other relevant South African social issues such as poverty, gender-based violence, homophobia and xenophobia.

Conclusion

The depth and complexity of FADA's engagement with HIV is truly astonishing. If there is any doubt about the boundless potential of HIV curriculum integration, then one need look no further than this faculty at the University of Johannesburg for evidence that the potential of integration has not yet been exhausted. The body of work presented at FADA in relation to HCI is of enormous and ground-breaking significance. It certainly bears testimony to the audacity and resolve of a group of academics who have been unafraid to push the boundaries of conventional teaching or to engage in awkward conversations if that meant awakening their students to the reality of HIV and of other crucial social issues. This faculty is determined to make a real difference. May the project go from strength to strength!

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“This project opened up our minds to some things we never thought existed. If we have societal problems, what can we do?”
– North-West University FGD

The institutional context

North-West University (NWU) was established in 2004 through the merger of two universities with very different histories: the previously Afrikaans-medium Potchefstroom University for Christian Higher Education and the University of the North-West. The staff and students of the Sebokeng Campus of Vista University were also incorporated, adding further to the richness of the institution's heritage. NWU is hence spread across two provinces.¹ In 2015 NWU had a total student enrolment of 64 070 students, making it the contact learning university in South Africa with the largest student enrolment.²

On 2 March 2017 a group of academics from all three campuses (Mafikeng, Vaal and Potchefstroom) were interviewed about their involvement in HIV curriculum integration (HCI), the challenges and rewards, what the experience had meant for them personally, and how they saw HCI fitting within the context of broader academic transformation.

Initiation of the programme for the HEI

In terms of NWU's HCI proposal, the overall objective of the HCI project was 'to research, develop, implement and evaluate best practices for the transformation of the curricula of higher education programmes across

faculties, so that they are relevant and responsive to the realities of living and working in the age of AIDS'.

The specific project aims were:

- To conduct an evaluation of the current curricular interventions to address HIV and AIDS (e.g. the Understanding the World modules)
- To conduct an empirical study to assess the current perceptions, competencies and experiences of participating faculty for working with graduates in the field of HIV & AIDS education
- To assist faculty to develop, implement and evaluate contextually relevant curriculum interventions for HIV & AIDS education that prepare graduates to better fulfil their roles as future professionals and citizens, and
- To assist faculty to develop, document and disseminate examples of the 'best practices' regarding HIV and AIDS curricular integration.

An action research approach was to be used to investigate, develop, implement and evaluate best practices for the transformation of the curricula of higher education programmes to make them more relevant and responsive to the realities of living and working in the age of AIDS.

The project hoped to be able to provide answers to the following questions:

¹ <https://www.nwu.ac.za>

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

- How do NWU faculty members think, feel and act in relation to the integration of HIV and AIDS into their teaching?
- What is currently being done in terms of the integration of HIV and AIDS into the curriculum at NWU?
- What more is needed to effectively prepare students to live and work in a world characterised by HIV and AIDS?
- How can HIV and AIDS be integrated meaningfully into specific university programmes?

The research was to be conducted within the following faculties³:

- Health Sciences (Potchefstroom and Vaal campuses)
- Education (Potchefstroom campus)
- Agriculture, Science and Technology (Mafikeng campus)
- Law (Potchefstroom campus), and
- Arts (Potchefstroom campus)

Challenges experienced

The HCI team leader said that for her the biggest problem had been basically:

... getting people on board. Because as it unfolded, there were some political ramifications, which made people withdraw from the project, not because they didn't want to be involved, but for other reasons. And also, it was difficult to get people from other campuses involved ... However, the actual people I have ended up working with in the project, I was happy with, that has gone very well.

A lecturer in the Health Sciences Faculty said that it had been difficult to retain academics in the HCI process, owing to competing work demands and the sheer volume of everyday academic output expected of them:

In the part of the project that we were involved in, between K., me and another colleague, due to her other workload she could not be involved anymore, then we had to go look for another colleague to replace her for a bit of support ... I actually extended

a bit on the project. I have a back-up team. It's difficult to keep people involved.

Another lecturer commented:

Many people have a teaching focus or a research focus, this is seen as an add-on. And academics are very overloaded, and serious about research. It is only seen as valid if you can produce an article.

One team member said that she had tried to address the issue of competing demands:

What maybe can help in future, and that is something I tried to do, is to find the alignment between the work that I do and the work in this Project.

Another academic commented:

I must admit, I feel a bit fragmented. And the project focus is not my main focus.

A lecturer from the Faculty of Agriculture, Science and Technology in Mafikeng shared her perspective:

The main challenge in Mafikeng has been membership of this task team, because when it was introduced ... it was perceived as a health issue. And people from Nursing were pushed forward. We belong to a Faculty that includes Agriculture, Science and Technology. So Nursing was seen as needing to take a lead on this, it was seen as their baby, they should take a lead on integration. And then we also had the Vice Rector (Teaching and Learning) from Mafikeng involved, but because of the work pressures, he fell out of the process.

A Potchefstroom colleague agreed that when she had been drawn in, she had shared the same limited perception, i.e. the belief that HIV was largely a health issue:

That is how I got here. I just got a message from the Dean to the Faculty, saying that I should be involved.

The Mafikeng colleague admitted:

But I don't blame them, because they looked at the title, they associated it with health issues, so it must

3 Initially the Faculty of Natural Sciences at Potchefstroom campus also agreed to participate, but later it withdrew from the project.

be the health people ... remember Nursing is the only Human Health Science (the other is Animal Science), so it was our baby. Only to realise, we should include HIV in all aspects of learning.

Even when HCI training was offered, staff were reluctant to attend:

The other thing is we realised the need for information about integration. So two workshops were organised. One was on participatory pedagogy, the other was on critical diversity literacy. However ... it was always nurses that came, instead of people that were targeted like Agriculture and what have you. And when we tried to involve people, they would give the excuse that they are busy with their research. They prioritised research more than teaching and learning.

Another lecturer argued that on the contrary:

If you are in a teaching position, you have a responsibility too to become aware of *how* to teach.

The lecturer from Mafikeng campus pointed out another challenge:

We have buy-in and support from top management, but when it gets lower down, for example, I tried to organise focus group discussions, to get an idea of people's attitudes – in the end I had to meet with them individually, because it was impossible to get a group of staff members all under the same roof ... But it is not only negative aspects that prevailed. We also learned a lot from this, as much as we thought we knew already a lot about HIV.

A sociology lecturer said that for her:

... the only challenge I had was what all academics have, which is the problem with time, and how to fit it in. It being an extra thing. But I think you managed it well [speaking to the project team leader], like scheduling meetings but keeping them short, and also insisting on these [research writing] workshops.

The HCI team leader said that although there had been some resistance to HCI, it was fortunately not universal:

It's great that we have done this. Because I have met so many people, that you can see, it's not the

whole of the campus that's negative towards HIV, it's just pockets of people. That gives you hope.

However, the increasing pressure on teaching in higher education made any form of innovation more and more difficult:

There is the context of education. Every year our classes are getting bigger How do you teach such big groups? The point is that makes it difficult for lecturers to think about curriculum innovation.

Enabling factors

Given the challenges – restructuring of the University, some institutional resistance to change, large classes and competing demands on lecturing staff – the HCI team was asked what had enabled their project to turn into such a remarkable success. The team members identified leadership as an important contributor to success, particularly the involvement of the HCI team leader:

The team leader is so very important. When someone is fired up and passionate about the work, it really drives the process.

Another enabling factor was identified as working through the Deputy Vice Chancellor (DVC) at each institution. The DVC at NWU was mentioned as particularly supportive about HCI:

So I think it was very clever of HEAIDS to get the DVCs involved, in one way, but in another way, because of the climate of higher education in the past year, where DVCs have been, HCI is nowhere on their agenda.

But our DVC has been brilliant. He hasn't even been able to come to a meeting for the last six months, whereas before, he was always there. He would stay for the whole meeting, and he was really involved. But he dropped back – and he had to – he was very busy with restructuring.

A colleague added her perspective:

I want to add something to the story of how we ended up in the project ... If you look at the structure of the University, how do they go about identifying people? I think it was a good filter-through exercise,

“HIV is a perfect springboard for so many other topics, from gender, to inequality, to cultural understandings of illness and disease. It provides a perfect complex case-study topic for discussing even stereotyping. It creates a platform for discussion.”
– North-West University FGD

to go through the Deans to identify who possible people could be. So it might seem negative, but in fact it turned out to be positive, using the structure. So back to a more positive experience, in terms of people wanting to get involved.

Whenever I had to go back to our lecturers involved in the module, or even in the Life Plan team, people were eager to get involved, and to reflect and to participate. But it is maybe the background we are coming from, because we are working in a team environment, and we are used to a collaborative, multi-stakeholder approach. So in that sense, it was very positive.

Engaging students

When asked about the issue of HIV/AIDS fatigue among students, a team member said:

Students at school are exposed to a lot of HIV, so the lecturers complain that when they bring up HIV in class, some of the students just shut down. They are not open to engaging in discussion.

I think the entrance point is empowering students to develop life skills, and to really open up the topic for discussion. So have the intention to include HIV, but use it as a vehicle for engagement with students, for including issues that are so pertinent to all our students. Learning skills is made more important, not acquiring an education.

Another lecturer commented about the critical diversity workshop that had been offered at NWU:

Regarding AIDS fatigue, I don't even think that the facilitator even mentioned HIV in his workshop. It is possible to create awareness of HIV without even using the word, depending on how creative you are. HIV is still there – in a case-study – but there is a way of introducing it as just one aspect – social and economic aspects of it.

Her colleague agreed:

It's not that the material has changed, it is that the way you process it or present it that has changed.

Unintended benefits

The team was asked about what unexpected benefits they had derived from being involved in HCI, and there was a broad range of responses. Benefits mentioned included learning qualitative research skills, discovering how to conduct evaluation, and working with an invigorating team outside the domain of health.

One lecturer said emphatically:

You learn that HIV is not just a medical issue. You begin to see the bigger picture.

HCI led to a different change in perspective for another academic on the team:

I learned to celebrate small successes. We had to go through a process, and if you look at the membership at the beginning, how few stayed right through ...

A more experienced team member mentioned that a workshop on participatory pedagogy provided through the HCI project had enabled a big shift in her thinking:

I have been teaching many, many years, and I thought I understood the process of education. But it really transformed my understanding of how do I get students to where I want them to go, in terms of their own personal learning. I just needed a bit of a nudge ... to really become involved in what is learning – as opposed to passing an exam.

A colleague added her thoughts:

Participatory pedagogy was a practical thing that really helped us in our situation. That was a need identified ... and in a teaching and learning

environment, also in community engagement, both the lecturers and the facilitators that came experienced it very positively.

Another academic agreed that her perception of what it meant to be an educator had changed substantially:

This teaches you how to get your students to do the work [of learning], it is not your job. Your planning is extremely important, but getting that the lecturer is not the teacher, but the facilitator. You get the learners to do the learning.

A comment was made about the impact of new technologies on teaching at a university:

Because now you have Google, you have all the knowledge available online. You don't have to provide all the information yourself. So much technology now. We also need to change what happens in the school classroom with teachers.

One lecturer spoke of her understanding of HIV having grown exponentially because of the HCI project:

So talking about the social aspects of HIV ... also in terms of stigma and discrimination, talking about the social aspects of HIV/AIDS – it has been a great learning experience for me. And ... there is so much more I have been capacitated to do.

Another lecturer alluded to how the HCI process had assisted her team with finding finance for evaluation of a community engagement programme, with knock-on effects:

And we are now revising that participant and facilitator manual, because you enabled us to go and upgrade it for community engagement. So that really helped. So it was a process within a bigger programme, which fitted with our aims and goals as well.

The team spoke warmly too about the critical diversity literacy training that they had attended and which had also developed as a result of the HCI project. A sociology lecturer said:

The critical diversity literacy training really set the tone for me – P. was such an excellent facilitator, I thought, that really got me thinking ... the social

aspects weren't new, that's what sociologists do, I suppose, race, gender, sexuality ... but it really challenged me to think creatively, not to just do the obvious, but try to think, how can I integrate HIV, or issues around HIV, creatively into the curriculum.

Another team member said that through critical diversity literacy:

We bring the conversation back to decolonisation, and the project has helped to facilitate that.

The benefits had extended beyond the academics themselves:

If I think of the responses of the community facilitators who also sat in, and the positive responses from them, just saying, this really equipped me to use the materials presented to me. How it just opened their minds, to working with lay people differently.

One lecturer thought that the impact of the process had been profound:

It is a paradigm shift of thinking more broadly than your own discipline.

Finally, what came through very strongly was how much the group members appreciated the opportunities provided for networking in a range of ways.

Opportunities for research

Appreciation was expressed for the particular contribution made by the team leader in facilitating opportunities for research:

The fact that you are a research professor, that was so helpful for me and still is. So, for example, writing up our little report, and you will say, 'Let's see if we can get this into an article.' So it helps so much, because, although I am not young, I am young in the academic world. Yes, it builds capacity and confidence to do this. And also that you are a professor in education, that was really helpful ... So this has really helped me get published. And in fact a paper I presented in Washington DC was also to do with curriculum development, integrating LGBT issues into health curricula. And that came out of this project – thinking about curricula and thinking about education.

“It has been a very unfortunate time, but it has also been a good time, because now we are talking about issues of decolonisation, and got the conversations going.”

– North-West University FGD

The team leader was pleased about the publications that had arisen from the HCI project:

I think for me one of good things is that we have had five journal articles out of this HCI work. We had a special edition of a journal, together with UCT, and although the whole journal wasn't devoted to HIV, about 50% of it was, and then other general articles. That was a good output for us as academics.

The advantages of publishing research findings were ongoing:

It benefits academic careers. And the University will benefit from it too, because it will now get additional subsidies because of those articles.

A colleague on the Potchefstroom campus said she had benefited in an unexpected way:

On that point for me personally ... although I published in my field, in relative higher impact journals, I never published an education article. The article I wrote for the journal gave me the confidence to now produce results for the other teaching projects that I have to write up. Because it is a different ball-game, so thank you for that!

A Mafikeng lecturer reflected on her experience:

How I benefited is that I never expected to get a research project out of it. We looked the experiences of education students regarding integration from fourth year to final year, and it showed that students need this integration, even though they get to learn about HIV elsewhere, but it is on a very minimal scale, still using the medical model. And students were saying that sometimes they had to deal with an HIV-positive child, or a child-headed household because of HIV, and they didn't know how to deal with these issues. This project even helped us to determine areas where we can allocate our students to do their research, and to do something relevant. And the research committee even felt that maybe we do need that inter-disciplinary approach to HIV, so that this person can teach the whole faculty about HIV.

HCI and broader educational transformation

The NWU team leader spoke of the link having been made clear between HIV curriculum integration and the issues of decolonisation:

I think that was a good thing. The workshops weren't just confined to the project, and we invited X, who dealt with HIV and how it could be brought into the decolonisation of the curriculum issue. And there were loads of people who weren't from this project. I love the young lecturers, they probably don't even know the impact that this project has had on them.

The process during the critical diversity literacy workshop had at times been uncomfortable, one lecturer admitted:

Something I can add that I experienced during the workshop. I don't want to say conflict in the room, but there was quite a few heavy discussions that happened in there, and if you looked closer, a bit of underlying conflict, but that was really good, well handled by the facilitator ... not getting into an exploding situation, but actually resolving the conflict, that was my experience.

The project leader wished that the conversational opportunities the project had created could continue:

It would be nice to keep this on as space where we can talk – about transformation and decolonisation – because these are African issues – how do people deal with homophobia, xenophobia, the right-wing shift in the developed world. This should be part of all induction programmes ...

Where to next?

The members of the HCI project team had a number of ideas about what the next logical steps could be. One lecturer looked forward to integration across other courses:

It would be good to see how other modules could develop integration.

For another team member, the next logical step was related to research:

In Agriculture we do action research, and it is a cyclical thing ... we could identify other areas where we need to take research. We have some tools which were developed during the workshop, and which we could apply to communities. They could be applied to identify community problems and possible solutions.

The project had unlocked multiple research options for one particular lecturer:

For us ... there are three possibilities – the Life Plan part, which we are anyway going to do irrespective of this, is to develop an evaluation tool that can measure the impact of what we have developed now, and so we can find out the impact of what we are doing now with communities. The second is to look at curriculum transformation and the impact on students. A third one is an initiative that I specifically need to drive now in Health Promotion that involves the health curriculum, whereby promotion of HIV prevention is one of the target areas.

The project had been a particularly creative experience, she said:

Yes, because in our unit we constantly have new research coming in, and ... it is now finding the links, how you can fit with other projects. I wouldn't have thought about it if I hadn't been involved in this project.

Another logical next step related to working with communities:

Our community-based research in education – it is not just about school education, it is also learning and education in communities ...

Thinking also involved a wider evaluation process being necessary:

The next step could be an evaluation of what HEAIDS has done in HCI and across higher education and learning – what has changed – and also the impact on communities. If there is a possibility of collaboration between HEAIDS and a team looking at the HIV part, it is a possibility.

One academic member speculated about the possibilities for working together expanding much further:

It would be interesting if it was possible to develop collaboration not just between disciplines, but between universities too.

A final thought was the centrality of critical diversity:

To me the vital thing is critical diversity literacy – and that should be part of all induction.

A colleague agreed:

Perhaps the induction process for new lecturers is the place to do this. Eventually the body of lecturers will change, as the old, more inflexible ones leave.

Conclusion

The focus group discussion with the HCI team at NWU was an invigorating and energising process. Leadership at this institution seems to have played a seminal role in driving HCI: both genuine support from institutional leadership and the unswerving determination and drive of a very canny team leader. The process seems to have had unforeseen ramifications, unlocking potential for not just transformation but also for new and exciting forms of research collaboration, not only across disciplines, but also between institutions. The team deserve accolades for their superlative achievement – this is a group of academics who have responded readily to the invitation to think 'out of the box' and seem to have found innovative ways to create community both within and outside the academic establishment. The HCI process at NWU leads one to believe that this is an eminent institution and a learning space that is well worth watching.

“So maybe this has started a quiet revolution.”

– North-West University FGD

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“Students were there with us in a space of equality ... They learned from us, and we learned from them. And it has been such a wonderful learning opportunity right through.”
– UNISA FGD

The institutional context

UNISA is the direct descendant of South Africa’s first university, the University of the Cape of Good Hope (UCGH), which was created in 1873 by an act of the Cape Colony’s Parliament. UCGH was modelled on the University of London, which provided no direct teaching, but set academic standards and examinations for associated ‘university colleges’¹.

After the unification of South Africa in 1910, the UCGH was mandated to oversee university colleges across the country. In 1916 it was renamed the University of South Africa (UNISA), and in 1918 it relocated to Pretoria, and became a dedicated examination centre for many of what are now South Africa’s major universities. In 1946, as the last of UNISA’s colleges moved to independence, UNISA began offering distance education in the correspondence mode. In 2004, the then UNISA incorporated and merged with several institutions, namely the Distance Education part of VUDEC and Technicon South Africa, making it truly comprehensive from offering non-formal courses, through vocational programmes right up to doctoral level. Since then, it has grown into one of the world’s largest distance learning institutions, with 337 944 students² in some 130 countries in 2015. It has eight Colleges (faculties) and a Graduate School



of Business Leadership. UNISA is the largest open distance learning institution in Africa and the oldest

¹ <https://www.UNISA.ac.za>

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

We have opened up new ways of thinking about our disciplines. Many people came to me and said, 'I think I can do this!'
– UNISA FGD

dedicated distance education university in the world, enrolling nearly one third of all South African students. Its vision is 'to be the African university shaping futures in service of humanity'. In its latest strategic documents the move to harnessing the affordances of ICTs in education has led to it's refocusing as an ODeL institution (Open Distance and e-Learning). The merger in 2004 brought a sound grounding to UNISA in terms of HIV and AIDS education in the form of a larger focus on Life Sciences, as well as the very successful Unit for Social and Behavioural Studies in HIV/AIDS, which was re-named the Tirisano Centre. The project manager for this programme is also the manager of Tirisano and his considerable skill in both project management and in the field of HIV and AIDS was one of the key success factors for the project. The Tirisano Centre offers non-formal courses in HIV/AIDS, and its hosting department, Sociology, offers formal programmes in Social and Behavioural Studies in HIV/AIDS. In 2005 UNISA participated in the project to mainstream HIV/AIDS which consolidated the focused offerings in HIV/AIDS, as well as adding a fundamental module in HIV/AIDS care to the BA General. HIV/AIDS was also already present in the basic Teacher Education programmes at UNISA. There was, therefore, a strong base present in UNISA on which the project could be built.

Initiation of the programme

When asked how the current HIV curriculum integration (HCI) process at UNISA started, one of the HCI team members responded:

A letter from HEAIDS went to the Vice Chancellor, and then the Vice Chancellor referred it to the Vice-Principal of Teaching and Learning, and she took it to the Sub-committee of Senate for Teaching and Learning, and we discussed there, in that committee ... There was good buy-in, and we had excellent support from the Vice Chancellor and Vice-Principals.

Another team member added:

People sometimes say UNISA is like an oil tanker, so it's sailing in a particular direction, and it's not agile and it can't change direction quickly. But what it also means is that there are a lot of systems in place for a lot of processes, and what we did was to insert ourselves into those existing systems.

What was key to the success of the project was knowing the HCI project fitted best:

We inserted ourselves into places where it actually worked ... our Department of Learning and Teaching Development was such a space, because through that we got reach right through the university. The other universities have such reach but I don't think other universities have the same kind of reach that UNISA has.

What was also helpful in the process was that UNISA had previously grappled with the issue of HCI, so there was a foundation on which to build:

From 2005 to 2006 we went through the initial training of 75 academics, and those people are still active in the programme ... There was also a vague strategy document to integrate HIV/AIDS, but the strategy wasn't implemented.

A female lecturer said:

What struck me when I went to one of our sister universities is that their focus was on where you would expect to find HIV. To me it's not integration, if you expect it to be there, like in Microbiology, Social Work, Psychology. So we decided deliberately not to go there, and we also deliberately decided not to focus on the free-standing HIV/AIDS module ... as had happened in the 2005 project.

Enabling factors

When asked what had enabled successful HCI at UNISA, one team member said:

We had a distinct advantage in that we came from an institutional perspective while the other universities selected either a college, a faculty or a department, so they started from a micro level, while we went very strategic ... that allowed us to start influencing a much bigger process here in UNISA ... It helped that we had pre-existing models of modules with elements of HIV in them.

Furthermore:

We had eight colleges at UNISA, and their executive deans could nominate champions. What helped us a lot was college based champions ... they help us take some these conversations further into their colleges into smaller spaces. And it's amazing because we are currently up to 61 members on this whole project. It's huge and it's growing and everywhere we go people want to become a part of it. And I think we have created a movement ... the momentum for the transformation.

Challenges experienced

Not all colleges found it easy to see the value of HIV integration. However, there were moments of inspiration. For example, in the School of Engineering:

The academics were actually made to think about it, and they realised that construction sites are one of the major flash points for HIV infection. To me that was very rewarding, when I saw that most of these engineers had been thinking about how to build their buildings and bridges, trained in a value-free way, and they learned that you can't only function like that. Not when you've got construction workers dying because of this thing.

When asked about resistance to integration, one team member said:

I always say South Africans like sports and they like winning. But there are two things we are losing at: reading – our ten-year-olds can't read properly – and HIV/AIDS, we are not winning there either. I think that if the resistance [to HCI] is too big, you need to just ignore those people, and there are enough people we can work with.

The administrative and reporting aspects of the HEAIDS grant had nevertheless been challenging:

HEAIDS had certain requirements in terms of reporting and accounting and then the university instituted a second layer, so we had to do one set of reporting, and then re-constitute it for HEAIDS ... it's a higher education bean-counting problem.

A major challenge was that despite being on the HCI team, HCI was only a small proportion of some academics' job descriptions:

... this project isn't even 2% of my work, so I had to deliberately create space to at least be at some of the dialogue sessions with students, for me that was the biggest problem, and then the frustration of sitting in management meetings! But the reward was meeting students and really being humbled by what they had to say about HIV.

Teamwork and a driving passion

A male member of the HCI team commented:

I joined the group at a very late stage ... I was so humbled and surprised by the passion and the enthusiasm and the participation of the different groups in HCI. It was amazing. It's contagious, I feel the necessity of this project, this project is a big thing and it's not just about being involved out of a sense of obligation.

A colleague agreed about the team spirit:

We get many of these contagious comments!

He added:

We are honestly a little worried about how fast we are growing because the capacity to manage this effectively is still a challenge ... the project leader is also a Deputy-Dean, I'm also an academic. So sometimes there's conflict in that space, you want to give more time to the project but there's other competing obligations that come in ... the capacity to organise this thing must be strategic. Our Vice-Principal helps us create that capacity to do that.

“Even months later I would bump into colleagues and they would say how it opened their eyes, talking to students ... UNISA, with distance learning, had the most student participation, I think we’re very proud of that.”
– UNISA FGD

The team were nevertheless proud of how academics had responded:

We have opened up new ways of thinking about our disciplines. Many people came to me and said, ‘I think I can do this!’ I really can’t recall any negative responses, people are really latching onto it nicely and the question is just: how do we sustain it so that the momentum is kept up?

Engaging with students

An important component of UNISA’s HCI project was engagement with students:

When we started talking to students in face-to-face classes and asked them about HIV we saw that there was a sort of trauma or discomfort, we could work with it, or immediately refer the student on ... We are conscious of the fact that we may be opening wounds that the students will have to deal with by themselves, and that’s why we felt it necessary to educate our academics about being sensitive. It’s all very well integrating HIV, but you can be causing harm. So that’s why we flagged colleagues to be sensitive to students who need support because they are affected by HIV.

There was an awareness right from the beginning of the importance of involving students in the process:

Here at UNISA we have a longstanding relationship with the Students’ Representative Council (SRC) and with Student Health and Wellness. We felt very strongly that we couldn’t do anything unless students were there with us in a space of equality ... They learned from us, and we learned from them. And it has been such a wonderful learning opportunity right through.

A female colleague commented:

Even months later I would bump into colleagues and they would say how it opened their eyes, talking to

students. And I think it’s an irony that UNISA, with distance learning, had the most student participation, I think we’re very proud of that.

That’s one of the unexpected spin-offs ... having the student voice at UNISA really improves what you do, and I say this often ... academics underestimate the contribution their students can make, and we have evidence now, which I am using in our curriculum transformation project.

Students who were present for the focus group discussion expressed appreciation for being drawn into HCI. One student shared his perspective:

I was surprised to be asked to participate because we don’t usually know how study materials come about ... This has helped us not just reading the materials to pass, but also to enrich ourselves. You can’t read the modules without getting something to take back home ...

The HCI project had encouraged the team to organise academic-student dialogue sessions:

We tried to create spaces for student dialogues, where we invited academics and said, ‘Please come and talk to students and hear about their needs’.

The experience had led the academics on the team to appreciate the real value of these interactions:

There were really nice conversations that took academic staff to new levels of understanding. So it’s very important that our champions report back to their departments about what happened. For the future we definitely need a space where academics can come and have more dialogues with the students.

The HCI team was struck by the fact that despite being a distance learning institution, UNISA was able to generate great interest from students in attending dialogue sessions:

“I was so humbled and surprised by the passion and the enthusiasm and the participation of the different groups in HIC curriculum integration. It was amazing. It’s contagious!”

UNISA FGD

... in one dialogue, we had sixty students, so that shows you the interest!

Orientation, in a practical way, so it really is useful to have those skills taught up front.

A team member pondered on the meaning of this:

I think there is a deeper level of commitment with distance learning students. Because of the nature of the learning there has to be a higher level of motivation to do it. They also want a real university experience. They want that interaction and to feel that they belong to the University ...

An opportunity had been provided by a process that was underway, and which involved rethinking the teacher qualification curriculum:

The initial teacher qualifications have to be redone and it's a huge and long-term process ... all of the BEd degrees will include language modules ... we can make sure that we integrate HIV into the language modules because ... we heard from students that the language around HIV/AIDS is very important. So we are using what we heard from our students to infuse the language modules, and those are compulsory modules, so all education students will take them.

He added:

Everywhere the student body is being seen the key for sustainability. Through this project the students see that there is an openness – that the University really wants to hear them, and wants them to contribute also to our processes.

The UNISA approach to HCI had been unique in that it had also involved the development of ethical guidelines for HCI:

We didn't realise that this institutional approach would be different to other universities. What we knew from the start is that we are a distance learning institution, we tried to make a niche contribution and some of our objectives speak directly to that.

A female colleague concluded:

Real interaction with students was the most precious thing for me, and every time I get so humbled by the quality of the leadership, the humility and the engagement.

A female academic spoke of her simultaneous involvement with the Human Sciences curriculum transformation process:

I'm responsible, with another team, for writing up our College of Human Sciences curriculum transformation framework ... And as I work with academics on this framework I use my experiences about the student voice, and it changes my perspective. So we've linking the HIV integration with existing processes and structures. There's a kind of natural synergy.

Innovation

In terms of HCI, the UNISA team was particularly proud of the work that had been done in teacher education:

We are responsible for training teachers all around the country and by including HIV/AIDS in our curriculum, we are not just preparing our students, but we are also empowering them with the responsibility to help the community. I think we have a huge responsibility and it came at the right moment ...

A colleague agreed:

It's certainly a thing teachers at schools often struggle with, how to manage HIV, especially in Life

The HCI project had led some academic staff to ask new questions:

You see academics asking themselves what it means to be a change agent rather than just being a lecturer with a distance between you and the student, or you and society ... for the first time they start to understand these processes.

I think this is also a huge gap that the university has, that we do not empower students to understand our processes.

A further insight was how HCI could reflect UNISA's stated mission:

Our mission is to be the African university sharing futures in the service of humanity. So we have a very strong community focus and that came through in the project ... And students' eagerness has given me the idea to push service learning even more. Because of our distance learning nature, it will be very difficult to manage. But we need to be more creative, and it's made me more courageous to push service learning.

Service learning involving HCI could take a range of forms, such as working with an NGO that focused on HIV.

An important step had been including HIV in some of UNISA's signature modules:

Each one of the undergraduate degrees at UNISA has what we call a signature module. And each college has their own signature module. The idea of the signature is that it relates to gradueness in the sense that it is meant to be values based. For example the one in economic and management sciences is called sustainability and greed, so it's intended to make students aware that sustainability is what it's about, because often students in those kind of courses are just in it for the money. And we want to build this values-driven gradueness into the signature module.

The other option that UNISA's HCI project could take was to integrate HIV into open education resources:

Open education resources means providing something that people use in different spaces, but also to design it from a very neutral point of view, so that each discipline can customise what they want to use it for ... a success of the project that at the end we give something that is a spin-off for the whole

University, that they can use it where ever they want to in their own discipline context ... The second open education resource is on how to develop a campaign for your community on social justice and HIV. These tools will be available not only from an academic side, we are also going to upload it into the open space for the students ... We are going to soon make it open license, so people can use it or replicate it, so that it has also a social mandate. And that's the beautiful part of using the institutional approach: we can use these things to create awareness beyond the University ...

UNISA was also engaged in assisting the TVET colleges with regard to HCI:

The Minister of Higher Education is imploring the universities to assist the TVET colleges in terms of upskilling and we have at UNISA a memorandum of understanding with the TVET colleges.

Other South African universities were coming to appreciate the importance of online distance learning (ODL), including with HIV integration, and UNISA had made a head-start in this regard:

They really need us to take their projects to the institutional strategic level. So UNISA comes with: 'We can help them with curriculum development processes, not only integration but also going into the ODL environment'. And then suddenly we get this huge exposure in national meetings when we present what we have done and other universities say, 'Please come and help us!' One thing we do insist on when we get involved is hearing the voices of their students. And some academics at other universities don't like that!

Students and wellness

The UNISA team had thought it important to link their HCI work with the Student Health and Wellness Programme from the very beginning:

We also see the Student Health and Wellness Programme as being almost like an external curriculum, so they have been in every way involved with what we do. We are now in a space where students can tell through the study materials that a student health and wellness programme exists, and Student

“The conversation was initiated with HIV, but it moves now to decoloniality spaces, it moves into transformation spaces and into new pedagogies.”

– UNISA FGD

Health and Wellness can share with students what modules have elements of HIV in them.

A big advantage of this link was the support that the Student Health and Wellness Programme could provide:

So then we can really market them as able to provide support services at a distance, because many of our students might be at home learning about HIV and need psycho-social support.

One HCI team member spoke about how HIV-positive students disclosed their status to him:

I have had a lot of students who have disclosed to me. So if you create a safe space in what you teach, you afford people that opportunity to disclose. Almost every three weeks I come across such kind of a situation ... Some of the students aren't infected but they are affected ... we do need to create safe spaces. The knowledge that they get from some of the dialogues, they are able to take back to their community, and to family level, and try to reduce the stigma. Sometimes I think that it's a privilege not many people have.

HCI and broader educational transformation

The HCI team spoke about the fact that some academics – and indeed, some members of the broad South African population – were still in denial about the impact of the HIV epidemic. A female lecturer said:

I call it the bubble, and if I can use the de-colonial term, it's to do with whiteness, people live in their little security complexes and don't engage with reality ... And when we say whiteness in decoloniality, black people can have a whiteness too. I think people just don't want to engage because they protect themselves in various ways ... they try and stay in this bubble which is not reality.

Another HCI team member agreed:

I think that's why we decided HIV is an entry point for us to address two other issues. The one is gender, and the other one is the decoloniality conversation. So we didn't really focus only on HIV.

From the very start we made it clear that HIV was an entry point to talk about intersectionality. So how does it intersect with class, Africanisation, gender? All of this adds value. So I think that made it so we could make the HIV conversation real and relevant today. That we had to be innovative ... breaking some of these barriers where people say, 'It has nothing to do with me'.

The team could see an organic connectivity between many of these issues:

The conversation was initiated with HIV, but it moves now to decoloniality spaces, it moves into transformation spaces and into new pedagogies, so I think there are so many spin-offs from this project that we don't yet know what the consequences will be. But I think we have a good basis to being with.

Where to next?

One of the team members said that he was relieved that the perception that teaching about HIV belonged largely with the Humanities had been shifted:

What stands out for me is the eradication of the perception that HIV only belongs to the Humanities, that's all. Getting involved in this project, we realised it's moving to a social justice aspect, which permeates developed sectors of our industries, like accounting, engineering, management. It can be an eye opener that all of us are affected by HIV/AIDS, and if we don't engage with it, we will be lost.

He added:

... even if basic education doesn't always get it right to do proper educating about HIV, it is nice that we

can still provide that opportunity for our students to learn about HIV through UNISA.

Another team member argued that HIV integration still had a long way to go:

The dialogue about HIV/AIDS cannot stop, we cannot reach a level where we say we have done enough. It's an ongoing dialogue, it goes from basic to advanced level, and even from advanced back to basic because there are young people who are still growing up, not exposed to knowledge about HIV and AIDS ... this conversation, it has to reach everyone if it is possible.

New issues relating to HIV emerged each year:

In fact the field is changing ... nowadays people are talking about the challenges of living with HIV and aging. And mental health issues associated with living with HIV, which are important to address because they affect people's adherence to treatment. It isn't always about the same old HIV101, which everybody is so sick of.

However, the next logical phase of the HCI project appeared to be:

... capturing all the case studies of curriculum integration across UNISA, and we have now got what appears to be quite a big research project. And it ends up with an impact analysis, so after two years we will be able to look at the impact of some of these modules on the students.

Champions of integration also had an important role to play:

We need the champions in the colleges to strengthen HIV integration through our curriculum development structures. In those spaces we will also be able to pick up more case-studies because of curriculum integration, so they can become the main drivers for further implementation ...

Finally:

We have created an entry point to start talking about other areas of curriculum transformation. I'm confident that in many spaces we are busy institutionalising our project, we will get the Vice-Principal to talk to her planning process so that she can also influence the process to create an enabling environment.

Conclusion

UNISA differs from most other South African institutions of higher education, both in terms of context and in terms of its range of resources. It is nevertheless astonishing that in so many ways UNISA has taken the lead in terms of benchmarking HIV curriculum integration. Part of its success can be attributed to the fact that the current HCI initiative builds on previous and substantial efforts at HIV integration. A further contributory factor is undoubtedly the full endorsement of the process by top leadership at UNISA. The engagement of students in the HCI process is worth emulating, and is nothing short of astounding, given the particular challenges of distance education. However, what stands out most clearly is the enthusiasm, passion and vision of the team which has led the process. This team has done a remarkable job, and deserves every accolade.

UNIVERSITY OF VENDA

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University of Venda

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Introduction – the institutional context

The University of Venda (Univen) is situated in Thohoyandou, in the scenic Vhembe district of Limpopo¹. Univen was founded in 1982 in the era of apartheid, with the initial objective of provide a tertiary education to TshiVenda-speaking people. Despite its origins, from its earliest years the university has drawn students from a wide range of communities, and it has recruited academic staff across South Africa and internationally. The University's vision is 'to be at the centre of tertiary education for rural and regional development in Southern Africa'. Univen has eight faculties (known as Schools, Health Sciences, Education and Human and Social Sciences), which in 2015 had a total enrolment of 14 146 students².

The large number of individuals who were present at the focus group discussion and who contributed to the conversation with great enthusiasm was testimony to the real support for HIV integration at the university.

Initiating the HCI project

The HCI co-ordinator related how the process began:

The project started in 2014, when we began by forming a team. When we wrote the proposal we thought at first we would involve everyone at the university. Then we were advised the funding would be insufficient, so we identified three schools: the School of Health Sciences, the School of Human and Social Sciences, and the School of Education, on which to



The initial UNIVEN HEAIDS Committee members at workshop in Karibu Lodge, Tzaneen on the 07th–09th June 2015.

focus our efforts ... So we started with students and staff members from these three schools.

The next step was carrying out an environmental scan in each of the three schools:

... to find out from each school, what are they teaching on HIV, how are they assessing it, how many credits it carries, everything about HIV and AIDS, because we are integrating HIV into our curriculum, we needed to know if there was already something in place.

The team found a range of integrated courses already in existence:

In Education at fourth-year level, there was a stand-alone module, entirely about HIV and AIDS ... In the School of Human and Social Sciences there was a compulsory, core course for English and Communication Skills that had been chosen as a course for HIV and AIDS infusion. All first year

¹ https://en.wikipedia.org/wiki/University_of_Venda; <http://www.univen.ac.za/index.php?Entity=History%20of%20Univen>

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

students within the University took this course. So infusion had happened through the setting of essay topics, essays about different aspects of HIV, so students had to seek out information, for example, through doctors or in the library.

Although students in the School of Human and Social Sciences who had taken the compulsory infusion course provided feedback that it was 'informative' and 'useful', lecturers felt that over time the course had become tired and needed revitalising:

As the years went by, we can't keep on reproducing the same thing ... asking students to write an essay, so we needed to find other interesting ways of infusing HIV into the curriculum.

Another finding of the Univen environmental scan was that although the original intention had been to integrate HIV into courses in each year of study in Human and Social Sciences, the process had never got further than first-year level.

With regard to the School of Health Sciences, the HCI co-ordinator said that, as had been expected, her team had found evidence of a high level of HIV integration:

With Health Sciences, there were no problems. In all our courses, all our modules and departments within the School of Health Sciences at all levels, HIV/AIDS was taught in those courses.

Choosing an approach for integration and securing buy-in

The team's next task was to explore whether or not there should be a standard approach to HIV curriculum integration throughout Univen:

We met and discussed which approach can we use throughout the university, but still we found that within the three schools there were differences [of opinion], we could not just come up with one model. It needed for us to check again, what the needs of each specific school were.

The preference was for

... the infusion model because the stand-alone approach was going to be more problematic because it may increase the credits... and we also had to find a way of assessing specific needs.

This meant that having completed the environmental scan in the three Schools:

What is still outstanding is to have the other Schools at Univen involved³. With the other Schools, some are doing HIV integration, some not, but we don't know to what extent it is happening.

Although the HCI team wasn't knowledgeable about HCI in all the five remaining schools, there was a perception that HIV was sometimes taught, but usually only at higher levels of study.

Although progress had been slow, the HCI team was satisfied that there was now

... a draft framework for HCI. The framework will elaborate on how to integrate HIV. At later stages we integrate the HIV questions into the course materials. We must then present to all the Schools, to get their buy-in. We hope it can be taken through the whole university and implemented⁴.

Another team member commented that some buy-in issues had already been addressed. At the start of the current HCI project funded by HEAIDS, she said:

We did have the buy-in of top management, and we also got the buy-in of the students, through the Students' Representative Council of that time, and we are working with peer educators.

It is important to mention that the Univen HCI team had enlisted the skills and time of a postgraduate student to gather other useful information about HCI on its behalf:

And maybe just to mention, we have recruited a PhD student to work on this project. Because we know there are some aspects we didn't touch. What

3 These were the Schools of Agriculture, Environmental Sciences, Law, Management Sciences, and Mathematical and Natural Sciences.

4 Extending HCI in order to include the additional five Schools at Univen (i.e. implementation of HCI throughout the University) fell outside the ambit of the HEAIDS project funding.

“We have had a conversion of the not-so-convinced-at-first slowly changing their minds, or at least warming towards the idea of HIV curriculum integration.”

– University of Venda FGD

are your questions, specifically [addressing the PhD student]?

The student responded:

I want to see what is the picture across the campus with regard to HCI, also look at the attitudes of staff and of students, and whether the current framework is applicable to all the other Schools at Univen.

Involving academic staff

A big challenge at Univen was finding ways to draw more academic staff into the HCI process. A major hindrance was the need for more skills development:

We need to conduct capacity building to all the lecturers [in the three specified Schools], so that they have the skills.

HEAIDS had facilitated a workshop for academic staff at Univen regarding infusion. However, now quite a number of new lecturers have joined, who haven't gone through that training, so my feeling is that we need to make sure of their ability to infuse HIV into their teaching. The challenge that remains is to keep on doing capacity building and to keep it fresh.

The issue of training was particularly acute with regard to lecturers in the School of Education, the team said.

Responses to HCI varied a great deal across Univen. Some academics were supportive to different degrees but there were also decidedly negative responses. A lecturer in Health Sciences confirmed this:

We have had different kinds of response to HCI. Some staff were very enthusiastic, others liked the idea, but were not very keen to put any time or effort into doing it, while others said, 'No, I was employed as a lecturer, I am not here to teach about HIV'.

However, judging from the turnout for the focus group discussion, there were a good number of academics

at Univen who were enthused about HCI and committed to enhancing implementation at the institution. As one academic put it:

Those who are in it are really giving it their best shot.

Identified challenges

In addition to ambivalent or negative responses from some lecturing staff, there were other challenges in implementing HCI at Univen. The HCI co-ordinator said that the administrative and financial implications of accepting a grant such as this were onerous:

When you are told, you have been given the grant, and the money will come in tranches, and you must deliver on this and this, it is very difficult, because you must remember, this is not my core responsibility, I must do this in addition to my normal job. Now I must spend nearly the whole day communicating, calling people together.

A further challenge was that not all academic posts at Univen were filled. This constrained HCI implementation at Univen, since lecturers sometimes felt burdened by heavy loads of teaching and consequently burned out, which fuelled their reluctance to take on new challenges such as HCI.

Another difficulty, an HCI team member said, was the uneven quality of information and communications technology (ICT) at Univen:

Access to the internet is a problem here. The connection is also slow when we need to use the internet to find new information. It is also very poor for our students. So developing new training materials or doing up-to-date research is difficult for us with such limited internet.

Building a comprehensive alliance

HCI does not in itself require the direct involvement of campus health services or of peer educators.

“I even give students my personal number – so they can even phone me after hours if they need to talk about HIV.”

– Nurse, Campus Health Services, University of Venda FGD

However, as part of a broad strategy to reduce the impact of HIV/AIDS on the student population, forming a comprehensive alliance with all role-players at an academic institution is an excellent approach. The HCI team at Univen has developed a strong and positive working relationship with campus health staff and with student peer educators, as shown by the presence of representatives of both these groups for the focus group discussion. A representative of Univen's Campus Health Services spoke during the focus group discussion about the challenges she faced in providing HIV testing, care and treatment to students:

Male students are very resistant to the idea of HIV testing. But we go to the residences to have group discussions about HIV. Then we get better results, and male students come forward for testing. We also have incentives: juice bottles, pens, key holders and sometimes T-shirts.

She felt that it was important not to be discouraged when the initial student response was restrained, particularly that of male students:

Now you mustn't stop talking about it because it is a taboo. When you keep singing a song, people get used to it, and they even start singing the very same song themselves. Especially with men, only two or three would come [to HIV awareness events or testing sessions]. But you don't stop sharing. So we would go to the residences, and say, 'Don't worry, just enter, it is free, just come in your pyjamas'. And if you can get five people, you have won a crowd,



from there they are going to share the information with the others.

In her experience, perseverance produced results in the long term:

And the other thing is that since we started 'Test and Treat' ... students are realising, what more can I do if I am positive? They are developing an interest in knowing more, people are taking this issue seriously. They ask: 'What can I do to have more life?' But the issue of confidentiality – fear that we will tell their families [about their HIV status]. So they often give us the wrong contact numbers. When they come back they will tell you, I was afraid you would tell my aunt, you would tell my mother. Keep on reassuring about confidentiality is what I say.

Furthermore, Campus Health Services was seeing improved incomes in terms of HIV treatment and adherence in comparison with previous years:

Also not being adherent on treatment. Being scared that people will find out they are HIV positive. But we keep on repeating the information, no, it will be confidential ... We also run a monthly support group for students who are HIV positive, and since 2013 we have 18 [HIV-positive students] who have graduated.

The question was asked whether HCI had impacted positively on the number of student utilising Campus Health Services:

The students are most definitely gaining more information about HIV. I think usage of Campus Health Services has increased, and more students are coming for testing. Utilisation of services is going up and up.

A peer educator present for the focus group discussion said that students were taking HIV testing more seriously than in years gone by:

Students are now understanding the importance of their health. Last year we didn't have incentives for

testing, and we still had students coming in large numbers. What I now understand is that they realise the importance of their health, the importance of getting something out of testing.

Another peer educator agreed that student behaviour had changed for the better:

To add on to what you are saying: last year when the students saw us carrying condoms, they ran away from us, they even called us names. But now, when we move around with condoms, they even fight to get condoms from us. Now they are running to us to say, 'We want to graduate alive!' We are seen as life-savers. Unlike last year, when it was different ... Even the SRC is working with us.

Responses of students

One of the HCI team members spoke of her perceptions of students' increased interest in HIV:

The student's enthusiasm for me was interesting. When they get here, you think they have been learning about this in high school ... they get it from the radio, from the TV ... But then when they are given this topic to research, they show such interest and enthusiasm, even going to local doctors to get information, I found that interesting. Because I thought they would say, 'Ag, and just rush through it, brush it off.' But the effort they have taken is exciting for us.

She felt that the way to increase student engagement was to be innovative in the development of HIV curriculum integration:

The challenge is to find new ways of presenting the information, and new ways of setting assignments. That keeps the interest and motivation of the students.

Another team member had also seen students responding well to learning about HIV:

In fact we find that some students do take HIV issues very seriously. They want to be better informed about this disease. Many are touched by HIV, because they also have family members who are living with HIV. They want to know how to protect themselves, and others, how to live a long life even with HIV.

Enabling factors

The HCI team was asked what they considered to be factors that had been helpful in developing HCI. The HCI co-ordinator responded:

I thought to myself, we can't leave out Campus Health, we can't leave out the students, so I did include them.

A team member agreed:

One piece of advice you can get from Univen is that you can't do this alone. Even as a committee, you need the buy-in from students, you need the support of top management, you need Campus Health, you need as many people as possible to make it work. But the big question then is: how do you do that?

For some individuals, the story of true collaboration was what made the difference and what they wanted to emphasise the most:

With me, I think the teamwork is so wonderful. There is teamwork from Univen. Committee members respond in numbers, the teamwork shows the support we are having with the project.

The Centre for Higher Education Teaching and Learning (CHETL) at Univen was also mentioned as having made a significant contribution to the HCI process. CHETL is a centre based on the Univen campus that focuses on disability, student counselling and support, as well as academic development:

"You can't do this alone ... you need the buy-in of students, top management, campus health, as many people as possible – that is what makes it work."

– University of Venda FGD

It has been responsible for training staff members about the approaches to integration. The tutors from CHETYL are passionate about HIV.

The crucial role of the top institutional leadership's commitment to HCI was highlighted by another individual as an enabling factor:

The Principal of our university is very, very strong on this one [i.e. HCI integration]. Especially if the top guy is for it, it makes a difference.

A colleague concurred with this view:

What helped was the involvement of management up there at the top – because it was easier then to form the team from the Schools, and even the students were involved.

Another comment made was that it was important to see HCI as a long-term process, and not to be disappointed if in the short term it was hard to see results:

We have been pulling together since seven years ago. The students, they finish and go, they pick up the relevant information. But the lecturers carry on ... we must not give up on the struggle.

Another enabling factor for Univen team members being able to visit other higher education institutions and getting to hear first-hand about their work in HCI, their successes and most important, their best practices. In particular, one team member recalled a visit to the University of Cape Town to attend a workshop where experiences of HCI were shared:

When we meet with other institutions, that is when we can learn from each other, how some of the approaches they have used are working for them, some of what we are doing we have learned from other universities.

A final enabling factor for HCI, the team acknowledged, was that Univen had been less disrupted by *#FeesMustFall* student protests than had been the case with other universities.

HCI and broader educational transformation

The HCI team at Univen saw a connection between HCI and the challenges of broader transformation in the higher education sector:

Currently there is this transformation process and this decolonising the curriculum going on at universities. And it can also help to stimulate the mind-set of academics. Because still, behind these issues, HIV is also inside those issues. Always HIV and AIDS is there. So we say, let's talk about transformation, it is a way to engage in these conversations.

Next steps

The HCI team was asked what they saw as the next steps that Univen needed to take. One view was that after completing the pilot project of HCI in three Schools, implementing HCI across all the Schools at Univen would be the next priority:

We need to finalise all the other activities in our curriculum. I think we are almost there. First the three Schools and then moving on to all the Schools at Univen.

When asked about a further step, the HCI co-ordinator said:

We need to talk to the local TVETS, because HIV and the curriculum are also issues for them. Sharing what we know is important.

Conclusion

Although the HCI team at Univen expressed some frustrations about the process of developing and expanding HCI, their enthusiasm and passion for the task was evident throughout the focus group discussion. It was clear that this institution of higher education took the issue of HIV curriculum integration very seriously; indeed, its staff were willing to take a long-term perspective regarding the process.

“I think this process has broadened our horizons so much.” – University of Venda FGD

The presence in the focus group of representatives of Campus Health Services and student peer educators bore testimony to the healthy and friendly relationship between the academics present (who are involved in developing and implementing HCI), Campus Health Services (which provides health care—including HIV testing, care and treatment—to students) and student peer educators (who assist in educating students about all aspects of HIV). Each of the three groups has a role to play in addressing a key aspect of the

HIV/AIDS epidemic. However, it is crucial that confusion does not arise about their respective responsibilities or their roles.

Nevertheless, given the determination and sense of zeal expressed during the focus group discussion, there is no doubt that Univen can make a substantial and meaningful contribution to academic engagement with HIV, and that it can help end the HIV/AIDS epidemic in South Africa.

UNIVERSITY OF LIMPOPO



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Introduction – the institutional context

The University of Limpopo was established in 1959 as the University College of the North, with the objective of providing tertiary education to black students, particularly from the northern parts of South Africa. At that time black students were barred from attending universities established for other races owing to government policies of racial segregation. Until 1970 the University College fell under the University of South Africa, but in that year it became an independent university in its own right. Between 2005 and 2015 the University of the North incorporated the Medical University of Southern Africa, based near Pretoria, under the new name of the University of Limpopo. When the Medical University had its autonomy restored in 2015, the name University of Limpopo was retained by the larger institution. The University's campus nestles in the foothills of the Hwiti (Wolkberg range) in Mankweng, midway between Polokwane and Magoebaskloof, in Limpopo province¹.

The University currently provides tuition in four faculties:

- Health Sciences
- Humanities
- Science and Agriculture, and
- Management and Law.

In 2015 the University of Limpopo had a total student enrolment of 18 907.²

Initiating the HCI project

The objective of the HIV integration (HCI) project at

the University of Limpopo was defined very broadly as being 'to integrate HIV and AIDS into the curriculum for all qualifications offered at the University of Limpopo.

Integration was envisaged as needing to occur along four dimensions, as defined by Kelly (2000):

- Learning to know, including having comprehensive and accurate information about HIV;
- Learning to do, i.e. fostering the acquisition of psycho-social health and skills;
- Learning to be, i.e. being informed of HIV at an individual level, changing attitudes and values, and
- Learning to live together, which includes greater social acceptance of people living with HIV and reduction of HIV stigma.

Specifically, the project aimed to:

- Review all current academic programmes and qualifications at the University to see what HIV and AIDS contents they had in their curriculum;
- Develop content around HIV and AIDS from a disciplinary perspective;
- Survey stakeholders on their views of HIV and AIDS curriculum;
- Develop an education and training manual for various categories of staff; and
- Develop a multi-disciplinary curriculum for all students for use during student orientation.

Three possible models of curriculum integration were considered at the University of Limpopo:

- A compulsory HIV/AIDS 101 course for all students, aiming at individual behaviour change (i.e.

¹ <https://www.ul.ac.za>

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

to reduce behaviour that puts one at risk of becoming infected with HIV);

- Extramural engagement with students in leadership programmes aiming at social mobilisation of students; and
- Intellectual integration of HIV and AIDS into discipline-based teaching and research, regarded as a 'structural intervention'.

After extensive discussion, the third model of integration was selected as the appropriate standard for the University.

A multi-disciplinary committee was then established, comprising a representative of each of the four faculties of the University. Each representative was tasked with collecting calendars and course outlines for their faculty. The documents were then reviewed for any mention of HIV/AIDS³. This review was intended to provide an HCI baseline for the University of Limpopo. The initial finding was that three of the four faculties (the Faculty of Health Sciences, the Faculty of Science and Agriculture, and the Faculty of Humanities) had included HIV in at least one academic course.

Following the document review, a three-day workshop was held for all heads of department (HODs) at the University, to share and discuss curriculum contents (including whether or not HIV was included in their curriculum). The workshop was planned in terms of two outcomes: to identify departments that lacked

capacity to infuse HIV into their curricula, and to identify which courses already had HIV integrated into their content. In terms of the first outcome, 44 academic staff members were identified as needing capacitation to engage in HCI. Progress with regard to the second



outcome in the identification of all courses that had HIV and AIDS contents in the calendar of each school. However, it appears that not all academics who were present had comprehensive knowledge of the contents of the courses in their departments, and so there was a number of courses where it was unclear whether or not HIV had been included.

The existing HIV and AIDS manual used during Orientation of New Students were revised rather than developing a new one. This material is currently being used during First entering students' Orientation. Two standalone modules on HIV and AIDS already in use by the Faculty of Humanities were also reviewed and adopted for use by the Schools of Accountancy, Mathematical and Computer Sciences as well as other disciplines where infusion was problematic. HIV and AIDS was infused in the rest of the qualifications in all Faculties especially at the undergraduate level.

Challenges

A number of challenges were experienced early on in the process of HCI project implementation.

Taking even the first steps was tricky. As the project manager remembers it:

"It was hard to involve the staff members, especially Academics. Each faculty had to appoint a representative for curriculum integration, and we made sure they were involved in HIV integration. It was really a big struggle, to mobilise my colleagues, who,



3 As the University of Pretoria (UP) discovered from a similar review of academic calendars and faculty guides in the course of their HCI project, this is a simple but not necessarily entirely accurate way of identifying HIV/AIDS content. UP found that some courses included HIV, but this was not always mentioned in course outlines.

like from the law school, in one of our meetings, said, 'Why are you wasting our time with curriculum integration?' Some were not keen. Others were willing. Some had started integrating even before the project".

A challenge identified almost at the start was that not all academic departments saw HCI as necessary or as being of benefit to their students. Examples of lack of interest included Academics in the Department of Mathematics, the School of Accountancy, and the Faculty of Management and Law. According to the project leader:

Some of them said: 'We can't do this!' Their argument was: 'We count with numbers.'

Some Academics in the Faculty of Management and Law were unwilling to even discuss the notion of integration. The reason given was that their curriculum is fixed according to the prescription on their professional body. In spite of the resistance from the School of Law, the project staff continued and identified areas in the curriculum that HIV and AIDS had to be integrated. Infusion was recommended to take place at the course outline/teaching guides level. Staff members from Management and Law (including the Rep) were disinclined to attend workshops. Despite efforts at persuasion, the views of these Faculty remained unchanged through the lifespan of the HCI project.

Other departments complained that they lacked information about HIV and AIDS and that integration would require time and resources that Academics did not have at their disposal.

The financial management of the HCI project was very demanding. Sometimes it was a nightmare, certain parts of the budget was sub-divided and earmarked for specific activities. This created problems trying to fit different kinds of expenditure to the budget. For instance, if you have 10% of the budget for materials, and you spend 30%, then you struggle with trying to make it fit. You almost spend more time with fitting into the budget than the actual work.

Finally, there was the thorny issue of how to implement HCI in a curriculum that was already packed with content that needed to be taught. The University



decided to infuse, not to develop stand-alone modules. This was very difficult to do because all the curricula were fully packed, according to the SAQA requirements, and all the curricula were so full. It was not possible to find space for something new.

Support of management

The HCI team members acknowledged that one particular factor had assisted the HCI process at the University of Limpopo: the backing and public support of top management:

"You know with Academics it is very difficult to get them together, that's why we had to involve top management. So it's basically usually better to work through management".

A colleague agrees:

"What I can say is that from top management down, from the Deputy Vice Chancellor down, they were supportive, and we had co-operation from the Deans, Heads of Department".



However, it seems that management support was not enough of an enabling factor to drive the project and overcome the negative attitudes or indifference of many staff members.

Lessons learned

Regarding lessons learned from the experience of attempting integration of HIV into curricula at the University of Limpopo, one issue to highlight was that the Academics selected to carry out the various tasks involved in similar projects should be carefully chosen, based on their real interest on the issue:

The faculty members who are constituted into a committee or into a group, should not just be appointed by the Dean or by the head of department. There should be other ways of getting those people who are interested in taking part in the process. Appointing a group of people arbitrarily results in their interest dwindling.

Another lesson to be learned was that HCI involved many tasks:

One key lesson from our experience is that there is the issue of integrating, documenting the integration, and operationalising that documented integration. The lesson learned is that there is a big gap between the two: the project must move beyond ordinary documentation, into looking more at how to operationalise that documented integration. So the obvious lesson, the gap ... if there is any money in future for integration, such money should be directed towards improving the capacity of those who should implement the process.

Conclusion

The process of HIV curriculum integration was confidently initiated at the University of Limpopo. Early processes such as the review of current learning programmes for HIV content were completed fairly easily and rapidly.

Four out of five specific objectives were achieved. The only objective that was not followed through was “to develop an education and training manual for the various categories of staff”. However, an existing



modules used by the School of Education was adopted instead. Fourteen out of eighteen activities planned were achieved. Two activities (curriculum designing and material development) were partially achieved in that an existing module was modified. Also, the workshop for 44 Lecturers aimed at building their capacity to deliver the modules where HIV and AIDS was integrated in their respective departments were not achieved. HIV and AIDS is integrated in all four Faculties in the University. The implementation of HIV curriculum integration was initiated at the University of Limpopo with great enthusiasm. Processes such as the review of current learning programmes for HIV content were completed fairly easily and rapidly. There were frequent attrition of members of the implementation team and little interest on the part of Academic staff members.

That notwithstanding, all undergraduate qualifications have HIV and AIDS infused in the course contents of their calendars. However, much work still need to be done at the level of course outlines and delivery.

There was a degree of interest on the part of some Academic staff resulting in the lack of follow-through in having the calendars of all Schools on the four faculties reflecting the output of the HCI project.

If the University of Limpopo is to provide an effective and comprehensive response to the HIV and AIDS epidemic, the outcome of HCI project needs to move beyond insuision into content areas to the level of course outlines/Lecture materials. Notwithstanding other pockets of HIV and AIDS interventions on campus, outcome of failure to do so is that the University of Limpopo may produce graduates who are poorly informed about one of the key social issues in South Africa today – our continuing vulnerability to HIV.

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HIV and AIDS Programme

Firstly it is important to describe the institutional context of the university in which the project was embedded. Secondly, the initiation of the programme and need for curriculum change and transformation will be presented followed by the new module as a response to this need. The University of the Western Cape project staff espoused the position that all change requires action and the response of the UWC, as presented in this report, is holistic in its approach and transformative in its purpose. Important to the content presented in this report is a reflection on transformation at the following three levels: Module development and implementation, using a pedagogy of transformation and possibility and preparing pre-service teachers (students) as potential future agents of transformation in schools w.r.t. learners, curriculum, colleagues, communities and broader practice.

Introduction – the institutional context

In 1959 South African legislation establishing the University College of the Western Cape as a constituent college of the University of South Africa for people classified as 'Coloured'. In 1970 the institution gained university status and was able to award its own degrees and diplomas. In its mission statement of 1982, the University of the Western Cape (UWC) formally rejected the apartheid ideology on which it was established, adopting a policy of non-racialism. From 1987, under the banner of 'an intellectual home of the left', space was created for curriculum renewal and for innovative research and outreach programmes.

The University now has seven faculties, as well as a number of schools, institutes and research centres, focusing on issues of national importance. In 2015 the Institution had a total student enrolment of 20 382.¹

Initiation of the programme

The UWC HIV curriculum integration (HCI) programme was developed by, and was to be run within the University's HIV and AIDS Programme Office, which fell under the UWC Vice Chancellor. The HIV and AIDS Programme had been initiated in 2001. It aimed to provide an integrated response to the HIV/AIDS epidemic through a comprehensive set of interventions that included teaching, research, care and support, community outreach, advocacy and prevention. One of the primary strategies of the Programme was to focus on gender, which included addressing gender power imbalances and involving men as partners to curb the spread of HIV on campus.

When HEAIDS sent out a call for proposals for HCI work at institutions of higher education, the UWC HIV and AIDS Programme was requested to respond to the call.

The first application for funding submitted by the UWC HIV and AIDS Programme to HEAIDS was unsuccessful, as it did not fit into the HEAIDS funding framework. However, after redrafting the proposal was accepted. In terms of the amended HCI proposal, the HIV curriculum integration programme would be based on education theory, rather than on either of the more conventional theoretical frameworks

¹ Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.



used to address HIV in South Africa, i.e. the public health and behaviour change paradigms. As a unit the UWC HIV and AIDS Programme staff has for some time critically engaged with and challenged these paradigms that appeared to be a disjuncture from the challenges that they faced. They felt more confident to meet these challenges through an approach that is engaging and something more than merely dealing with symptoms but one to effect change. The approach adopted for its programme submission was thus informed by a philosophy of transformation. The programme thus aimed to transform teaching practice in South African schools, through two primary components:

- Refining an existing course within UWC's HIV and AIDS Programme, which was entitled *HIV, sexuality education and good teaching*, and
- Developing a complementary course on the prevention of sexual and gender-based violence (GBV) and develop appropriate associated responses for educational settings. The new GBV-focused course would carry 5 credits, and the intention was to both pilot it and to get it properly accredited within the 2-year period of funding provided by HEAIDS.

The programme's broader purpose was, particularly through the new GBV-focused course, to develop a national and replicable model which would enhance

South African teachers' capacity to respond to sexual violence and sexual diversity as part of a holistic approach to HIV prevention in schools. If successfully developed, the new course was expected to be of particular benefit to students in pre-service teacher education in the Education Faculty at UWC. In addition, the programme (and the GBV-focused course in particular) hoped to target students in other faculties, and also be of value to academic and support staff at the university.

The programme plan envisaged reaching 220 students annually in the Education Faculty (or a total of 440 students over the full programme period) and 250 students annually in other faculties (or 500 over the full programme period). In addition the programme planned to empower 15 UWC staff members. If successfully developed, the new courses focusing on GBV and sexual diversity could, alongside the existing *HIV, sexuality education and good teaching* course, become a national model for holistic engagement with HIV in higher education. This could ultimately feed into a conversation on curriculum transformation and social pedagogy, i.e. how education could contribute to social development.

The module that was developed and implemented was a 5-credit, 50 hour stand-alone module titled 'Sexual Diversity and the Role of Educators' for final-year pre-service education students at the

University of the Western Cape in South Africa. Module development was in collaboration with 147 final-year students in 2014.

One of the Programme's staff members said of this existing course on sexuality education for pre-service teachers:

What we have done is so amazing, it really does work. It transforms the students, they completely change their minds about HIV, and then when they go to the schools, they want to apply this [new approach to sexuality education] in the schools.,

He added:

We develop a stand-alone six week module on sexual diversity for education students at their request.

The hope was that the HCI programme would, therefore, enhance the existing course on sexuality education for trainee teachers through enabling the development of a complementary course with a core focus on gender-based violence.

At a later stage of the programme a decision was taken by the UWC team to add two more components to the educational courses on offer: a course on sexuality education and sexual diversity, and an antiretroviral treatment sensitivity training module.

Challenges

UWC's HCI programme faced a number of challenges early on. One of the first challenges, even in the drafting of the funding application, was that there were a range of opinions within UWC, particularly at senior management level, regarding what HCI meant. However, this issue was fairly easily addressed through the HIV and AIDS Programme engaging in extensive discussions with those who held different perspectives to theirs, and the approach adopted for Programme approach won the day.

The HCI team at UWC pointed out challenges related to the funding process and with reporting obligations. One staff member responded as follows:

Funding limitations of course ... the financial management from the HEAIDS side. Sometimes I waited

for months for money. And I had to pay vendors! So that is the major gripe I have.

This "gripe" is an expression of the level of frustration endured at times in the process of the execution of the programme. It is also clear that the extent of the issues to be addressed by the programme and the available funding was not evenly matched. The extended administration around disbursements proved to be another impediment that requires streamlining. This created tension, mistrust and a negative reputation for the programme and the UWC in its dealings with vendors.

The internal coordination within the HEAIDS office related to the scheduling of training meetings and co-ordination also proved to be disruptive at times. One of the programme staff expressed this as follows:

So scheduling is a major issue ... You are sometimes out of your office for a full week. And there is no communication in the office between different HEAIDS individuals. People overlap sometimes in their functional roles. It has been extremely demanding in terms of that, they all want their work done, they have deadlines to meet. And it has been like that for a long time, they weren't communicating with each other.

It is crucial to point out that the extreme student protests under the banner of Fees Must Fall (#FEMF) impacted severely on programme delivery. HDI's, were subjected to extreme student protests (#FEMF) so much so that campuses were closed for extensive periods in the final semester in 2015 and practically the entire final semester in 2016.

Innovation

It is essential to acknowledge the innovation of the UWC course for trainee teachers entitled *HIV, sexuality education and good teaching*. The academic who had developed and refined this course spoke about how this particular approach has endeavoured to understand the particular historical moment in South Africa within which HIV emerged and spread and how this historical moment might be part of what creates particular personal vulnerabilities.

There is a link between a sense of worthiness and resilience; worthiness and the ability to protect yourself from harm. If someone feels their life doesn't matter, how invested will they be in protecting their life, in not contracting HIV? Colonialism and apartheid intentionally undermined many South Africans' sense of their own worthiness. What does this mean for the HIV epidemic and how people respond to messages about protection and prevention? Are the messaging and information campaigns enough? I don't think so. I have never thought so.

He felt very strongly that teachers should not be trapped in a didactic role and mindset in sexuality education but rather view students and learners as whole and complex beings confronted by daunting socio-economic-political challenges amid their own crises of becoming and identity development.

You need to talk about things like worthiness. You need to talk about historical voices that have been put there. And you need to understand shame.

Furthermore, he stated:

The historical moment requires that teachers are able to create healing spaces. Classrooms should be healing spaces. And teacher education should provide healing opportunities built into them.

The feedback from trainee teachers once they had spent time in full-time teaching and implemented their learning from the course had been overwhelmingly positive. The course enabled some to identify and begin to work through some of their personal discomfort regarding sexual experiences, an obstacle for many Life Orientation teachers who are uncomfortable discussing intimate and taboo subjects such as sexual behaviour and sexual risk. It also gave trainee teachers a crucial point of entry for initiating meaningful discussions with learners in the classroom about sexuality, gender-based violence, HIV, etc. This enabled them to transform classrooms from mere spaces of curriculum delivery to spaces of healing and affirmation and acceptance.

South Africa is sometimes referred to as the 'rape capital of the world', an unfortunate description but unfortunately apt because of the high rates of gender-based violence, sexual violence and rape reported across all parts of the country. To date, a

wide range of different strategies – including legal, social, educational and health-systems interventions have been implemented by different role-players and in different parts of the country. However, very few of these effected real change or any significant impact on the incidence of sexual violence and rape. Given this reality, an unconventional component of the HCI programme at UWC was an innovative and relatively simple stratagem to help inspire new thinking about how to address the problem.

The stratagem was to invite the highly respected founder and director of the University of California's Santa Cruz rape prevention programme for over 30 years, Ms Gillian Greensite, to visit UWC, in order to share her knowledge and expertise in addressing the issue. Key to this programme with students was to build awareness and responsibility and help all people find their voices to speak up for themselves and each other. And to help men recognise what the abuse of their power means for the lifetime of their targets. Just as we needed to find a way to make HIV 'real', sexual violence is just two words until it is made real in a victim's life. The challenge of teaching is to make it real for men so that they would never consider perpetrating such a crime against anyone. Ms Greensite was asked to provide input in two ways:

- Lecture Education students about rape, and provide input on how best to support rape survivors (the title of her lecture being 'Understanding rape – How to help a friend and how to help a learner'). The real value to be derived from her addressing Education students about rape was that they would be able to pass on their knowledge to learners in the classroom, once they qualified. Hence there was a clear multiplier effect in terms of benefits, and
- Provide input to university staff members about how to develop a rape prevention strategy appropriate for the university and taking cognisance of the South African context.

The lecture for Education students was attended by 52 individuals, and was well received. Various students gave feedback on the session. For example, one student said:

I can start working next year as a teacher who will fight and strive towards a rape-free generation.

Therefore, I will teach my children how to look out for each other, rather than hurting each other.

Unfortunately, immediately following an initial meeting between Ms. Greensite and some members of UWC management, student protests associated with the *#FeesMustFall* campaign began. Sadly, this ended the chance for Ms. Greensite to work with a broader range of academic and university staff members on the UWC campus, as had been hoped.

Networking across universities

One of the HCI team members at UWC spoke about the value he derived from networking with individuals from other higher education institutions also involved in HCI through HEAIDS meetings:

In the beginning the networking was good, I used the networking platform to pick up on things from others. Because I think it is not about institutional competition, there is nobody in a competition with anybody. I am not there to say, hey look at me, I am the best, but to say, this is what we have done, maybe you can learn from this?

By the same notion, I would say, 'You have done something interesting, that is wonderful stuff, can we chat over lunchtime?' That is my motto, I train and learn, and I pick up from other people. We all get stronger together. So that for me in the beginning was really good. A couple of people were doing really wonderful stuff, and we could chat about it. That for me was one of the best things that came out of the HCI programme, to really feel, I got a lot of good out of that.

The two critical points made are that it is not about competition but collaboration and learning from each other and that we should affirm rather than negate.

Research publications

Two articles were submitted for publication during the time-frame of the HCI programme. One resulted from a small conference looking at sexual and reproductive health research among adolescents. The second captures the development of the programme's *Sexual Diversity and the Role of Educators* module for final year teaching students. The latter

was accepted for publication by the South African Journal for Higher Education.

Conclusion

From an evaluation of the programme two findings evolved. At the level of **module development** and **curriculum enhancement** the programme has responded on at least two levels. **Firstly**, it addressed a concern raised by UNESCO in its global review of comprehensive sexuality education (CSE) as reported in 2015. This finding argues for quality training of teachers which, as revealed by their report, "... remains limited in scope and, if provided at all, is usually delivered through in-service training" (UNESCO 2015, 8). The introduction of this module has extended the current PGCE and B.Ed. in terms of content, skills and thinking. The programme, **at level two**, has also addressed the concern about pedagogy raised by the same UNESCO report namely that, "They (teachers) also do not consistently use participatory methodologies to engage learners fully in health and life skills education" (UNESCO 2015, 8). At level three, that of preparing teachers for the future, our participatory development of the module, *Sexual diversity and the role of educators*, not only harnessed the experiences of the student teachers themselves but gave them co-ownership, a vested interest and unlocked and acknowledged their agency for transformation on the personal as well as the public levels. They engaged with learners and teachers during teaching practice and introduced new ways of thinking and doing challenging the more established and at times prejudicial and conservative views and practices. These students are our investment for the future as we develop our human resources to meet the challenges of our time head on. Finally, Lees (2017) states that, "At the conclusion of the module, students were asked to evaluate their experience qualitatively and quantitatively. The evaluations showed a self-reported increased in confidence that they will be able to respond to learners questioning their sexualities and that they will be able to intervene when fellow teachers speak inappropriately about homosexuality or LGBTI individuals. ...students experiencing themselves as less judgemental because of the module and more apt to be compassionate toward all difference". (p265)

A key aspect identified at the start of the programme has been to what extent the programme would be

successful in developing a replicable model aimed at increasing the capacity of teachers to respond to the issues related to HIV and AIDS and Diversity? In response to this critical issue it is the view of programme staff that in many of the communities our students come from prejudices and discriminatory practices are dominant and prevail as legitimate and sometimes the only means of engagement with these issues. Students are also engaged in community based organisations whether sports, cultural or church based. It is at these levels of community and fora that we believe they can also have influence. It is fair to conclude that the distinctive feature of the programme that the module addressing innovative teaching skills as opposed to simply imparting 'knowledge', referred to by Freire as the banking model, enhances the students' ability to understand the application of education theory to their classroom practice and beyond. This also responds to the issue of graduate competency as indicated in the broader HEAIDS initiatives.

With its focus on training pre-service educators (education students at university) the UWC programme, through its programme offerings, made valuable contributions to the national response to HIV and AIDS, sexual violence and diversity. On a very direct level the programme provided a direct support to the 2013 Eastern and Southern African Ministerial Commitment on comprehensive sexuality education and sexual and reproductive health services for adolescents and young people. The key feature of the declaration is that, "Schools and other education institutions must deliver good quality sexuality education which is culturally appropriate, gender sensitive and evidence informed".

Responding to the changing nature of the epidemic and new challenges in schools in South Africa, an application to refine the content of the course to include sexuality education and changing the name of the module to *HIV, Sexuality Education and Good Teaching* is one that make logical sense.

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The context

Walter Sisulu University (WSU) is a technology and science based in the Eastern Cape. The University currently awards more than 175 different qualifications. The main campus is in Mthatha, with campuses in Buffalo City (East London), Butterworth and Queenstown. WSU came into existence in July 2005 as a result of the merger between the Border Technikon, the Eastern Cape Technikon and the University of Transkei. The university is named after Walter Sisulu, a prominent figure and African National Congress (ANC) leader in the struggle against apartheid¹.

In 2015 total student enrolment stood at 25 993².

The beginnings of HCI

According to Dr Twaise, the director for the Centre for HIV and AIDS and the project leader co-ordinator of HIV curriculum integration (HCI) at WSU, it was important to understand the historical context of HCI at WSU. The seeds of HCI were sown as far back as 2009, she recalled:

... back in 2009, when the Centre for HIV/AIDS was established at Walter Sisulu University, we organised a huge workshop for the integration of HIV into the curriculum with all faculties at WSU. It was held in Mthatha campus over three days. At that stage there was a lot of debate about whether to have infusion or stand-alone courses, but nothing really moved forward.



Nevertheless, she said, the initial workshop had been useful because it helped identify academic staff who were enthusiastic about HCI:

So we then brought together some members who were passionate and we agreed to develop a facilitator's guide. We started then, with no budget, that was 2012-2013, and we were able to produce our first edition of the facilitator's guide. At the time we had only four faculties involved, but they were passionate about it and were committed.

A further promising development was an agreement with the University of Fort Hare regarding the conceptualization and development of the guide:

We agreed that the guide that we had produced should be distributed to all faculties at both universities. The Centre [for HIV/AIDS] did some training sessions for academic staff on how to use the guide properly, and then we tried to organise two evaluation sessions to see whether the guide was used and if integration was taking place. But the big challenge during that time was funding and the difficulty of penetrating the academic world.

¹ <https://www.wsu.ac.za>

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

Hence, when in 2014 HEAIDS offered funding for HCI at institutions of higher education, WSU was ready to take the HCI process further. The University submitted an application comprising activities that tried to build on what had already been achieved. The main aim of the HEAIDS project was, therefore, defined as:

To increase integration of HIV/AIDS into the curriculum of learning programmes offered at Walter Sisulu University in all eleven faculties, in order to prepare and produce graduates who are competent in HIV/AIDS both at personal and professional levels.

The proposal outlined four sub-objectives:

- To conduct a situational analysis (described as an environmental scan in the funding proposal) on the current state of HIV and AIDS integration in curricula in all faculties across the university
- To review the existing integration guide through a multidisciplinary approach within two faculties, viz. the Faculty of Humanities, Social Sciences & Law, and the Faculty of Science, Engineering and Technology
- To carry out regular monitoring and evaluation on the integration of HIV and AIDS into curricula, and
- To conduct and carry out research and to publish in peer-reviewed journals and present at relevant conferences about the HCI process.

The current HCI project began with a survey to establish what progress had been made at WSU with HCI:

We developed two different questionnaires to find out what had been achieved across all eleven of our faculties. One questionnaire was sent to lecturers and the other one was for deans and heads of department to complete. Getting them back completed took much longer than we expected because of student disruptions. The other problem was the lecturers didn't see this survey as a priority. The student who was given the job of collecting the forms had a hard time – sometimes being told to come back the next week because lecturers were too busy or they were out of office.

Despite the difficulties, the response rate for the questionnaire was 40%, which was an excellent outcome:

Out of 250 questionnaires that were sent out we got 100 back from the lecturers. When we analysed, the

results showed that all the faculties have integrated HIV and AIDS, but it varied a lot. In a few faculties, almost all departments had done integration, but in others very few had done it. Such programmes as Engineering, Mathematics and Accounting had not been able to integrate HIV and AIDS. The results also told us that infusion was the common method of integration that was used.

The HCI team felt vindicated by the survey results, which suggested that previous initiatives at HCI had already borne fruit:

We realised that, aha! People have been doing some work! Because even with Engineering we picked up that with all their students being on the extended programme, they were getting something on HIV/AIDS. I would say it felt good ... Even with the Accounting Department, when they called to say, can you please send us a policy for HIV and AIDS, that told me that these people, they want to start somewhere at least. And then I said, I am satisfied now, because all the departments, all the faculties, are involved in this, whether in one way or another, but HIV and AIDS is a topic to talk about in each and every faculty.

A further success of the HCI project was training 53 lecturers at the University to engage in integration, which meant that the responsibility for implementation of HCI could be spread across the institution, and not reliant on a small task team.

There was an initial plan to hold two separate workshops of two days each to engage academics from all the faculties in developing HCI. However, a decision was made to instead arrange a single writing retreat with representatives from all eleven faculties, and make it a longer duration of four days in all. The retreat was, therefore, arranged for 17–20 May 2016. The extended time period turned out to be a particularly wise decision, as the extra time allocated gave the academic staff who attended sufficient time to get to know each other well, to come to terms with what was expected of them, and to develop some skills in developing materials. The writing retreat was also graced with the attendance of the resource persons – Dr Paulette Powell and Ms Managa Pillay, HEAIDS Senior Programme Manager. This was a very successful retreat that culminated into the production of the second edition of the facilitator's guide.

The brief given to participants was to review the existing facilitator's guide, and to make any changes necessary to bring it up to date and ensure that it had real, practical value for academic staff across the University. The HCI team were surprised at the enthusiastic participation in the workshop process, and how meaningful the process was:

So we had this writing retreat, with all the members of the task team representing faculties. And we really believed that everybody can contribute something to that guide. So we were not going to just stand there and do all the work. We wanted them all to contribute something to this facilitator's guide, as their names would appear there as contributors.

Ms Gogela-Smith, Acting Manager in the Centre for Learning and Teaching Development and HCI project team member continued the story:

So one of the activities was to divide the lecturers into two groups, and we give them a structure of what a lesson plan should look like, just a template, and we said, 'Ok with the outcomes and the assessment criteria, now you have got to come up with part of a lesson plan. You must develop it in terms of how you would teach it.' And I was really impressed with the creativity that came out of that, the level of discussion and the fact that people were not just taking things at face value, but there was a lot of interrogation of facts in terms of how things were presented.

The HCI team were in fact delighted with the output of the workshop:

The fact that we used our staff members' learning – their pre-existing knowledge – and also that we gave them the confidence that they could do this, and for us just to have to edit the document, pull it together.

Another positive development for HCI at Walter Sisulu University was the decision taken by senior management that the Lifestyle Management course (which included HIV as one of its components) should be offered on all extended programmes in the university. This increased considerably the number of students reached by courses that included a meaningful HIV component.

Challenges experienced

A big challenge was trying to persuade academics of the importance of integrating HIV into their teaching. For some faculties, such as Health Sciences and Humanities, this was hardly necessary. However, for others such as the Natural Sciences and Management Science, HCI was a hard sell:

It was a really big challenge because there was nobody in some of the faculties who was interested in driving the integration at faculty or even at department level. If we find more real champions ... maybe it could make a difference going forward.

The project leader agreed:

The struggles we had, sometimes to move at all with integration – struggling to establish collaboration with departments, and not finding easy access. I was also on the Academic Advisory Committee and I would tell the Deans, 'This is what is going to happen! You must release one of your people!' So I said it so often that eventually one dean said, 'No, you must take two of them.' And so I was happy, I got two instead of just one.

A further difficulty was the absence of the HCI project leader who went on study leave for a six month period, which unfortunately coincided with the project's most active phase. Although contingency plans were put in place for another individual to take over her function, the project's momentum did falter somewhat during her absence.

In addition, during the HCI project's lifespan, Walter Sisulu University was undergoing substantial change in terms of structure and management, which made it impossible to achieve all the project sub-objectives.

Passion and leadership as crucial drivers

The members of the HCI team agreed that there was no replacement for real conviction about the added value of HIV integration, and for having an unswerving commitment to seeing it through:

If you want to be involved in this kind of work, you have to feel a true passion for it. Only if you have

that passion, will you be able to survive when you feel as if you are on your own, or there are other more important priorities, and HIV is far down on the priority list for management.

Leadership was also highly valued. Leadership was required: not only leadership from those occupying top management positions at the institution, but also team members themselves were required to develop leadership qualities:

This work teaches you to stand up for what you believe in, not just to give in when

you are struggling and you feel as if you are alone ... and the road looks long. It has been a long, long journey to get to where we are now, nothing came easy.

Students' responses

The project leader said that despite the scepticism of teaching staff, students had always been eager to learn about HIV:

When we still had the Centre for HIV and AIDS, we would get interns from Public Administration, and their work was to produce a newsletter, each and every intern has produced a newsletter there ... they were so excited that they would not want to leave the department before they had produced an HIV newsletter.

And other students, they would choose the topic of HIV and AIDS to do research on, and they would come to our Centre to find information ... it has been very nice to know you are imparting knowledge to students, even though you are not in front of a classroom. And many students have participated in the peer education programme, which they love so much because it makes them understand issues – not only HIV and AIDS, but also about relationships, how to help people.

Positive feedback encouraged even today:

I hear from students who learn about HIV in their studies, how much they find useful in what they get to learn, and how there was so much they didn't learn at school. Being at a tertiary institution also



gives an opportunity to ask questions about HIV they were too scared to ask at school. So many say to me, that they didn't learn at school what they needed to know ... HIV integration has really opened their minds.

Unexpected benefits

Some benefits that accrued from the process of integration were completely unexpected:

When we drafted the first guide, we were trying to address the needs of our institution. We were dealing with the challenges that our own academic staff were facing and trying to help them. And I learned how to integrate HIV/AIDS into the curriculum. When we drafted the second version, I think it is only then that we realised after all, what we have done here applies to other institutions as well, it could extend beyond the boundaries of Walter Sisulu.

Hence, when we were giving the facilitators' guide a name, we decided to call it a *Guide for Universities*, not just for *A Guide for Walter Sisulu University*. Hopefully it has been seen as a resource by other universities, because whenever it was mentioned in different platforms by staff from different universities, they said they found it useful, and they wanted to lay their hands on it, even if it was just to customise it for their own purposes. We didn't have that in mind when we started!

Ms Gogela-Smith shared her story of seeing HIV integration come to be appreciated:

I felt for a long time I didn't have the blessing of my department [in being involved in HIV integration], I was doing something I should not be doing, outside of my scope of work, it was questionable. And it wasn't always easy. But I think that has now changed. So much so that going forward with this project, I think my counterparts on other campuses have been given the task to run with this project when the project leader is no longer here. So, it is not just going to be me. There are going to be three other me's on the other campuses! And it has been made formal, that it has to form part of our curriculum development.

Where to next?

The HCI team members said that HIV integration had taken firm root at Walter Sisulu University, which made the future for integration less uncertain:

Some of those important battles have been fought, and even I think we have established that HIV is an important part of the knowledge base that a well-educated graduate should take away when he or she leaves this university. It isn't just seen as an important topic just for Health Science or Social Science ... it is knowledge for everybody.

Ms Majiki-Mabandla, HCI project participant said that she was keen to see how the second edition of the manual would be used, and whether it would become a valued resource for other tertiary institutions to dip into, or even to adapt for their own purposes:

We didn't develop this manual to stand on a shelf somewhere in a library and just to gather dust! It is a resource, it is a resource that we want to be used, wherever there is a need for information, and a need to integrate HIV. I hope people will look to it, and use it, however they want to, even if it means adapting the contents to meet their own needs in their own context, Ms Gogela-Smith added.

Conclusion

HIV curriculum integration at Walter Sisulu University has a long history, a substantial part of which precedes the current HCI funding cycle. Despite an uncertain and evolving institutional environment, the HCI team has been admirable in its commitment to seeing HCI become a reality throughout the University. Although it was not possible to achieve this across every faculty and every department, the production and printing of a new edition of the HIV integration manual is testimony to the drive, passion and integrity of a group of dedicated individuals at WSU. It is to be hoped that although HCI team membership may change in the immediate future, the course of integration will continue. It appears that the seeds of curriculum transformation have been planted in fertile soil and hopefully they will continue to be fed and nurtured.

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“How can we learn to be resilient, without compromising our humanity, as we engage with the enormous challenges of change in the 21st century?”

– Drama for Life proposal, Wits University

Introduction – the institutional context

The origins of the University of the Witwatersrand (popularly known as Wits) lie in the South African School of Mines, which was established in Kimberley in 1896, but transferred to Johannesburg as the Transvaal Technical Institute in 1904. It became the Transvaal University College in 1906. Other departments were added as Johannesburg grew. In 1922 full university status was granted to the institution, and it was thereafter known as the University of the Witwatersrand¹.

Over the years the University became internationally recognised for academic excellence and extensive research activities. Wits was the first South African university to have a nuclear accelerator, to have a computer, to study building materials, to produce a systematic climatological atlas of Southern Africa and to achieve a successful graft of a plastic cornea. The University's scholars have also greatly advanced the theory of human origin and evolution².

Today, with five faculties (Commerce, Law & Management; Engineering & the Built Environment; Health Sciences; Humanities; Science) and 33 schools, Wits offers approximately 3 600 courses. Over a third of the student body are postgraduate students. Wits has a rich array of sports and cultural facilities, including 42 sports clubs, 60 student societies, a theatre, art galleries, a concert hall and seven museums³. In 2015 total student enrolment stood at 33 777.⁴

Initiation of the HCI programme

The Wits University proposal for HIV curriculum integration (HCI) was developed by Drama for Life, a postgraduate academic, research and community engagement centre at the University that utilises applied drama, drama education and drama therapy within the context of a critical reflexive praxis. The overall goals of the HCI programme were to improve the nature and delivery of the curriculum, to evaluate the



¹ <http://www.wits.ac.za/>

² <http://www.wits.ac.za/>

³ <http://www.wits.ac.za/>

⁴ Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

“Something we feel very strongly about is engaging in dialogue. We are not experts just passing down knowledge. We expect to reflect with our students and our students are expected to reflect back ... And I think linked to that is the idea of participatory pedagogy – that we learn as much from each other as we learn from any book.”

– Drama for Life FGD

efficacy of the pedagogy and its content, to innovate HIV/AIDS interventions through the curriculum, to assess the impact of the overall curriculum upon the learners and to produce accessible teaching materials, applicable across the curriculum, for universities and FET colleges.

Initiated on a longstanding relationship with the Wits Counselling and Career Development Unit, Drama for Life's approach to the HCI programme aimed to engage in HIV/AIDS education and prevention using its distinctive and unique pedagogy. Core to Drama for Life's approach to engaging communities is based on the principle that learning about sexuality, sex, gender and HIV/AIDS needs to involve embodied learning. The centre believes that reflective practice enables the development of consciousness and mindfulness, which it sees as critical attributes in addressing HIV and AIDS effectively. Drama for Life teaches about HIV and AIDS within a human rights and social justice discourse.

Drama for Life's approach is distinctive:

We seek to create a methodology that invites embodied and verbal dialogue about sexual health while encouraging the student to access the information/memory that is already in their body and mind and let it inform the nature of HIV/AIDS and sexual health education today. The curriculum is dependent on this opportunity to sit within oneself and seek a dialogue in yourself (mind, body and heart) and bring that into the broader dialogue.

The Wits project proposal defined its HCI objective as being to strengthen Drama for Life's capacity to streamline HIV/AIDS and related social issues in its curriculum, particularly in two courses which Drama for Life offered⁵. Through such strengthening, the centre hoped to be able to educate professional

artists, therapists, facilitators and educators more effectively in drama techniques for purposes of social transformation.

The aims of the HCI project were:

- To enhance the effectiveness of the reflective practice and critical reflexive praxis postgraduate courses in dealing with Human Rights and Social Justice issues, inclusive of HIV and AIDS, through a collective and interdisciplinary review of the courses
- To increase the knowledge base and methodological skills of 15 Drama for Life academic and project staff to deal with HIV and AIDS in a more integrated and holistic way

To increase the teaching and supervision capacity of Drama for Life Academic Staff in HIV and AIDS within a human rights and social justice context

- To promote research and documentation of 'best practices' in the use of applied drama and drama therapy in HIV and AIDS interventions, particularly with university students and staff
- To educate postgraduate students in the fields of Applied Drama and Theatre, Drama Education and Drama Therapy regarding HIV and AIDS and sexual health education, and
- To develop professional, creative and socially sensitive and responsible facilitators, educators and therapists in HIV and AIDS education through the postgraduate courses offered, including enhanced skills in relation to sexualities, race, gender, disability, and youth at risk.

Training

An important component of the HCI project was meeting a wide range of the training needs of staff

⁵ Reflective Practice in Applied Drama and Theatre / Drama Therapy Iv A (WSOA4061 and WSOA4049) and Critical Reflexive Praxis in Applied Drama, Arts Education and Drama Therapy A and B (WSOA7092 and WSOA7093).

and students. Guest lecturers were invited to talk about specialist topics. These included:

- A workshop providing comprehensive HIV/AIDS education, particularly relevant for younger and less knowledgeable staff members
- A training workshop facilitated by Pierre Brouard of the Centre for Sexualities, AIDS and Gender at the University of Pretoria, focusing on gender, sexuality and violence in relation to HIV/AIDS, and how contemporary research could shed light on these issues
- An art therapy workshop presented by Prof Kim Berman of the Faculty of Art, Design and Architecture at the University of Johannesburg, which employed the production of paper prayers as a form of dialogue about HIV/AIDS, and
- A symposium and training workshop on authentic leadership, run by Dr Mark Rittenberg, which also looked at how different leadership styles relate to student learning.
- An MA Research Workshop @ Wits Club provided an opportunity for our students to engage in research methods with specific reference to creative research in health and wellness. This workshop was facilitated by a panel of Drama for Life staff members including Warren Nebe, Munyaradzi Chatikobo, Dr Petro Janse Van Vuuren and Cherae Halley.

Under the HCI programme, the Reflective and Reflexive Practice academic team attended a writing retreat that aimed to interrogate the Reflective and Reflexive Practice curriculum across the PGDA, Honours and MA courses. Beyond the curriculum review, the team was also tasked to begin the dreaming and designing of an Arts Based Reflective Practice manual – an envisioned product of the HCI programme. The vision for the manual is to possibly integrate reflective and reflexive practice amongst social workers and practitioners. Overall the workshops addressed the gaps within the current curriculum with focus on the developmental levels across the courses, the student's trajectory and the learning outcomes of each course. The workshops also sought to address gaps in standardisation for each course with

explicit indication of core principles to be obtained in practice. The academic team were also exposed to three concepts of curriculum development for their consideration in their review:

- What concepts of their curriculum are socially exclusive?
- How can they make these concepts more accessible?
- How can they achieve the concept of 'constructive alignment'?

In addition to training workshops, the HCI programme afforded Drama for Life the opportunity to provide student scholarships and build its student and staff resource centre and archive in the fields of Arts for Education, Healing and Social Transformation. Through the procurement of relevant and current print and digital publications, and archiving of new and previously undocumented indigenous knowledge, the resource centre intends to provide students, researchers, practitioners and educators working in Arts, Education, Social Development and related fields access to best practice methods and approaches.

Collaboration

Drama for Life collaborated with the Wits University Counselling and Careers Development Unit to host a silent protest march aimed at raising awareness about gender and sexual violence in South Africa, which is a major contributory factor for new HIV infections. Prior to the day of the march, survivors of rape shared their stories at an event hosted by Wits Campus Health, which focused on HIV counselling and testing, and post-exposure prophylaxis (PEP). Also prior to the march, protestors and members of the general Wits community 'claimed back the university space' by wrapping trees around the campus in purple cloth. On the day itself Drama for Life students developed five 'performance installations' to encourage passers-by to consider issues that inhibited speaking about sexual violence, such as homophobia, fundamentalism and violence. Drama for Life staff and MA Drama Therapy students were

**"Almost from the beginning our drama students are trained, to reflect and to feel, not just to perform."
– Drama for Life FGD**

also available to offer counselling and debriefing to protestors and other students after the event. The Drama for Life Playback Theatre, an innovative form of storytelling theatre that addresses various social issues, was used as an interactive vehicle for the reflection of people's stories and experiences of the subject matter and protest. Performed at the end of the day, this innovative practice encouraged audience members to witness and be witnessed. The Drama for Life team said that silence was a key dimension of the protest:

The issue of silence has been questioned and challenged over the years ... it is really important to clarify for those who come across the protest why the Silent Protest is silent, and why most of the protesters have their mouths taped shut. It's a powerful reflection of the reality that so many who are the subject of rape and sexual violence in our society have no voice, they cannot speak for themselves.

Drama for Life provided a platform for Wits staff members and students to come together and engage in 'town hall dialogues' about contemporary social issues at the university and across South Africa and the global world. For example, Justice Edwin Cameron gave a talk on HIV/AIDS in the context of the South African Constitution, which included his own story of living with HIV.

Another campus intervention was the Drama for Life Sex Actually Festival, a cross-community arts education, activism and therapeutic intervention, curated to allow holistic engagement with and interrogation of the complexities around themes of sex, gender, identity, relationships and reproductive health. The title of the 2016 festival, *Revealing Vulnerable Bodies*, located its conceptual frame and programming in the definition of vulnerability as racially politicized in socio-economic contexts and deeply embodied in understandings of what it means to be human, and to be witnessed by others. Bringing critical, bold conversations into the public domain, as part of the DFL Sex Actually Festival and HCI programmes, the SexSports! created and managed by the Reflective Practice class, aimed to engage Wits students in team events that required them to work together, answer questions, and share information on reproductive health, sex, relationships and HIV/AIDS, using

a fun-day approach. This task required Reflective Practice students to apply concepts of development and advocacy in planning and implementing the event, and to reflect and report on their experience within the context of the University.

An important undertaking in terms of the HCI work was an intervention involving resilience building with learners in the Eden Park community regarding prevention of alcohol and substance abuse⁶. Postgraduate students from Drama for Life began by engaging with various groups which played an important role in the community, through face-to-face interviews, focus group discussions, performances and observation, to establish their views and perceptions of alcohol and substance abuse. These groups included teachers from two schools, nurses, police officers, pastors and assistants involved in a community feeding scheme. The information gathered from these groups provided a baseline for the intervention. Following the baseline research the students engaged in a two-day intervention with grade 6 and 7 learners at the local primary school to explore alternative coping methods to alcohol and substance abuse, and to help develop resilience in learners. The intervention ended with a performance arranged



⁶ Alcohol and substance abuse are both risk factors which increase individuals' vulnerability to HIV infection.

by the students in which the learners performed for each other and for their teachers, based on the theme, 'dreaming about a better future'.

The Moutse East Intervention and Festival, which is part of the Universities Reflective Practice In Applied Drama And Theatre / Drama Therapy and Critical Reflexive Praxis in Applied Drama, Arts Education and Drama Therapy aimed to engage the Moutse East Villages, situated on the border of Mpumalanga and Limpopo provinces. Like many rural South African and African villages, there are high incidences of teenage pregnancy, transactional and intergenerational sex, unemployment and substance abuse. Whilst we have previously worked with the youth/school learners in the community, it became crucial for us to include teachers, nurses and counselling staff into the programme in order to create a more wholesome approach. Through this execution, teaching, nursing and counselling staff in the community of Elandsdoorn, Moutse East; were integrated into our training programme.

Other collaborations included the Constitution Hill excursion and examination, Wits Art Museum Engagement, and School Interventions. The Constitution Hill excursion and examination, which took into consideration the Reflective Practice Course; the rights engaged within Drama for Life Festivals as well as service projects in communities, and our current socio-political context at universities. Students were required to work in groups to design and lead a performance art experience at Constitution Hill on one selected human right to raise awareness and deepen understanding about our constitutional democracy. The goal was to create a performance art experience that will enable South African youth (16 to 30 years of age) to reflect, dialogue and negotiate meaning in a culture of respect and dignity. Performance art experiences facilitated an understanding of the selected human right, integrated expressive arts media, and incorporated reflection and dialogue.

MA Wits Arts Museum (WAM) Engagement—Our students were given a lecture – demonstration by artist Gabriella Le Roux focusing on her portrait work of LGBTIQ people from across the globe.

MA School Intervention @ St Stithians College—our students were required to create a diversity workshop for Grade 11 and 12 learners. The intervention had to

probe issues of race, class and access to education in a privileged setting. A core insight that was realised through this intervention was that race and class are complex and divisive issues in South Africa, requiring systematic support for inclusive dialogue, democratic engagement, and increased investment in the futures of local youth.

Benefits

When asked how the HCI project had been useful, one of the Drama for Life staff members said:

I think the grant gave us an opportunity to sharpen the tools we already have.

For another colleague:

It has given us the opportunity we have desperately needed to reflect, and to deepen our capacity and our understanding. And also to share our skills base.

A team member added:

A lot of the work we do has been innovative, and there isn't often much opportunity to reflect back on the work that has been done. And understand what makes a difference and what doesn't.

In particular, the project funding had made possible a retreat which the team had found very helpful:

We went away on a retreat with a long-term partner organisation for four days ... to an artists' centre on a farm between Machadodorp and Dullstroom ... near Kloppenheim, where a lot of work is done in rural and small town contexts ... complicated by HIV/AIDS on all kinds of different levels, and by diversity issues and socio-economic hardship.

A colleague found that:

Going away ... gave us the chance to think. One of the things we want to do is to take our students into rural spaces and small towns across South Africa, because social problems are heightened there. So we were looking at building a partnership. Then there was X's disclosure of his own HIV status, and his own history. As well as our own storytelling that was so important.

“If you’re going to deal with the social-political context, you’re going to deal with psycho-social trauma.”
– Drama for Life FGD

One team member mentioned that a movement / dance facilitator had been invited to join them on the weekend retreat:

And she worked with us around intimacy, and different kinds of movement, improvisation, which can be used outside the dance space, but in an extremely effective way that enables everybody to engage on a very intimate and open level.

A colleague agreed:

Her movement process, which was just very simple touch, leaning into, holding, guiding, that kind of thing. Our group took to it literally like ducks to water. And it was like ducks that had been through a drought!

It was important that Drama for Life team members also had the opportunity to facilitate some of the weekend’s activities. However, what was most valuable, one team member said:

It was having those different perspectives and the time to engage in reflection where we’re not constantly having to deal with the ramifications of an economy that’s in a bad state, a university that’s under pressure, a political system that is putting huge pressure on the university.

A colleague agreed:

So for me at the heart of the retreat ... was the sharing and the engaging of really innovative ideas ... right from the space of cultural leadership crossing over to the therapist to the educator to the theatre-maker, and to the issues that our student body keeps on raising for us, which change a little every year.

Another team member spoke about being struck by how essential reflective practice was to all the teaching done by Drama for Life:

What I gathered from the retreat ... is that for all our other modules to work optimally there’s a need for

us to use reflective practice as a holder or as a pivot on which all the modules are anchored.

The retreat also provided an opportunity to consider how far the Drama for Life approach had come:

I think the retreat was also for me a point of realisation that the course we have nurtured over the years is getting to a point of maturation ... where we saw the possibilities how it is applicable and how it can be implemented in any space ... how the reflective practice can be lifted from the HIV context and placed in a malaria context, or in an unemployment context ... it can be implemented in all sorts of contexts. I realised that we’ve developed something which is almost universal ... and that can be distilled at different levels.

Challenges

One of the challenges of the work in which Drama for Life was engaged was that working in communities that were troubled had consequences for students and staff members:

The work that we do, we’ve just been to Eden Park for the last two days ... and I know that I can feel the after-effects. Yes, it’s hectic. Our postgraduate students go in and work with huge groups of students there, and our students are mentored into learning how to facilitate, but the ramifications are huge because of the issues that confront our students. The moment we enter those spaces we are confronted by a very different reality ...

This created the need for reflective practice to be used for both debriefing and supervision:

We’ve been talking about this for a while now, but we’re in the right place to do it ... is instilling a model of reflective practice within the structure of our system... so some form of supervision for all our staff, but not all together. What we’re looking at is supervision for the different years, because staff in each year need to work as a team, to be conscious

of the dynamics of the milieu, of the group. But there's also something to do with methodologies and processes ... I mean you could talk more to that, but here's the thing, if you're going to deal with the social-political context, you're going to deal with psycho-social trauma...

Another member of the team felt very strongly about this issue:

I keep emphasising this ... that we do need supervision for the staff because we enter into this context. For example, next we're going to go into Moutse East, and F. and her story and whatever it is she's coming into Moutse East with. How has she reflected before entering that context? And being in that context, how is she being affected by it? And how then is she or how am I either consciously or subconsciously taking on certain aspects of that space?

A colleague agreed:

We are so passionate about the work that at times we are blindsided by our passion ... we miss the ability to step back a bit.

Another team member suggested that in engaging with HIV/AIDS and sexuality, the issue of spirituality was too easily ignored:

One of the things that has come up in the last two years is acknowledging one of our blind spots in

relationship to teaching sexual health, which is around not addressing spiritual belief systems.

A colleague concurred:

One of the UNICEF directors spoke about this too, saying that we have excellent information giving across the continent, and yet we weren't addressing essentially the soul of people. Essentially what he was saying was we are not looking at who these people are and their belief systems and practices, and so when we started we thought we were very holistic, but we still didn't address the spiritual beliefs.

Conclusion

The Drama for Life programme at Wits University is extraordinary in its capacity to challenge conventional ways of engaging with gender, sexuality and HIV/AIDS. Teaching staff have the courage to themselves model reflective and reflexive processes which underpin this way of engaging with communities. This is the mark of true participative pedagogy. The accessibility of so much of Drama for Life's work with young people is striking; the willingness of its staff members to share their methodology and what they have learned is also remarkable. There is so much here for other academic institutions to consider and to take on board, particularly in terms of making a difference in the real world. This project deserves to fly.

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Introduction – the institutional context

The educational institution which was later to become the Mangosuthu University of Technology (MUT) was founded in 1979 as a technikon, in response to the need for more technical and technological education in KwaZulu-Natal for students from historically disadvantaged backgrounds. Two years later the fledgling institution moved to its current premises in Umlazi, Durban. In 2007 it changed its name to Mangosuthu University of Technology, in honour of a well-known political figure and the leader of the Inkatha Freedom Party, Chief Mangosuthu Buthelezi. MUT currently has three faculties: Engineering, Natural Sciences and Management Sciences¹. In 2015 the University had a total student enrolment of 11 518.²

Starting the HCI process

MUT applied for a grant for HIV curriculum integration (HCI) from HEAIDS, but the proposal was approved only after it had undergone some amendment, to align with the objectives of HCI.

According to the amended proposal, the main purpose of the Project was:

The design and implementation of a robust and customised HIV/AIDS curriculum as an instrument for reducing the threat of the spread of HIV/AIDS within the MUT community (and the larger KZN community), minimising its current impact through



planning and capacity development, in a way that reflects ethical and social transfer, and generation of knowledge, which is part of the mission of higher education institutions in South Africa.

The proposal envisaged that the following tasks would be carried out:

- Performing an environmental scan that captured existing HIV curriculum integration within MUT
- Selecting and designing an appropriate curriculum approach to be adopted in the pilot project
- Developing academic staff capacity with regard to HCI
- Developing appropriate learning materials
- Piloting the HIV/AIDS-integrated curricula within the selected departments
- Monitoring and evaluating curriculum design and implementation, and

¹ <https://www.mut.ac.za>

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

- Reviewing and documenting the CDI process, and disseminating these results to all stakeholders and to other academic departments within MUT.

Limited progress

At MUT the environmental scan was completed first. It took the form of a questionnaire which was sent out to academics in all three faculties. However, only 18 out of 193 questionnaires were returned, which reflected a disappointingly low response rate of just 9.3%. The returned forms showed that those most interested in HCI were generally female and over 40 years of age. Unfortunately up to half of the returned questionnaires showed a lack of interest in being involved with HCI, meaning that effectively just 9 of the 193 academic staff expressed a willingness to be involved in HCI through the survey. Such apathy emphasised that staff engagement needed to be central to the HCI initiative.

An initial overview of courses at MUT found that some departments incorporated aspects of HIV into their courses, but that this was fragmented and poorly co-ordinated. Furthermore, academic respondents said that there was insufficient support for HCI and that capacity building was needed. Students and new graduates who were surveyed were of the opinion that higher education should have compulsory HIV courses, both within and outside of the formal curriculum, and that such courses should use non-traditional approaches.

Two departments were chosen for the pilot project: these were Environmental Health and Biomedical Sciences. Some progress was made in both these departments with HIV integration initiatives during the funding period.

Another development was the development of a website for MUT in the form of an intranet, providing a range of information about HIV/AIDS, and facilitating curriculum integration. However, the website appeared to poorly received, as reflected in the words of one academic:

Access to information about HIV/AIDS is so readily available, lecturers say, 'Oh we don't need that, we can just get it from the internet'.

It also appeared that MUT would be able to develop a Communications module incorporating HIV, before the funding cycle came to an end. This was likely to be of value in a range of faculties.

Multiple challenges

In December 2015 a curriculum writing retreat was held to encourage academic staff to think about HCI. Although staff members showed an interest in integration, arranging workshops thereafter proved to be impossible owing to competing work demands. Sometimes a workshop date and venue would be scheduled, academic staff would be advised well in advance, but then the event would have to be cancelled because of poor turnout – often only two or three individuals appeared on the scheduled date.

As one staff member saw it:

Everybody wanted to do something, but they didn't have the time. And they just wouldn't commit. Maybe real interest was lacking ...

Then there was a divide amongst the academic staff in terms of seeing HIV as a real and serious issue:

People who wanted to attend the workshops were those who really believed in HIV. But there were also denialists amongst the staff.

Not only was denialism an issue amongst academics at MUT, but so too was HIV stigma. KwaZulu-Natal, the province in which MUT is situated, has the highest HIV prevalence in South Africa. Yet, perhaps unsurprisingly, there is also evidence in the province of high levels of HIV stigma, which are likely to have contributed to the rapid spread of HIV, as well made it more difficult to prevent new and current infections.

A further challenge was that some staff members argued that the funds for HCI should rather be spent on addressing other health priorities:

Some people started saying, 'Why HIV? There are other diseases!'

A further problem was that although the HEAIDS funding was specifically allocated for the capacitation of academic staff:

Lecturers wanted to do the integration, but they wanted the funds to spend on the students. But the grant did not allow for that. It had to be for capacity development of academic staff. However, the staff at MUT believe that they could spend the money on the students. Some of the lecturers had great ideas for going into the community and doing community work, with students doing things like health promotion. Again, one lecturer said, 'I want to go to the community to do ABC prevention with the students'. But the terms of the grant did not permit spending on such initiatives!

A further difficulty was differing perceptions of the right approach to be used for HCI. The person who initially headed the HCI initiative at MUT, and a number of other staff members, leaned towards the biomedical model, whereas HEAIDS wanted a more inclusive framework that included a strong focus on social and behavioural issues.

Since the orientation of MUT leaned heavily in the direction of engineering, marketing and hard sciences, it did not have a faculty of education or a humanities faculty. This made it even more difficult to persuade MUT staff members that the psycho-social aspects of HIV deserved attention.

Another issue was:

... we have a department called the Teaching and Learning Development Centre. The curriculum development should have been done by that Unit!

Even when limited progress was made with HCI:

People have minuted in meetings some HCI that they do, but the problem is that the integration isn't properly documented. I walked down the passage the other day, there is my friend who is a lecturer, she is showing videos on HIV and AIDS. She never said a word about what she was integrating ... Now if she only just wrote a paragraph, it could go into the progress report to HEAIDS!

The HEAIDS grant made provision for employment

of additional staff to free up time for academics who wished to be more involved in HCI. However, as one academic pointed out, for her and a colleague who were both very committed to the process, this was a pipe dream:

The problem is that in both our fields we struggle to find practitioners, especially professionally registered. We couldn't find replacements, we don't have tutors (although some departments do have them), and so trying to manage and juggle the HCI work and our normal lecturing load, it was impossible.

Changing leadership at MUT was mentioned as having been a major challenge. Some individuals in management positions were supportive of HCI, but there were also those who were indifferent to it, or who opposed it.

In addition, reporting processes relating to HCI and reporting to HEAIDS had to be channelled through the office of the Deputy Vice Chancellor (DVC) (Academic). When the administrative staff who worked in this office were aware of the HCI project and supportive, then processes such as submission of quarterly reports went smoothly and unimpeded. However, where staff were uninformed or indifferent, then lengthy and unnecessary bureaucratic delays could ensue, which could have negative consequences in terms of the release of funds.

A further difficulty was the lack of administrative support for those actively engaged in HCI:

The problem is not having admin support, whereas if the project was in the Teaching and Learning Development Centre, where the infrastructure is there It would have been a lot easier. We spent too much time trying to put systems in place.

Learnings

When asked what kept her going with the HCI project, despite numerous frustrations, one lecturer answered:

Everybody wanted to do something, but they didn't have the time. And they just wouldn't commit. Maybe real interest was lacking ...

– Mangosuthu University of Technology

I am 'old school', I think, I diligently go on. Myself, I did some integration, I put it into my study guides. I assigned some tasks, I put in an exam question.

She nevertheless believed that the HCI project had equipped her with a wide range of skills, which would be useful in other academic ventures, and she said she was 'tremendously grateful' for the opportunity that she had been given, and the support from HEAIDS staff.

In terms of attracting academics to HCI events, one strategy was proposed for the future:

We can arrange for outside experts to come and talk to staff, but we can't ensure that enough staff attend. I thought if you make the event earn CPD points for anyone who attends it – that might attract more people. Or provide a certificate, people do want that. Which takes time, but we could have done it.

Another useful tactic for future HIV projects might be focusing more on involving younger staff members, who seem to be more willing to learn about HIV and to talk about it openly to students. The younger generation also appear to associate HIV with less stigma that some members of the older generation do, and they are more comfortable talking about sexual transmission of HIV, an unavoidable topic in providing meaningful HIV education.

Student response

Despite the reluctance of many academics at MUT to become involved in HCI, there seemed to be a real interest on the part of students to learn more about HIV. One comment was:

They are keen! They are reluctant to go to the university clinic to be tested for HIV, because it is open. Stigma is a big issue. But if you talk about HIV as being a social problem, or poverty related, they do talk. In Management Practice, when they had to come up and say something about HIV/AIDS and how it is affecting communities, the students talked. We, the older ones, are the ones that are not talking.

Another anecdote was shared:

I did an activity with the first-year students involving peer educators, and it was quite amazing. But we must remember that students don't want to hear everything from us – the personal stuff that the peer educators talk about – and I could see, because I was sitting in the class, as part of the assessment. And they even got to a point where they wanted to talk about something sensitive. And I told them, I will cover my ears, so you can talk freely.

A further positive experience with students involved role-plays:

When I asked the students to do a role-play about HIV, they would act the whole thing out so well. There are educational programmes involving drama, maybe they should have been brought on board, it might have helped.

Where to next?

Despite the challenges of the HCI project, some academics at DUT thought that the process needed to continue:

I don't think the project should end. I think there should be some form of continuity.

There seemed to be a preference for housing any future HCI initiative within one particular unit:

I would say in the Teaching and Learning Development Centre. That is where we are all doing programme reviews and introducing new degrees. If we had a curriculum person there, just to make sure that HIV was in every part of the curriculum. Even if you seconded a lecturer to that department, to work on some of those activities.

Conclusion

The experience at MUT with regard to HCI is regrettable. Although a small number of academics was enthusiastic about integration, this group had insufficient clout or voice to shift the institution to comprehending the benefits of academic capacitation or of engagement with HCI. Neither were the majority of academics open to learning about participatory pedagogies or to thinking about broader issues of

transformation, as other institutions of higher learning were able to do as a result of their HCI journey. Leadership – particularly the challenge of change in senior leadership – is obviously a necessary factor but not sufficient in itself to make HCI take root.

The purely scientific and technological orientation of TUT also seems to have made it more difficult for staff members to think about the challenges of HIV/AIDS using anything but a biomedical lens. Bureaucratic

challenges, including bottlenecks in communication, also seem to have been stumbling blocks in completing various HCI sub-projects. Undoubtedly the shifting context in tertiary education as a result of the *#FeesMustFall* campaign contributed to the difficulty. Nevertheless, it is to be hoped that this experience may motivate MUT to seize any future opportunities provided by HEAIDS to higher educational institutions to engage critically with HIV/AIDS and to make a meaningful contribution to ending the epidemic.

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Introduction – the institutional context

Nelson Mandela Metropolitan University (NMMU) opened on 1 January 2005, as a result of the merger of the Port Elizabeth Technikon, the University of Port Elizabeth and the Port Elizabeth campus of Vista University. The new institution came into existence because of government's restructuring of higher education nationally, in order to provide more equitable and more efficient education services. The University of Port Elizabeth, the country's first dual-medium residential university (teaching in both Afrikaans and English), had been founded in 1964. However, the Port Elizabeth Technikon had its roots much further back in time – it grew from South Africa's oldest art school, the PE Art School, which was founded in 1882¹.

NMMU provides a broad range of technological and academic education, and employs approximately 2 500 staff members. The University has six campuses: five in the Nelson Mandela Metropole (previously Port Elizabeth) and one in George. In 2015 total student enrolment stood at 26 305, spread over seven faculties.²

Initiation of the HCI programme

The task of co-ordinating the drafting of a proposal for HIV curriculum integration (HCI) at NMMU was

delegated to the University's Centre for Post-School Education and Training (CPSET), based at its Missionvale campus. CPSET collaborates with relevant stakeholders to develop and implement research-informed strategies to enhance various aspects of the post-school education and training sector regionally and nationally. It also provides strategic direction and co-ordination to NMMU in this regard³.

NMMU's HCI project was defined as the integration of HIV&AIDS content into the curricula of both the University and of TVET colleges (with the hope that this would lead to a reduction in HIV- and AIDS-related stigma). This project had three components:

- Building discipline-specific 'communities of practice' of NMMU and TVET lecturers
- Utilising the existing HIV&AIDS module on the University's Moodle web platform, to facilitate integration, teaching and learning, and research, and
- Conducting research to determine students' needs and their views of integration, as well as critically examining the integration process (using Moodle too as far as possible).

This idea of collaborating with the TVET colleges came about because CPSET traditionally engages with the TVET sector. The Project Team was successfully established. It consisted of the following members:

1 <https://www.nmmu.ac.za>

2 Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

3 <https://www.cipset.nmmu.ac.za/>

- a. Three staff members from CIPSET responsible for coordination and establishing contact with the TVET Colleges
- b. Two staff members from NMMU's HIV/AIDS Unit responsible for reviewing and updating the HIV/AIDS content for Moodle
- c. One staff member from NMMU's Centre for Teaching and Learning Media (CTLM) responsible for technological support for Moodle and the training of college lecturers
- d. The HIV/AIDS Education Research Chair in the Faculty of Education, and
- e. A group of TVET college lecturers from Port Elizabeth College working in Tourism; Safety and Society; Renewable Energy and Life Orientation.

In addition to the leadership provided by senior CIPSET staff, three other senior NMMU staff members were drawn in to provide leadership for the Project:

- The HIV/AIDS Education Research Chair in the Faculty of Education
- A staff member in the Centre for Teaching and Learning Media, and
- The director of the HIV/AIDS Unit (who was on leave at that time, and so a colleague took her place for this Project).

The proposal and budget were largely drafted by these individuals.

The reason for CIPSET's promoting the idea of a curriculum innovation hub was so that it could provide a meeting place of lecturers from the colleges and lecturers from the University, to think together about curriculum innovation and curriculum change. The hub was to be open to eight colleges across the province. However, because of NMMU's closer proximity to Port Elizabeth College (PEC) and to Eastgate Midlands College (EMC), in Uitenhage, these two Colleges eventually became the most regular participants.

The project was publicised through the media in May 2015. PEC circulated information about the project through its internal institutional communication system and NMMU published a short notice in *NMMU Talk* in May 2015. The article in *NMMU Talk* was also published in *DHEN*, and another article about

the project was published in the *Fourth Post-School Education Review*.

In addition, an environmental scan of current HIV & AIDS integration in the curriculum was undertaken to understand what was currently being done at NMMU. Although sufficient data was utilised, the necessity for continuous environmental scanning was recognised in order to take account of changing trends related to HIV/AIDS. The data from the environmental scan was used as part of the process of reviewing the previous module on HIV & AIDS.

The learning programme was successfully loaded for use through Moodle. All project participants had access to the online programme. Training on the use of Moodle was provided to PEC staff.

Thereafter workshop training on Communities of Practice (CoPs) was provided to assist participating lecturers to understand the purpose and value of this pedagogical approach. As part of the workshop, a reader was compiled and handed to lecturers as a resource (also available online through Moodle). The initial response to the CoP concept was positive.

Challenges experienced

Although the HCI project at NMMU was initiated with great enthusiasm, it soon ran into major obstacles. These difficulties can be grouped into the following categories: systemic challenges, motivational challenges, financial and practical challenges, and leadership challenges.

Systemic challenges

Soon after the Project was initiated, EMC experienced major disruptions, leading to 66 lecturers going on strike and being suspended. So its staff members started to withdraw their participation. They said, 'We are really interested, but we are troubled.' A new principal for EMC was only appointed in 2016. Due to these internal difficulties, it was agreed that EMC would not participate in the Project.

Port Elizabeth College also faced its own difficulties. The college did not have an individual appointed to the position of principal – for much of the time period,

the post was filled by an acting principal, therefore with limited authority to show leadership.

In the words of an HCI team member:

The culture of the institution has fallen apart, partly because they have never dealt with the transformation issues. In fact they still talk about themselves as campuses, and then it becomes racialised. The principal came along (but in an acting capacity only) and stabilised the institution. Things got restored, but then 6 months later I sat there and really thought long and hard whether I still wanted to be on that Council, because it was incredibly difficult. Just the cultural context of the institution was highly problematic – staff were in conflict, all kinds of issues ranging from race to union differences, so it is not a healthy environment.

The TVET colleges had also undergone a period of rapid change:

Since 1997 student numbers were swelling and staff numbers were cut. So there is a clear need for institutional culture building, which has never happened. And here you come in, thinking that you can work with people, so they agree to come and work, and then you see how they disappear or withdraw, because the institution itself is going through its own turmoil.

So what then happened was that I sat down with one of the principals, and we eventually came to an agreement that you can't really implement a project of this kind in an institution where the institutional culture is not conducive.

Motivational challenges

A second problem related to getting NMMU and College staff members involved in the HCI project, and retaining them:

It has been very, very difficult to get staff from the Faculties [of NMMU] involved. I have made presentations to Faculty boards and so on. I think the issue is firstly, lecturers carry heavy teaching loads, and it is true that there are lecturers teaching 300-450 students. So they have a heavy teaching load. There is pressure on them to do research, and quite a lot of them are also PhD students. So that becomes an immediate barrier.

So participation is still very much driven by personal motivation. It is reliant on voluntary participation, i.e. individuals really thinking this is important work. Some of my colleagues in other Centres based at the University have had the same kind of experience.

At the first meeting ... where we got the buy-in, so to speak, from the Colleges, we had more than 20 confirmations, and 12 people came, and they were going to expand it ... because the idea was to strategically bring lecturers from different campuses together ... if you bring them together, they could eventually form a little community of practice. We would eventually end up with smaller subject or discipline-based communities of practice, which would then contribute towards a larger trans-multi-disciplinary community of practice, where people can then share and showcase their work. Unfortunately this didn't happen.

Matters didn't improve as time passed:

So the numbers over time started to decline, and the campus, it became difficult to secure lecturers' time, so meetings kept on being moved, and then you get pulled into meetings to sit with the management to discuss the issues ... and then you get hit by exam time.

Financial and practical challenges

Another issue was financial support – for many lecturers, the absence of financial incentives or financial assistance deterred them from getting involved:

So we settled things down, got the first group of people in at least to get into the Moodle work, attended the Moodle, and then immediately other issues started to surface: Who is paying for this? How am I getting to the University? Am I going to get a certificate? What is this going to do? We don't have internet access, we don't have Wifi on campus, so the kind of barriers started to surface.

Although communities of practice (CoPs) were seen as a good idea, implementation was far from straightforward:

A great difficulty with the idea of CoPs was implementation. It was expected that lecturers would establish discipline-specific CoPs and wider CoPs within the college. This was not being achieved

because of rigid calendars. The college timetable did not provide sufficient time during official hours for CoP meetings and engagements. Working within a wider CoP was difficult because different disciplines were taught on different campuses. CoP engagements, therefore, could only be scheduled outside official working hours.

There were also shifts in the key focus areas:

The initial focus of curriculum integration at PEC was the disciplines of Mathematics and Science, Arts and Culture and Renewable Energy. Following discussions with PEC staff it was decided that it would be more appropriate to focus on Safety and Society, Tourism and Renewable Energy. The reason for this change was Port Elizabeth College's commitment to strengthening collaboration between staff from its three different campuses. The college wanted to include Life Orientation staff members in the Project, since it was taught to all college students. However, a major challenge was securing the ongoing involvement of PEC staff, given their heavy workloads and given that participation in the project was on a voluntary basis.

Even Moodle presented practical difficulties, with lecturers using the programme infrequently. Reasons for low usage included:

(a) demanding lecturer workloads and rigid college calendar; (b) the programme was regarded as an extra-mural activity which lecturers only accessed after hours; and (c) accessing an online programme required internet access which many lecturers only had access to at work without using their own financial resources. Finally, although Moodle materials could be printed lecturers had to do so at their own expense.

Leadership challenges

It appears that another contributing factor to the failure of the HCI project at NMMU may have been the issue of leadership and failures to communicate effectively. One of the project team members said:

One of the things we didn't do is ... we did not explain to the College well enough how this process [of communities of practice] was going to unfold. It was written down, it was emailed to the Principal, it was

spelled out. But I think I made the assumption that he would take this with his Vice Principal, through the academic board, run through this, make sure they understood it, but we should probably have done that ourselves.

Clearly, institutional leadership in the TVET colleges was problematic for much of the period. However, actual project leadership was provided by four individuals in senior positions within units or departments of NMMU. Did the Project perhaps have too many leaders, so that responsibility for addressing project issues was sometimes unclear? Or were leaders overcommitted because of their other academic and research responsibilities?

Major learnings

The HCI project had nevertheless been an important learning experience for the team:

We have to think far more carefully about the approaches that we use. I am not absolutely convinced that we should have involved the Colleges right from the beginning. The second issue is that you have to have a much deeper understanding of the context, because sitting with the Vice Principal of the College is very nice, and they are office based people, it is easy to agree on things, but you have to understand the dynamics within a context like this.

We also have to look at how we design these kinds of interventions, this was the first big lesson. To try to roll out this kind of work in a two-year period was too ambitious. We should probably have worked at one level, with 10 people, a small number, and simply deal with the issue of HIV integration into the curricula and stop there. So cut back, it was way too ambitious, and make it more manageable.

I also think the Colleges wanted to also show that they were open to this, but we didn't know that were such deep historic issues in the College that are unaddressed.

Conclusion

Although the HCI Project was terminated, the work didn't stop completely. Beyond September 2016 sessions were still being run by the project

leadership with a small group of 4–5 lecturers from the colleges. Sadly, NMMU lecturers never really came on board.

The experience of the HCI project at NMMU raises a number of important questions. Were there early signs that major problems were developing which may have been overlooked or ignored? Decisive interventions early on could possibly have resulted in different outcomes.

Or were the systemic and practical challenges from the outset so severe that the Project had in fact no chance of success? It is clear that the TVET college sector provides particular challenges because of ongoing transformational issues that have not been fully resolved.

Did the concept of communities of practice provide a workable framework, or would another framework have been more appropriate for this project? It appears that some TVET college staff were unconvinced about the value of this approach.

The individuals who led the project may also have been overcommitted because of other academic work. A clearer focus and prioritisation of roles may have been helpful in this regard.

It is nevertheless to be hoped that a useful process of reflection about the project experience at NMMU may enable some useful lessons to be drawn from the process, and that these will inform any engagement with HCI and other aspects of transformation in the future.



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Introduction – the institutional context

Rhodes University is a public research university located in Grahamstown in the Eastern Cape. The University was established in 1904 with an endowment from the Rhodes Trust, established by Cecil John Rhodes, a nineteenth century Cape politician and colonialist. The institution became a constituent college of the new University of South Africa in 1918. In 1951 it opted to become an independent university. Today it has six faculties: Humanities, Commerce, Law, Science, Education and Pharmacy¹. In 2015 the total student enrolment of Rhodes University was 8 007².

Previous history of HCI and raising student awareness

Unlike many other South African higher education institutions (HEIs), Rhodes University had an established history of engaging in HIV curriculum integration (HCI), prior to the HEAIDS call for proposals from HEIs by September 2014. The integration process at Rhodes goes as far back as 2003/2004, when the University first received funding to undertake HIV curriculum integration in various forms across departments and faculties.

A mapping exercise undertaken at Rhodes University in 2009 established clearly the substantial progress which had been made with HCI. The 2009 mapping process looked at 39 departments spread across six faculties: the finding was that 59.1% of all curricula addressed HIV & AIDS in some form.

HEAIDS funding in previous years was also used to finance the development of an educational theatre production by Ubom!, an Eastern Cape drama group based at Rhodes University, which aimed to raise student awareness of HIV & AIDS and associated risks. The production was entitled *Risky Business*. The production was presented annually in the orientation programme for new students at the start of the year. The production succeeded in engaging students, and also improved students' understanding of issues related to HIV & AIDS. *Risky Business* later became subsumed into another production for first-year students entitled *The Amazing Other Show*. The enhanced production dealt extensively with the social phenomenon of 'othering' and thus provided a contextual space to engage with for issues such as HIV stigmatisation. At the end of every performance, audience members were requested to remain in their seats, and were encouraged to ask cast members (who stayed 'in character') any questions which they wanted to ask, relating to what had happened on stage. This form of audience engagement worked exceptionally well, often initiating animated discussion at the end of the show. Hence it appears to have been an effective and accessible medium to engage students regarding significant current social issues.

Initiation of the current HCI programme

Prof Chrissie Boughey, the Dean of Teaching and Learning at Rhodes University, submitted the 2014-2016 funding proposal to HEAIDS. It was approved fairly rapidly, and HCI-related funded activities were

¹ https://en.wikipedia.org/wiki/Rhodes_University

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

scheduled to begin at Rhodes in the fourth quarter of 2014.

The HCI project was managed by the University's Centre for Higher Education Research, Teaching and Learning (CHERTL). CHERTL is engaged in the development of academic staff as professional educators, including the promotion and assurance of quality in teaching and learning and the development of student learning in conjunction with academic departments, the latter through the work of the Extended Studies Unit. In addition CHERTL serves as an academic department of Rhodes University focused on higher education as a field of study, and it assists with the development of teaching and learning in higher education.

The Rhodes University proposal defined the broad purpose of the 2014-2016 HCI grant as being 'to enhance Rhodes University's curricular response to HIV & AIDS'. The proposal outlined the following aims for its HCI project:

- To map the curricular response to HIV & AIDS at Rhodes University in 2015
- To identify the way this response might have shifted, given changes in the South African national context
- To identify appropriate strategies for enhancement of the response
- To evaluate the 2015 response critically
- To provide support for curriculum development, including consultations and workshops
- To insert a new segment into *The Amazing Other Show* related to stigmatisation and HIV & AIDS, and
- To produce a set of learning materials which would form a component of a 'cross-curricular' response to HIV & AIDS.

Mapping the current HCI response

While a previous study of HCI at Rhodes University (in 2009) had found that 51.9% of all courses at the University showed evidence of integration, research conducted as part of the current HCI project and completed in 2016 established that the proportion of courses which had an HCI component had

declined since then³. The most recent research study (Fleischack, 2016) did not specifically try to explore pedagogical approaches, but it did enquire about the 'ways' in which issues relating to HIV & AIDS were integrated into curricula.

The most common response was that HIV & AIDS was 'one of many issues' that concerned academics who engaged in infusion. Although this response was most common, it was closely followed by the use of case-studies, scenarios and stories. Academics said that they preferred to use these methods in order to ensure that students became actively engaged with issues relating to HIV & AIDS, rather than being mere passive recipients of so-called 'basic facts'. One view of the 2016 research findings is that they imply that Rhodes University academics are generally convinced of the importance of utilising a participative pedagogy.

Shifts in the curricular response

In 2015, at the same time as protest action at other higher education campuses across South Africa, Rhodes University also experienced various student protests, known as *#RhodesMustFall*. A university meeting convened to discuss curriculum transformation was disrupted by protesting students who called for 'decolonisation' of the curriculum and for the inclusion of the work and knowledge of African scholars. There was particular sympathy from some academic staff for the call to engage in curriculum transformation, particularly from those who belonged to the Humanities and Education Faculties. However, the students' demands were less favourably received by academics in other Faculties, such as Science and Commerce; this group who found it difficult to understand how decolonisation would be applied to their areas of expertise.

Following the student protests, Rhodes University academics engaged in extensive discussion about curriculum review, which led to a proposal for a campus-wide curriculum review process, the results of which came before the Rhodes Faculties and the University Senate in 2016. The approach which was approved had 'a critical realist ontology' and

3 Although no figure is given for HCI in the 2016 study.

“The real issue may be more to do with how things are taught rather than being about what is taught.”
– Rhodes University

associated methodology as its basis, in accord with the approach taken for an earlier meta-analysis of the entire South African higher education system.⁴

Second round of protests

Later in 2016 a second round of campus protests took place on the Rhodes campus, focusing particularly on campaigning against sexual violence and rape on campus. The protests eventually became known as #RUPreferenceList. In response, the Vice Chancellor appointed a task team to examine the underlying issues, and the team in turn requested a sub-committee to consider how these issues could be dealt with through the curriculum. This particular response, i.e. fixing on curricula as a major part of the solution, attracted some criticism, in that yet again curriculum content was the focus of attention, and pedagogy was not being interrogated for the role it might play in creating or sustaining the primary problems.

Accounting for the decline in HIV & AIDS integration

As mentioned previously, the 2016 HCI research study found that there had been a decline in the number of academic courses which incorporated issues related to HIV & AIDS. This issue relating to HIV & AIDS had been privileged, supported by the HIV & AIDS activism of groups such as the Treatment Action Campaign. However, in the seven subsequent years, such activism had been on the decline (partly owing to key activist demands being met, viz. HIV care and treatment being rolled out by government health services countrywide). In the most recent period protests and social change had led to prioritisation of alternative discourses, particularly ‘decolonisation’ of the curriculum, and addressing sexual violence and rape culture. Hence the new discourses were more dominant in the present and would demand more attention in the curriculum.

Curriculum conversations

Following the 2015 protests, staff at the Centre for Higher Education Research, Teaching and Learning (CHERTYL) initiated a regular series of discussions between Rhodes academic staff members to reflect on the new issue of ‘decolonising’ the curriculum. The events were scheduled after 5 pm on a Monday evening. An initial plan was to organise such conversations fortnightly, but this turned out to be not always feasible. At each event selected academic members of staff would present on aspects of their academic work, and there would then be a discussion as to whether or not this could be seen as decolonisation. The focus was predominantly on curriculum content, although certain individuals chose to focus instead on participatory and interactive pedagogies.

The conversations were well attended, with on average twenty academics turning up for each event, in addition to the presenters. Such consistent and good attendance may lead one to deduce that academic staff probably experienced the curriculum conversations as intellectually stimulating and of value.

Despite the regular scheduling of curriculum conversations, some staff members of CHERTYL have argued that the real issue in the academic environment may have more to do with *how* things are taught rather than being about *what* is taught.

Using educational theatre

Ever since 2004/2005, when it obtained an initial grant for HIV & AIDS work, Rhodes University has utilised educational theatre to engage new students to reflect on important current social issues, such as gender politics, gender-based violence, HIV & AIDS, and various forms of discrimination or social exclusion. The first educational theatre production at Rhodes was entitled *Risky Business*, and it was performed by a campus-based theatre group known as *Ubom!* The production was subsequently revised and adapted almost annually, in response to new

⁴ See Boughey and McKenna, 2015.

pressing social issues at Rhodes. It was always presented in Orientation Week at the start of the new academic year. At the end of the performance members of the audience were encouraged to stay and to ask the actors any questions they wished to. The actors remained 'in character' for the entire question and answer session.

Later, the production was substantially altered, and given a new title: *The Amazing Other Show*. At the start of 2015 a new section addressing aspects of HIV & AIDS was developed through a workshopping process and added to the production, with the costs of developing the additional material covered by the HEAIDS budget. Following the student protests, the show was radically reconceptualised to take account of the latest issues which confronted the University and its students. The result was a more 'edgy' and challenging production, which was also given a new title when it was premiered in early 2016, *Abantu Stand!* The response to *Abantu Stand!* was by and large positive. Student audiences found the show very engaging. It has been described in the following terms:

Abantu Stand! provides an experience which immerses the audience members in scenarios and images which invite them through humour and thrilling theatricality to consider their own situations: their own conditions of privilege, of prejudice, of fear, of anger, of empathy, of frustration, and more importantly, those of others. The show celebrates our shared human capacity for empathy and tolerance, without shying away from bringing light to our often unacknowledged fears and prejudices.

In 2016, for the first time a special performance of *Abantu Stand!* was arranged for an audience of Rhodes University staff members. The show was well attended, and as with student audiences, there was a question and answer session at the end of the performance, with actors answering questions 'in character'. Staff members appreciated the opportunity they were given to attend the performance.

Since the start of the educational theatre project at Rhodes University, more than 1 700 students have watched at least one of the performances, suggesting that the theatrical project has significant reach across the student population.

Development of learning materials

CHERTYL has provided academics with support for the development and provision of appropriate learning materials related to HIV & AIDS. This has taken place in a number of ways. Firstly, academics make extensive use of the online platform *RUconnected* to support learning in courses which they teach. Secondly, a unit within CHERTYL provides specialist assistance in the use of information and communication technology (ICT) for teaching and learning. Members of the unit are able to assist lecturers in developing online materials, and can facilitate workshops on the design of online learning materials.

The third form of support for learning materials took the form of developing a booklet on the infusion of HIV & AIDS into the curriculum at Rhodes, with the costs covered by current HEAIDS funding. Previous published booklets had addressed the principles of curriculum design, assessment and other issues. The new booklet would describe in detail 24 case-studies of curriculum integration which had been identified as good practice.

In addition, CHERTYL has produced a range of materials to support academic staff to become better educators, with materials available online. The management of a local TVET college, East Cape Midlands College, was informed that these materials were also being made available to TVET staff members. Many, but not all educational materials developed by CHERTYL have application in the context of TVET colleges. The materials have been made available to HEAIDS to distribute more broadly within the TVET sector, when and if appropriate.

Innovation

A particularly unusual focus area of CHERTYL is working on the structure of knowledge produced in a particular field, which is known as the 'sociology of knowledge'. Various kinds of knowledge structures are legitimated in disciplinary spaces. This approach enables the development of a critique of participatory pedagogy, which is seen as often making assumptions that are in fact relativist. In reality, some knowledge structures (such as some of the sciences) require pedagogies that construct a coherent, hierarchical knowledge structure. Attempts to develop

participatory pedagogies need to be highly nuanced and theoretically informed, according to CHERTYL. This work is a cornerstone of the Postgraduate Diploma in Higher Education, which has been offered by the Centre since the early 2000s.

Where to next?

Rhodes University – and specifically CHERTYL – is committed to continuing with efforts to promote and enhance HIV integration through various means. The scheduling of an annual HIV awareness week will therefore also continue. Educational drama will be sustained, not only because of its value with regard to informing students about pertinent aspects of HIV & AIDS, but also because it provides a vehicle for meaningful discussion and engagement with a wide range of contentious social issues.

At the time of writing this report a new development in relation to the curriculum had taken place. A further major curriculum review had begun at Rhodes University, which sought to account for why students failed, and to account for the student experience.

Conclusion

The relative geographic isolation of Rhodes University – in a small town with limited economic opportunities away from the major metropolises, and

at a remove from most other higher education institutions – seems to have been a blessing in disguise. While critics might argue that such isolation could encourage parochial attitudes, conventional and unimaginative ways of thinking, and an inherent resistance to change, this has not been the case with Rhodes University and certainly not with regard to its engagement with HIV & AIDS, and HCI. Early on in the South African epidemic, the institution seems to have become convinced about the urgency of developing an effective and co-ordinated response to HIV & AIDS.

Relative isolation may have in fact encouraged Rhodes University academics to develop a strong spirit of independent thinking, and to acquire the courage needed to be innovative and to take calculated risks. Staff members within the University have been able to develop thoughtful and sometimes pioneering approaches which grapple with a range of pressing psychosocial issues. Intellectual rigour and critical engagement are widely evident, in areas as diverse as HIV curriculum integration, psychoeducation for students utilising theatrical performance, conversations about decolonisation, ending rape culture or developing a critique of participative pedagogies. Other higher education institutions in South Africa might find it useful to pay more attention to the processes and thinking at Rhodes about how to address critical issues, not least of all HIV. Much of the innovation that this University engages in is of undeniable value to South Africa and to South Africans.

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“What is AIDS fatigue? It is just word fatigue! People switch off. You need to find engaging ways of talking to them.”

– Sefako Makgatho University FGD

Introduction – the institutional context

The Sefako Makgatho Health Sciences University (SMU) was formally established in 2014 when the health sciences campus was separated from the University of Limpopo by a promulgation in the *Government Gazette* and given its new and current name¹. However, the institution has a much older history. It was founded in 1976 as the Medical University of Southern Africa (Medunsa), with its campus situated 30 km outside Tshwane (Pretoria) and adjoining the George Mukhari Academic Hospital (formerly known as the Ga-Rankuwa Hospital). Medunsa's mission was to 'empower the educationally disadvantaged by providing excellent community-orientated education, training and research in the health and related sciences and by promoting services at all levels of health-care in the community'².

Over the years Medunsa made a significant contribution to the education of health-care professionals and to addressing racial inequities in health-care training. More than half of all black African medical doctors and dentists in Southern Africa were trained at Medunsa. In 2005 Medunsa was merged with the University of the North, which became known as the University of Limpopo. This arrangement was not workable, and it came to an end in 2014³.

SMU currently comprises five schools:

- The School of Medicine
- The School of Health Care Sciences
- The School of Oral Health Sciences
- The School of Pharmacy, and
- The School of Science and Technology.

In 2015 SMU had a total student enrolment of 5 074.⁴

A late start

The HIV curriculum integration (HCI) project at Sefako Makgatho University began much later than at other higher education institutions. The principal reason for the late start was that when the original call for HCI proposals went out, SMU was not yet a fully independent institution, and so it could not apply for funding for HCI in its own right. According to the HCI team at SMU, the process of formal separation from the University of Limpopo at the end of 2014 was a complex and traumatic process, involving loss of some resources, followed by a period of readjustment and renewal. Hence some time passed before SMU was able to decide to embark on its own HCI project, and to develop a project proposal to submit to HEAIDS for funding. Given this history, the project only began in

1 <https://www.smu.ac.za>

2 <https://www.mantramedia.us/sites/studysa/universities/medsa>.

3 Ibid.

4 Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

earnest in the second half of 2016, when most other institutions were completing their HCI work or writing close-out reports on their two-year HCI projects.

Why become involved in HCI?

Despite the late start, SMU brought a great deal of enthusiasm and passion to HCI, as shown by the excellent turn-out in early December 2016 for the first focus group discussion at SMU to discuss HCI. The HCI team members had ideas about what form the HCI project should take. However, during the course of the focus group discussion many of them spoke very openly too about why HCI was important to them – because they had been touched in a wide range of ways by HIV. Some deeply moving personal stories emerged.

For example, one team member said:

I had an aunt, who was diagnosed with HIV in 1989 – I don't think we even had a definition for HIV. But she was quite open, and my family were open, and I think it was a good thing, we were even open at her funeral. So that was my experience. Then I got involved in helping others who were living with HIV ... They called me the AIDS man ... some people would run if they saw me! Others would call me, and say, 'Somebody is dying there in the back room. Can you help?' And I would go and see. People were living and dying in the most horrendous circumstances in those years. And I would try to help.

One academic spoke of how she was working in Virology when she first became aware of the virus:

I started in Virology, it was in the 1980s. And I remember the first HIV-positive patient I came across, in 1986. And then I remember we were worried about babies, we didn't know if the HIV tests showed they were positive or whether it was the mother's antibodies we were picking up. The developing of the PCR, growing HIV in the laboratory. Exciting times! That was how it first started for me.

A staff member recalled her own awakening to the reality of HIV:

For me, HIV is very personal ... So I was diagnosed with HIV 13 years ago. I was raped ... it has been a

long journey with HIV, having the virus when there was no treatment ... I wanted to learn more about the virus itself, to know all about it, also to take care about myself, but in the long run, it opened me up too to other things. How to look after myself and how to give support to people with HIV ... And Manto [Tshabalala-Msimang, previous minister of health,] with her beetroot and garlic. Then I remember being in support groups, trying to stay healthy, then my involvement with the Treatment Action Campaign, fighting for treatment. I knew many comrades who died before treatment came in 2004. It took so long to roll out. I also learned that there are challenges beyond treatment – that we are really facing many life-challenges, living with HIV.

Another team member said that she had also been touched by HIV in a very personal way:

So I was affected by HIV – my sister died of it. So you know when you have that one nurse in the township who everybody comes to? – that was my mother. My sister hid it away, until my mother confronted her about her symptoms. My sister had TB, and when she passed on, the township was quite sensitive about HIV. It was 2004.

So my family wanted to make a message, they made red ribbons. And everybody coming to the funeral got a ribbon. And people asked, 'Why red ribbons?', and we said, 'Because she died of HIV'. Then my mother did HIV counselling, and people came to her house. After that my best friend got diagnosed. She was about to get married, and was worried, about how to tell the parents and the husband. And I didn't have all the answers, and I went to a professional psychologist ... So I got interested in studying the virology of HIV.

Yet another HCI team member spoke about how her life had been indirectly touched by HIV stigma:

My friend had a helper who was HIV positive, who needed to go to the clinic for her meds. She somehow could talk to me but couldn't tell her employer, my friend, for fear of losing her job. So the helper pretended to come to me to work, when she wanted to go to the clinic. Until one day, I got fed up, and I said, 'This must change!'

So I spoke to my friend about HIV. She was well off and educated, and had a rich husband. But still her

“We want to build champions, so they can spread the conversation, like a fire, that spreads, and goes on and on.”

– Sefako Makgatho University FGD

views were skewed about HIV, I had to correct her. And I said, ‘So what if you had someone working for you who was HIV positive?’ and she flipped her lid! But I said to her, ‘It is actually safer, someone who is HIV positive and knows her status, she will protect herself and others.’

It took about two months of discussion before she would change her mind. As time went on, she looked at me, and said, ‘So what is going on?’ And I said to her, ‘Do you love your helper?’ And so she had a conversation with her helper, and the woman disclosed her status. And now she drives her helper to the clinic. It took three months to get to this point. And now it is like a joke between us, because the helper never actually came to clean my house!

Project objectives

The SMU proposal for HCI outlined the following objectives:

- To conduct a survey within the University to ascertain to what extent HIV has been integrated
- To plan and implement workshops to build the capacity of academics of selected departments (Virology, Pharmacy and Public Health) to integrate HIV into their course offerings, and
- To develop a curriculum that integrates aspects of HIV into the curricula of selected courses.

However, the HCI team also had a range of views about additional priorities for SMU in relation to HIV. There was some agreement that in the process of engaging in HCI, HIV stigma was a major issue that needed to be addressed:

Our perceptions about HIV haven’t changed. My family still say the same things: you mustn’t talk too loud about it, it is too personal! Even the myths about HIV are still very strong – about how HIV comes about – that if you sleep with too many people, you manufacture HIV. Internal stigma is really fear of shame.

A colleague concurred:

In my community ... HIV is always ignored or glossed over, like nobody talks about dying from HIV – it is only mentioned in passing.

One staff member wanted to learn more about how to provide support to HIV-positive students who disclosed their status to her:

I want to support students with HIV and their psychosocial issues – empower myself to support patients better.

Another group member had similar views about needing interpersonal skills and knowledge to support students living with HIV:

I have been a mentor to students. I want to know what is happening. I am a pharmacist, but I want to know about the social aspects.

One academic was keen to focus on HIV research:

I am starting an HIV and social science research track. Because there are quite a lot of things we can do.

Yet another team member spoke about her fears regarding non-adherence to antiretroviral treatment, and she felt that HCI needed to address that too:

My fear is we will develop a resistant strain of HIV because our clinics are not providing sufficient support about adherence. We need to start working on those aspects.

The issue of managing diversity also came up in the discussion:

I am interested in social diversity – how we accommodate each other, just like linguistic diversity. I was intrigued when I attended the HEAIDS workshop on diversity, and at the end HIV was mentioned. So I am very new ... I am very interested in those issues.

Some progress

The HCI team began by developing a questionnaire for academic staff to complete, to establish to what extent HIV integration had already taken place and in which courses this had occurred, as well to find out to what extent academics perceived a need to engage in integration. The research project had to be presented to, and approved by a University research ethics committee, which effectively delayed conducting the survey until the first semester of 2017.

Of approximately 400 questionnaires sent out to academics at SMU, about 80 were returned, representing a return rate of 20%, which is a fair response, given that pen-and-paper surveys of this nature generally have a low response rate. Some team members felt discouraged by the response rate, and believed that it reflected academic staff members' lack of interest in HCI. At the time of a second focus group discussion at SMU, in April 2017, the results for the returned questionnaires had not been entered into a database for analysis.

In addition to the survey, various training courses and workshops were offered in 2016 and early 2017 to SMU staff members, and academic staff attended training sessions along with academics from other higher education institutions in Pretoria or at OR Tambo International Airport. The training courses covered the following issues:

- HIV/AIDS101 – i.e. basic knowledge of HIV, including transmission, testing, care and treatment, and prevention
- HIV curriculum integration
- Sexuality, gender and HIV
- Participatory pedagogies, and
- Critical diversity literacy.

In terms of the approach that should be taken for integration, most HCI team members said that they preferred an approach that utilized infusion. One member of staff said:

The challenge for the academic is that you are adding an extra topic to an already full curriculum. But we are not wanting people to add – we are just wanting them to be mindful – and if there is an opportunity, to address HIV, gender, sexuality. It is not about going about and asking people to design

something brand new. But to get there, certain things need to happen. A dialogue needs to begin. Like the values workshop. It is about challenging people's belief systems.

However, not everyone agreed with this approach. Another team member said:

It's not going to work for everybody, we need to come out with various different strategies to integrate HIV ...and we need to share best practices ... So we can learn from each other.

A further development was arranging so-called academic conversations on the University campus. These were short but regular meetings arranged by the HCI team, often scheduled during an official lunch break, in the course of which input would be given on a particular issue (either by an SMU staff member or by an external speaker who had expert knowledge about the topic under discussion), followed by informal discussion. The objective of the academic conversations was to provide an informal forum in which staff members could be exposed to new ideas in terms of HCI and transformation, and provide a context that encourage frank and meaningful discussion. Topics that were covered in such conversations included various aspects of sexuality, gender, racism and diversity.

Unfortunately SMU made no further progress with formal integration before the project time-frame came to an end.

Where to next?

There were a range of ideas on the way forward for HCI. In addition to formal integration of HIV into the curriculum, there was a view that all new staff members should receive training in HIV and integration issues as part of the staff induction process. Some HCI team members were keen to engage in fundraising, in order to finance development of an HIV/AIDS unit on campus. Others were keen on developing a wellness programme for students that included HIV and TB. Another issue in relation to HIV that was mentioned was the need to continue HIV testing campaigns on campus for students, and the importance of providing psychosocial support for students living with HIV.

Conclusion

It is regrettable that SMU was only able to begin its HCI initiative at such a late stage in the funding time-frame. Given that SMU's focus is Health Sciences, one would expect that HIV would fit naturally and easily into much of the curriculum, unlike the scenario at many other South African universities. However, while some HIV biomedical issues are likely to have been addressed in course curricula at SMU prior to the integration initiative, it is unlikely that psychosocial aspects of HIV, which are equally important, would have received the same attention.

There is clearly real enthusiasm among team members and some of the academic staff for HIV

integration into the curriculum. Willingness exists to develop teaching materials which are useful and accessible, and indeed, there is also a desire to engage with other complex transformation issues that confront South African higher education, such as diversity and decolonization of the curriculum. However, given that SMU was not able to implement a large part of its HCI plan, the HCI team will need to engage in critical reflection on what has been achieved so far, and what can be learned from the process. Team members will then need to develop a strategic plan of action for taking HCI forward into the next phase. Given the human and intellectual capital that SMU enjoys, there is no doubt that meaningful HIV curriculum integration is within their reach.

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“HIV curriculum integration reminds us that we need to revisit the university’s social responsibility, and [look at] its obligations to society.”

– Tshwane University of Technology (FGD)

The institutional context

Tshwane University of Technology (TUT) is a South African higher education institution that came into being in 2004 through the merging of three academic institutions: Technikon Northern Gauteng, Technikon North-West and Technikon Pretoria. It has six campuses, located in Pretoria, Soshanguve, Ga-Rankuwa, eMalahleni, Mbombela and Polokwane respectively. This gives TUT the distinction of having a geographic footprint in four of South Africa’s nine provinces, viz. Gauteng, Mpumalanga, Limpopo and North-West¹. In 2015 it had 57 246 students, the second largest enrolment of all the South African universities that provide contact tuition².

How HIV curriculum integration began

The HCI co-ordinator shared her perspective regarding how HCI began at TUT:

So the Student Development and Support Unit wanted to develop a proposal for the Deputy Vice Chancellor (DVC) for HIV curriculum integration. They felt it belonged much more with them, because HIV is what students encounter, and it fits with the life-skills model, where obviously it is incorporated. Their intention was to run with the project, but then

I think after engagement with HEAIDS, HEAIDS said it must be run by an academic department, and they literally didn’t ask us [the Engineering and Built Environment Faculty], they just nominated us to do it.

Her colleague continued the story:

So the DVC, Mr Mokola, just said, ‘Engineering, you are going to do it!’ We tried very hard to convince him that our faculty was not the right place to do HIV integration, but ultimately, you know, we just had to agree with him.

However, the reluctance of some of the engineering lecturers to become involved in the project shifted once they learned more about the impact of HIV/AIDS on South African engineering projects:

... at the end of the day, I saw the advantages of doing it within our framework, because when I started doing the reading, and I began recognising the areas where HIV is affecting us, then I realised engineering is the right place, because of construction, engineers, other workers, at high risk, leaving their homes and working away sometimes for long times, there are real risk factors in this field. So I thought yes, it can add value to engineering, so let’s just take the baby and run with it.

¹ <http://www.tut.ac.za>

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

One individual who was present for the focus group discussion said that he was a member of the Curriculum Development and Academic Support Unit. His enthusiasm about engaging with HIV integration nevertheless suggested that rather than staying involved because of a sense of obligation, his sense of personal commitment had made him stay involved in the project.

Challenges

The HCI co-ordinator expressed her disappointment that the HIV/AIDS Unit at TUT had been unwilling to offer assistance with HCI:

I think that as a university we are working very much like silos. We thought the university had an HIV Directorate. We invited them to be part of HCI, but they said, 'We are not responsible for curriculum development!' I was very disappointed. And yet we are helping them to deal with the HIV pandemic on campus, why don't they want to collaborate?

The group declared that in general, getting other staff members at TUT involved in HCI had been extraordinarily challenging for them:

As such, it was very difficult for us to get buy-in from academics ... I think we had about two or three meetings in this very same boardroom, and we would get Cal from the University of Cape Town [HAICU Unit] to come and do a workshop with all the HODs and the Dean about HIV. And only a handful of people would turn up for the workshop. It was so discouraging.

When asked whether the poor response was a reflection of AIDS fatigue amongst TUT academics, one lecturer responded:

It is not that, it is absolute ignorance. Everyone knows that it [HIV] is there, but they don't want to have anything to do with it ... In my department, at the top, especially with the older lecturers, it is like hitting a brick wall.

A colleague agreed with him:

I think with the older generation, it's a question of: 'I didn't sleep around, so what the hell do I care?' The perception is also that their children are bombarded with information about HIV at school, so for the younger generation, what more is there to know about it?

The HCI co-ordinator felt that the psychological and social dimensions of HIV were particularly uncomfortable for some teaching staff to think about:

Some of the hard sciences have a difficult time dealing with HIV, especially the human aspects. They find that disturbs them.

Staff lethargy at TUT seemed to persist:

We had a workshop last week – in the beginning of the workshop there were seven of us from Engineering, by the time we got to lunch, there were two of us left. I don't know why. The workshop content was really interesting, it was really informative, and useful, because the presenter was HIV positive, and yet people left ... Everyone seems to be so busy, they have no time, it is very strange.

The others concurred:

... and it is especially among engineers that there is a feeling, this thing doesn't affect me, what has it got to do with me? It doesn't affect my immediate surroundings, so why must I who teach design, why must I teach about HIV? What they don't realise is that in the real world, everyone is affected by it, everyone.

The absence of consistent senior leadership was raised by an HCI team member as one of the reasons why it was so difficult to attract lecturers into HCI:

We have one person in authority pushing HCI. If we also had all the heads of departments pushing this, it would be easier.

**"There is no point in asking here for staff to get involved – nobody here volunteers."
– Tshwane University of Technology FGD**

One individual argued that personal experiences sometimes helped attitudes to shift:

I think that it is typical, if you think about HIV and even other diseases, the belief is 'It affects everybody else but not me'. Someone you know first has to be affected by it before you become serious about it.

However, one lecturer felt that there had been a slight shift for the better in the attitudes of TUT academics to HCI:

It's not so bad now ... now that people see that teaching about HIV isn't going to cut into their time. If we had to incorporate it into every subject, then we would get resistance, but now, we have already covered it in the two programmes we are going to provide in 2018 – so that is done, and we have got people who can teach it.

The absence of academic buy-in at TUT was not the only challenge which the HCI team had experienced. The group mentioned other disheartening factors as including recent student protests (*#FeesMustFall*) and the cancellation of HIV/AIDS awareness days. A further issue that needed to be addressed related to the issue of lecturers needing to have personal convictions about the value of teaching HCI:

For us as academics to incorporate HIV into a module [course materials], that is the easy part. To make the lecturer, who is going to present that HIV module, effectively, convey it to the students, now that is the challenge. Because if the lecturer doesn't believe in what he is saying, then the message won't get across to the students.

However, the group persisted in seeking solutions:

So what we are trying to do is to identify the lecturers who are going to offer that HCI programme in 2018, and then try and get them into capacity building – we focus on getting them fully involved.

One individual argued for a cautious and more gradual approach:

We must be careful not to shove it down the throats of everybody. There are also many other social issues. There are also so many topics in the curriculum.

Innovative ways to involve lecturers

The group was undeterred by the lack of academic buy-in, and looked for inventive ways to draw in lecturers:

We have a training course called Licence to Teach, which trains people in how to lecture, because many of us in Engineering are from industry, for example. And the training includes something on HIV. But the training is only for new lecturers, it doesn't address older lecturers. All lecturers should attend this, at least for a day.

Another team member had a different suggestion to enhance lecturers' sense of comfort with regard to HIV:

I am thinking, we can develop a one-day short-learning programme, we can offer it with Student Development and Support. Maybe then the lecturers can learn to feel comfortable around HIV.

The team believed that in the long term an infusion approach made the best sense for TUT:

It should be the ultimate aim for every lecturer in every subject to have something to say about HIV. Not just here at TUT, other universities need it too. Especially from the second year onwards. Everybody should be able to talk about it, without being embarrassed.

Positioning HCI appropriately

The HCI team was asked whether it had made sense to them to locate HCI within the Engineering and Built Environment Faculty. They said that they thought it was a logical fit. One lecturer said:

I realised Engineering is the right place for it, because construction, engineers and other workers can be at high risk, leaving their homes and working away from home sometimes for long periods of time ... so there is greater risk because of more casual sexual partners and maybe unsafe sex? So I realised that it can add value to Engineering.

A colleague argued that it was also appropriate for other reasons:

“Since it takes so much energy to get everybody involved in HCI, rather focus on those who have a natural interest in it.”

– Tshwane University of Technology (FGD)

In some ways Engineering and HIV is a natural fit because we have lots of work to perform in workshops, where health and safety issues come up, so there is a risk of HIV transmission through injuries and blood. So you can bring up HIV: What should you do? How do you treat employees safely?

One of the team members said that he believed HIV might have a direct impact on students:

Even in class, up to ten students may disappear from the class in a year, and nobody asks: why did they leave the class? And yet some of those ten might be HIV positive. As a country, for us that is a serious problem. So many engineering students start the first year, and yet some disappear by the end of every year because of HIV. You are losing those people and it is happening everywhere – in the workplace, in the academic environment too. I think awareness of that is so necessary.

The model for integration

The HCI team were asked which method of integration they had chosen for Engineering and Built Environment:

We decided on the add-on approach. We have two programmes starting from 2018 – one is the Higher Certificate, it is a one-year course, and the HIV course is done at the beginning, in Life Skills. Every department has to go through it. In the B Tech degree, in the final year, there is a subject called Engineering Practice, which has to be done by every department – and HIV fitted into that. We already had approval for the course content, but we can make change of up to 15% to it.

The member of the Curriculum Development and Academic Support Unit explained:

The Life Skills part, that is presented by Student Development and Support ... in the sections where they deal with coping skills, etc. we have introduced the concept of HIV. For example, it looks at how do

you deal with trauma in your life, or how will you deal with somebody else's trauma, for example, a close family member who is HIV positive. That is the context of on a first-year level.

An engineering lecturer continued the explanation:

Whereas the other level, in the final year of B Tech – Engineering Practice – is a subject that deals with all the aspects of HIV that can arise in an engineering context. How HIV might affect you in the working environment, safety issues, dealing with an employee who is HIV positive, and other the problems that can arise.

Facilitators of HCI

Given that the HCI project at TUT had faced so many hurdles and challenges, the group was asked what had led them to persevere with their project, rather than throw in the towel. The HCI co-ordinator spoke frankly:

Maybe it is because I personally don't like to fail – for me, it's about once I have made a commitment, I believe in sticking to that commitment ... And also we had taken the money from HEAIDS, so for me, we just had to do the work.

She added that she knew from her own experience that it was more difficult for women to get access to an engineering education and to engineering employment opportunities. It was nevertheless vital for women to have equal access to such male-dominated occupations. Yet, at the same time women in South Africa, particularly young black women, had a greater vulnerability to HIV:

We work very hard to get women into Engineering, but if women are so much more vulnerable to HIV, imagine the impact it could have on the profession?

Other group members mentioned other factors that they felt had facilitated the process of HCI:

Good communication is so important. And running workshops, to get proper information about HIV. That gave us a new and clearer understanding.

His colleague argued that another important factor was deciding not to invest too much time and effort in trying to draw unwilling academics into the project:

It takes too much energy to get everybody involved, rather focus on those who show a natural interest.

When the project showed slow progress and left them feeling discouraged, adopting a long-term view had been helpful:

You know, it will not happen overnight. And not everybody will get the message. We have to continue with this. That is why we went out of my way to get funding, so that we can continue with this. It is a struggle, but the momentum is there. After three years, when we look back, we will see how far we have come. If we had just handed it over to the HIV Unit, it could have died.

Students' responses

The HCI team was questioned regarding whether its members had encountered AIDS fatigue amongst TUT students. One team member responded:

It seems to me that we may have misunderstood it ... thinking that students are bombarded with information. They are told all the essential facts about how HIV is transmitted, but not much more. We assume students have substantial amounts of information. But maybe they don't really have all the information they really need?

The team said they had observed the general student response to HCI in Electrical Engineering:

We also piloted two days of HIV input in Electrical Engineering – it was introduced using the material we got from UCT. It was piloted with third-year students, and focused on the issue of the world

of work. Two or three students came afterwards, crying, 'Why didn't we learn about this before?' And some were struggling with the disease or had family members living with HIV. And one of the lecturers came to my office to give feedback, he was very emotional, he said, 'You know, we should do this every semester.'

Unfortunately implementation of the pilot course at third-year level had been interrupted by the student unrest. Nevertheless, the feedback about what had been covered was encouraging:

So the student response was very positive. It seems that it is better to focus on the broader issues, not just prevention, and show how it affects their working lives, and pitch it at the right level.

The HCI co-ordinator said that once the piloting of the Engineering Skills course had also taken place, it would be easier to assess how students felt:

We will get feedback from the pilot course in Engineering Skills. We will get a very good impression of what is useful and what is not. And where the students are at. Then we can go away and change the course, to strengthen it, if necessary.

In the first semester of 2017, after the focus group discussion had taken place, news was received from TUT that a session on HIV had been presented during the orientation of first-year students. The response from staff members and students had been mixed, with feedback that some individual were offended by the 'graphic' nature of the presentation and the 'sexual' elements that were included.

The response, as South Africa moves into its 36th year of the HIV/AIDS, is worrying for two reasons. Reluctance to engage with young people and youth frankly about sexuality and about the sexual transmission of HIV is one of the key drivers of the South African HIV epidemic. The fact that some people were allegedly offended by 'graphic' and sexual' elements of the TUT presentation is a prime example of our national discomfort about frank and open discussion about HIV and sexuality in educational settings.

"It is easier to integrate HIV with students, the problem is with lecturers."

– Tshwane University of Technology FGD

“Some lecturers now see that HCI is not going to take away from their work, but actually extend and enrich it.”

– Tshwane University of Technology (FGD)

Sadly, ignorance can have tragic consequences, increasing the risk of new HIV infections. A response of being offended also seems insensitive given that in terms of the new *South African National Strategic Plan 2017–2022*, sexuality education is a core component of the response required nationwide to create a reduction in numbers of new infections (particularly in adolescents and young women³).

Unforeseen benefits

The HCI team members felt pleased about the benefits they had derived personally from being involved in the integration process:

Well, a big benefit for me was learning about HIV in depth. I thought I knew the basic facts, but this gave me a whole new perspective on HIV.

Another team member said that he welcomed the opportunity that HCI had provided to publish a research paper, which was also of benefit to his career.

A third individual commented on what it had meant for him in terms of developing educational materials:

It also helps us to grow as academics. One benefit is that it has also taught us how to write in terms of outcome, and assessment criteria, the process of refining how you write educational material. That is another type of learning that took place.

He lamented:

I wish every lecturer in my department could realise that they could learn something too, and that [HCI] is actually important to do. They are missing out!

Where to next?

The HCI team at TUT contended that sustainability was an urgent issue that needed to be addressed.

However, team members had different views regarding what needed to be done in order to make this happen. One view related to simply making sure that teaching took place:

For now, the most important issue is to ensure that what we have developed is taught. Sustainability means that three years down the line, it is still being taught.

Another perspective was that one needed to be strategic in order to make HCI sustainable:

To get to sustainability, we need to be creative and also strategise. HCI need not be in each and every subject!

A further consideration was making sure that training of academics continued to be provided:

So we said to the Dean, ‘We want to run this course [HCI in Engineering] even after HEAIDS funding stops, we want to carry on. Otherwise all this work will be for nothing.’ So we have to make sure we can continually train the people who may be offering the module.

The HCI co-ordinator was adamant that input on pedagogy for academic staff was crucial for sustainability:

I still want to organise a workshop later on learning pedagogy – to help lecturers understand how to teach. We don’t usually learn these skills, we learn to teach as we go along. It would be so good for academic morale.

In addition, HCI needed to be developed in other departments and faculties at TUT:

I think that it is very important to get HIV into other parts of the curriculum in other faculties. Once we have lecturers aware of HCI in their area, then it is sustainable.

3 SANAC. 2017. *South Africa's National Strategic Plan for HIV, TB and STIs 2017-2022: Let Our Actions Count*.

The last issue raised was the question of how HCI fitted into TUT's vision of the type of graduate that the institution wished to produce:

Understanding about the impact of HIV and what to do makes more sense if you think about what kind of graduate do you want. We need to revisit the university's social responsibility and its responsibility to students. What is the broader view of the university's responsibility to society? What sort of community members do we want to produce at the end of the day?

Conclusion

Engaging in conversations about what constitutes a multi-skilled and informed TUT graduate might be a

useful entry point in terms of obtaining broader buy-in from academic staff and securing their participation in HCI. If lecturers across the institution saw HCI as contributing significantly to desired graduate strengths, then perhaps HCI is more likely to become an imperative across the entire institution.

The small HCI team is nevertheless to be highly commended for its dogged commitment to the HCI project at TUT, despite what appears to be widespread apathy in many academic quarters regarding the relevance of HIV to the university curriculum. The team members' persistence will be no less than inspirational for academic staff in other institutions facing similar challenges.

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“As an Africanised university, one of our pillars is community engagement, and we should ensure that it is real engagement benefiting the broader community.”

– University of Mpumalanga (FGD)

The institutional context

The University of Mpumalanga, based in Mbombela, is a very new institution of higher education. It was established only in 2014, and is the first state-funded university to be based in the province of Mpumalanga. The University incorporated the infrastructure of three educational facilities at its founding: the Lowveld College of Agriculture, the Hospitality School at KaNyamazane and the Siyabuswa Education Campus¹. In 2015, the most recent year for which figures are available, the institution had a modest student enrolment of 816².



He was asked to provide an overview of HIV/AIDS as an epidemic affecting many lives, including those involved in higher education and in the economy, with regional and global implications.

A late launch for HCI

The HCI project at the University of Mpumalanga started relatively late compared to other institutions, with the institution's HCI proposal being approved and funds being made available to the University only in April 2016.

The HCI co-ordinator said that a formal institutional launch was then arranged, to enable the project to become known by a wider circle of people than just the project team members. A guest speaker was invited to the opening:

The speaker was Leon Roets, a sociologist by profession who manages the HCI programme at the University of South Africa (Unisa), and who has extensive experience of engaging with HIV/AIDS at many levels.. The HCI co-ordinator said that he had been pleased about the turn-out for the launch, since about 50 people, including both students and academics, had attended the event.

After the institutional launch, planning meetings were held where team members considered where best HIV could be integrated into the curriculum at

¹ www.up.ac.za

² Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

the University. Although a number of programmes expressed an initial interest in HCI, many of these fairly soon lost interest. It was decided that given the limited time left in the funding cycle for implementation, the focus should then shifted towards:

identifying areas that would render themselves easy for incorporation. Information and Communication Technology (ICT) as a school, also Development Studies and the BSc Programme in Agricultural Extension were chosen ... eventually two programmes were left, namely ICT and the BSc in Agricultural Extension.

Senior HEAIDS staff members were present at the initial meeting and facilitated a workshop to provide more information about the different models of curriculum integration. The Mpumalanga HCI team decided that by and large, the infusion model was most appropriate for their setting.

Challenges

An HCI team member remarked that the first challenge she experienced was when her students could not understand why HIV/AIDS should be included in an information and communication technology (ICT) course, and why it was necessary. She related that the students' comments were:

'Why are we doing this? This is for medical students, not for us!'

She then deviated from her lesson plan and decided to employ a more adventurous strategy to engage student interest. So she requested them to look critically at current myths about HIV:

I did not give them time to go [and research the topic] – I just said, 'This is a class topic, you must discuss this in small groups. And then after a few minutes, each group will report back to the class.' Then I was so excited, I did not expect the response that I got. So I decided to expand the project to a real research focus ... they can engage with different sources of information. That's when I started to get buy-in from students.

Instead of adopting one of the most obvious strategies to address myths – such as introducing common

myths and then providing evidence to discredit them, or bringing in a guest educationalist who had expert knowledge of HIV:

I brought in a guest lecturer who was knowledgeable about ICT – how it could be used in searches. The students did not have enough time [to do searches], they wanted to do more.

The HCI co-ordinator said that for him a challenge had been:

getting buy-in from academic staff – because they say about HIV/AIDS that a lot has already been said, and they consider it as something peripheral – it is not part of their core responsibility, not teaching in their line of specialisation.

He felt that many academics lacked a serious commitment to HIV/AIDS, as reflected by the dwindling numbers of University staff members attending HCI meetings:

In the beginning, we were eight or nine academics, but now we are left with only a few active members, and this speaks to how we fail to prioritise HIV and AIDS.

Passion and conviction

The HCI team members were asked why they had stayed committed to the project, when so many other individuals who had initially seemed keen had fallen away. A female lecturer responded:

What made us stay was, number 1, that we understood the importance of HIV/AIDS awareness for communities, and number 2, our students are coming from families that do not understand ... HIV and AIDS.

One of her concerns was that HCI work should ultimately impact on communities. She remarked that people in her community who were HIV positive were told:

there is an *inyanga*, who is a prophet that heals HIV and AIDS ... So by the time they drop their [HIV] medication, then their health deteriorates. We want to bring that awareness, that yes, go to

the prophet, but carry on taking your medication, and get the correct [laboratory test] results, that say you have been really healed, before you stop your medication.

She added that a third reason for staying involved was that if the HIV prevalence in Mpumalanga rose

because our university feeder schools are from the local disadvantaged communities, then that means we are also going to be affected, as the university. We have to be proactive ... As an Africanised university, one pillar of the university is engagement, and we should ensure it is real engagement that benefits the broader community.

The HCI co-ordinator believed that the HCI team members shared a strong sense of agency about engaging in HCI. He saw plenty of evidence that showed their determination:

They are excited by innovation ... My colleague who just spoke is actually a game-changer. It speaks to the level of professionalism and level of commitment that she shows. She has even been able to influence a colleague to get involved [with HCI] at a later stage of the project.

A male colleague suggested an essential quality that team members should have:

It is really and truly being able to identify an opportunity [for HCI] – when you are asked to engage – you look at your curriculum and see whether there *are* opportunities.

Innovation

A young female lecturer in the Department of Agricultural Extension said that the challenges of small-scale farmers who had felt the severe impact of HIV had provided her with an unforeseen opportunity to develop HCI:

So HIV affects the farmers themselves, and thereby it also affects production, and the economy of the

country. So it was really a perfect fit [for HCI]. The way I approached it was to bring someone from the Department of Agriculture to present to the students, and tell them the situation that HIV/AIDS is presenting in agriculture, and suggest what we can do to assist the small farmers. So for second-year students the HIV aspect doesn't have to be too detailed. It is really much more about conscientising them and notice the impact of HIV on these small farmers.

One of her colleagues said that he was present when the official from the Department of Agriculture spoke to the agricultural students:

Judging from what I saw of the students, they definitely responded in the form of asking questions – follow-up questions – and so on. That shows that they were very much engaged. The presentation generated a lot of interest.

The young lecturer added that the next step for her was to take her students to a farmers' field day, on which occasion an advisor from the Department of Agriculture usually gave a talk to the group of farmers. If the students were given appropriate prior briefing regarding how to engage appropriately with small-scale farmers, the field day would enable students to get to know such farmers in terms of their human dimension, and thereby learn more about the excessive vulnerability of small farmers in South Africa, including their greater risk of contracting HIV infection. In this way her group of students could become sensitised to the distinctive impact of HIV/AIDS on the agricultural sector, she said, and they could begin to think about appropriate and sustainable interventions to assist these vulnerable farmers and their families.

An ICT lecturer spoke about the creative process she had utilised to engage students with regard to HIV:

We went online, and we looked for myths and false information that is communicated about HIV/AIDS to communities. I just took a few, and then I gave them to students and asked them to do research how such false information affects the rate of HIV

**“Good academic practice is thinking outside the box, and HCI really encourages us to do this.”
– University of Mpumalanga (FGD)**

“Our students come from families that believe in false myths about HIV, such as an inyanga being able to cure people of HIV. We want awareness to our students that, yes, they can consult with such a traditional healer, but still take their ARVs to stay healthy and alive.”

– University of Mpumalanga (FGD)

infection in our country, how it affects families, and how it affects individuals. And how ICT can be used to intervene or to bring awareness regarding the false information about HIV and AIDS.

project of the awareness portal. But also looking at [using] billboards at the taxi ranks [for educational messaging], we are wanting to have a slot there if we can get funding.

She continued her explanation:

Then the students had a workshop for a week, where we had an external person from Data Quest, explaining how they can find valid information in valid databases. They did the Data Quest, which was actually very exciting, and then each group presented their findings. And after having interrogated all these myths, they were actually articulating very well on how this affects the economy, how it affects families, how it affects individuals. And that ICT can be used to mitigate this particular challenge.

What intrigued her was that because HIV was introduced in a somewhat incidental manner to the ICT class, students were more easily engaged. In contrast, if HIV had been introduced in a more conventional and direct way, she said, students might have complained that it was a very sombre topic and the issue of AIDS fatigue would have been raised.

A staff member involved in Academic Staff Development said that he thought very highly of the ICT initiative:

There is something I like about their project ... Some students use their ICT skills to look for information on HIV, and then present their information to the others, there they are also learning to use presentation skills. So I found it to be real integration.

The ICT lecturer expressed her satisfaction about being able to contribute to the institution's meaningful engagement with HIV in yet another way:

I belong to a task team that is developing the awareness portal for the university, specifically HIV/AIDS is one of our key components. That is our first

Impact on students

The ICT lecturer said that what touched her the most as her HCI project unfolded was realising that it enabled students to have a real change of awareness:

There was a difference between the start, when the average student could be seen closing their minds, as if they didn't want anything to do with this, it's not my business, and anyway, it is not ICT. And afterwards, when some light has been brought to the subject of HIV/AIDS, and the same person is now engaged and opened up, and they might even say, 'You know, I think that if I had had this HIV awareness before, I could have saved my sister.' You can then see the damage that it has done to families and communities.

The Agricultural Extension lecturer said that she had noticed her students responding positively to the HIV theme in her course:

You could see they were initially a bit tense, but in the feedback I got the real story. They were actually touched by it, they felt lots of emotions. And when they go into the field, they will have more stories to tell.

Another team member remarked:

Most academics link HIV to obvious subjects like Health Sciences, but I think here the students were truly taken by surprise – for instance, opening their eyes to how ICT could be used to investigate, and then to challenge, regarding wrong information circulating about HIV.

There was consensus that most students appreciated well-conceptualised HCI. Lecturers reported overhearing new student conversations about HIV:

I heard some of the students saying, 'You know, this is really a killer, you find out a young child is taking care of a young sister, because the parents have died. And they died because of this kind of myth that is moving around.' So then I can hear for myself that they have a new view [perspective] on HIV/AIDS.

Unanticipated benefits

A female member of the team was persuaded that HCI provided a remarkable vehicle for academics to experience challenge and to grow as educators:

HIV curriculum integration is actually a platform for academics to grow, with regard to engagement, research, and teaching and learning. HIV is also a generic issue that affects everyone in this country in some way – nobody is immune to feeling its effects, directly or indirectly, both in the university and in society at large.

Another team member commented:

For me, there were concepts that I never knew about HIV and AIDS. I had to come up with the outcomes for my module, so I had to go and research about certain topics. And when I was doing my research, I came to realise that what we know is actually very little. I was actually utilising the University of Stellenbosch website, in order to be able to teach the concepts more effectively. I had to understand the concepts myself, and I had to customise the notes, my presentation slides, for my teaching in class. Then that also empowered me, because I didn't know some of the things before.

The HCI co-ordinator reflected on a wide spectrum of benefits he perceived as arising from the HCI project:

Academics definitely benefited themselves. And the students, their involvement in these creative activities, somehow they also derived real benefits. Those were the intended benefits. Not in the usual way of saying, what are the symptoms of HIV? ... But instead, finding creative ways of inclusion in the curriculum, without even speaking of symptoms, signs, what, what. Then from our side also we also gained some skills in project management, project design, project implementation.

He acknowledged the crucial contribution of his colleague in Academic Staff Development:

I couldn't do it alone. So we said, 'You focus on this, you focus on that,' etc., so together we were able to put together this proposal. We also had to learn how to engage with colleagues. That is an empowering and nourishing process, where you learn unintended skills of managing a project of that magnitude, as well as being able to track it and monitor it, and evaluate it, particularly in terms of the outcomes and impact.

The Agricultural Extension lecturer said that her journey with HCI had altered her own understanding, as well as shifting her students' perceptions:

It has prepared me in terms of seeing farmers not only in terms of their agricultural output, but also as vulnerable human beings. Facing the normal challenges that everyone faces. [As for my students] it has opened up their minds, and I think it will benefit them when they go into a rural setting.

She also spoke about how HCI had invigorated the relationship between academics and the Department of Agriculture:

Through partnership everything became possible. We are definitely going to work further with the Department of Agriculture.

An important comment that was made was that involvement in HCI could be useful for academics' own career advancement:

"I have heard a student say, 'You know, if I had had this HIV awareness before, I could have saved my sister from dying from AIDS'."

– University of Mpumalanga (FGD)

Something of note like this cannot go unnoticed. You can actually document it as part of your teaching portfolio, and you can raise it as evidence of your contribution to the body of knowledge, something outside your job description.

Next steps

The ICT lecturer said that for her the next step would be to develop the university's awareness portal (an important medium of communication, although, it must be noted, this falls outside the actual scope of HCI):

Our pilot is HIV and AIDS, but what we want to have is a portal for the range of diseases and health conditions. We want that to be the first project in the university. When anyone logs onto the university website, a pop-up will appear with some enticing information relating to health ... Another idea worth pursuing later for ICT would be TB and the myths surrounding that, given that TB is probably still the number one killer disease in South Africa.

Regarding Agricultural Extension, discussion had started about incorporating HCI into further years of study. However, this would partly depend on the buy-in of other lecturers.

The HCI co-ordinator said he would like to see the project extended

to other disciplines such as Economics, Development Studies and Psychology. Once it is spread across a number of programmes, that will ensure greater impact.

The expansion of HCI to other subject areas is also important for long-term sustainability at the University of Mpumalanga.

Conclusion

Given two significant constraints – that staff at the University of Mpumalanga were still struggling with many of the start-up challenges of the institution, and that the HCI project had started relatively late when compared with most other institutions of higher education which had received funding and embarked on HCI initiatives – it is remarkable how much the HCI team has initiated and done. The camaraderie and mutual encouragement that team members gave each other were striking. Such cohesion is invaluable when a small team has to traverse unfamiliar terrain and develop new approaches to teaching.

Hopefully, as the University of Mpumalanga expands and the range of academic subjects offered becomes more diverse, HCI will become an integral component of good teaching practice. Alongside curriculum innovation it will be equally important to take time out periodically to reflect critically on what has been achieved in HCI, in order to learn, strengthen, consolidate and adapt course offerings to address changing needs.

Another challenge will be to define the appropriate limits of HIV curriculum integration, and what the terms of academic engagement with the HIV/AIDS epidemic should be, given that some biopsychosocial responses to the HIV epidemic are better developed and managed by institutions outside of tertiary education. Recognition of limits is essential if university academics are to avoid taking on too much of the responsibility for ending HIV, with the accompanying risks of burn-out and AIDS fatigue.

Nevertheless, the work done by the small University of Mpumalanga HCI team with regard to HCI is truly commendable and innovative, and will hopefully lay the foundations for future expansion of HCI across the institution.

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Introduction – the institutional context

The University of Pretoria (UP) had its origins in 1908, with the establishment of the Pretoria Centre of the Transvaal University College. The colloquial name of the university, Tuks or Tukkies, is derived from the acronym of the first college – TUC. The college opened as an English-language institution with four professors, three lecturers and 32 students. In 1930, through an Act of Parliament, TUC became the University of Pretoria, and focused on teaching in Afrikaans. Today it offers courses in both English and Afrikaans and has transformed from a mainly white, Afrikaner institution into a multicultural university. UP has nine faculties comprising 137 departments, with academic facilities and student residences on four campuses spread across the city of Tshwane¹. The university's explicit goal is to increase the diversity of the student body, while seeking to recruit students with the capacity to contribute to its goal of being a research-intensive university.

In 2015 the University had a student enrolment of 55 984², the third highest enrolment of all South African universities that provided contact tuition.

Start of the programme

The University of Pretoria's HIV curriculum integration (HCI) project differed significantly from the projects



chosen by other institutions of higher education. Instead of focusing on currently developing HCI, UP decided to review a history of HCI across all its faculties, focusing on processes followed, curriculum evolution, challenges and lessons learned, resources, sustainability and staff support needs, over the period 1999 to 2015³. The HCI project was conducted by the University's Centre for Sexualities, AIDS and Gender⁴ (CSAG) (previously known as the Centre for the Study of AIDS). The Centre had been established in 1999 with the mission of assisting UP to develop an appropriate and multi-faceted response to HIV and AIDS.

The publication that emerged as a product of the HCI project at UP described the unusual approach taken in conducting the research⁵:

In setting out to make sense of more than 15 years of HCI at UP ... we took as our starting point that

1 <https://www.up.ac.za>

2 Department of Higher Education and Training. 2017. *Statistics on Post-school Education and Training in South Africa: 2015*.

3 Centre for Sexualities, AIDS and Gender. 2016. *Integrating HIV and AIDS into the curriculum at the University of Pretoria: Time for transformation?* Pretoria: University of Pretoria.

4 In 2016 the CSA was renamed the Centre for Sexualities, AIDS and Gender (CSAG).

5 Centre for Sexualities, AIDS and Gender. 2016. *Integrating HIV and AIDS into the curriculum at the University of Pretoria: Time for transformation?* Pretoria: University of Pretoria, p20.

we would find different and contradictory notions of what constitutes HCI and its desirability or feasibility, and, accordingly, of how it has been or might be realised. Our approach was, therefore, necessarily inductive, seeking to provide an in-depth account, from which we would be able to draw out both common threads and divergences in the pattern of HCI at UP.

The review of HCI, therefore, used a mixed-methods approach to research, including:

- Examining documentary sources, such as university policies, faculty yearbooks and brochures, and other records
- Conducting in-depth interviews with key senior academic staff
- Further in-depth interviews with academic staff involved in developing and implementing HCI
- Interviewing some students who had experience of HCI, and
- Gathering additional information at HCI workshops involving academic staff, known as ‘collegial conversations’.

The CSAG staff members involved in the HCI project were asked why they chose this particular project, rather than take a conventional approach. One individual responded by saying:

The budget was problematic – it was too small to give something to each of the nine faculties – so it was suggested instead, because of the history of HCI at UP ... to maybe review it and see what lessons we could take from that process, looking at HIV curriculum integration, but also at other social and contemporary issues.



His colleague added:

... given this is an incredibly large institution, it's not one person tasked with HCI, it is often devolved to faculties or departments on a more organic basis, so we thought we should rather document what had been an interesting history, locate our current work in that history, and see what we have learned in the process so far, that could inform best practices.

It seemed that UP was at a different stage of development compared to other universities, and that the process of developing HCI had actually occurred a decade or more earlier at this institution.

The HCI project that the CSAG chose also articulated the notion of intellectual curiosity, helping students (and academics) to develop a more sophisticated understanding of HIV, which is a core value of the CSAG. It is also in accord with the University's current emphasis on research as a primary activity, and its view that curricula should be research-informed and evidence-based.

A CSAG team member explained how the project had evolved:

There was getting together a proposal, there was a bit of a backwards and forwards process ... mostly to tweak it, largely I think HEAIDS acknowledged that it was a slightly different approach ... from what other universities took. There was a process of desk-top review, field enquiry, focus group discussions, individual interviews ... we tried to get each dean, to get all their academics to fill out a questionnaire, there wasn't a strong appetite for that, they weren't seen as meaningful, too time consuming, so we opted for a more organic process, which was to document what we could, from yearbooks, curriculum and so on, prospectuses, study guides.

His colleague continued the story:

The process also evolved in terms of documenting good practices, we had a long history of championships, we also had the Centre's model of working with students in an extra-curricular way ... that was something different ... and there was also a small capacity-building aspect, that involved taking staff through what we called collegial conversations – about their perceptions of HCI and values and

Theme	Department	Nature of integration	Description
1: An oblique approach to HCI	School of Engineering	Core course for first-year Engineering students	Gives students 'an introduction to some of the key debates in the social sciences about the characteristics and challenges of contemporary society'. Includes case-study about the Treatment Action Campaign.
2: Lessons from life	Educational Psychology	Core course for BEd students	Utilises a strength-based approach for creating supportive learning environments. Includes HIV as a common issue which teachers face in the classroom.
3: Sustaining HCI	Social Work and Criminology	Core course for third-year Social Work students	HIV used to illustrate social work in health-care settings.
4: A reflexive approach	Archeology and Anthropology	Core course for Social Anthropology	Covers themes such as religion and ritual, witchcraft and superstition, ideas of beauty, conspiracy theories and globalisation. HIV is included as a context in which a number of these themes are expressed.
5: A multi-dimensional, contextual approach	Public Law	Elective course	Provides an overview of the legal aspects of HIV and AIDS, and describes the social context of HIV.
6: Opening up a broader discussion	Public Law	Core course	Basic principles and practical application of criminal law, including where alleged perpetrators are HIV positive.
7: An opportunity to raise ethical questions	Biochemistry	Core course	Includes immunology, immunity and vaccines. Teaches students to weigh up evidence and introduces ethical issues. HIV and AIDS are used extensively to apply these principles.
8: Engaging with transformation	Centre for Sexualities, AIDS and Gender (CSAG)	Extra-curricular	Uses HIV as an entry point for broader conversations about contemporary social challenges and promotes notions of active citizenship and collective agency.

models ... and whether they had an interest in taking it forward.

The HCI research itself had two focal periods, according to the team:

the one was 1999 to 2011, and then 2012-2016 – the last five years. We found that except for Veterinary Science, all the faculties currently had some form of HCI. One of the unintended consequences of this HCI review process was that Veterinary Science revived some of their previous activities.

The table summarises eight case-studies involving HCI implementation at UP that were selected during the

review process for closer scrutiny and analysis, and which described in particular detail in the published report that emerged as a key output of the HCI project⁶.

Challenges experienced

The CSAG's project members were asked what challenges they had experienced:

Trying to document everything that a very big institution like this is doing around HIV, it was quite time consuming, and also, it isn't just one person who runs HCI. So that meant effectively treating each Faculty as if it was a separate enquiry. There

⁶ Centre for Sexualities, AIDS and Gender. 2016. *Integrating HIV and AIDS into the curriculum at the University of Pretoria: Time for transformation?* Pretoria: University of Pretoria.

“One of our findings was that so much of this work is driven by champions – and if champions leave, a lot of the institutional memory goes with them.”

– University of Pretoria FGD

was support from the Deputy Deans (Teaching and Learning), in each case, but you know, there is significant autonomy in faculties at this university. There were different kinds of welcoming, and ... the idea to do a survey of staff didn't fly in the end. There was just a lot of resistance to that, those were some of the challenges I can think of.

Then it was the difficulty of defining models of HCI, because there is considerable literature on it, but not necessarily real consensus on what those different models are, and what makes them distinct from each other.

His colleague added:

One of the challenges, which came out in the desktop review, was that not all HCI is documented properly. One of our findings was that so much of this work is driven by champions – and if champions leave, a lot of the institutional memory goes with them.

Then the #FeesMustFall student protests also impacted on aspects of the research, particularly making the process of interviewing students with experience of HCI virtually impossible to carry out:

... another challenge was the Fallist movement, which meant, diverting from our ability to access participants to some degree. And also, as the project evolved, we realised, we had always argued for the idea of HIV being the entry point for having discussions around transformation, but in fact the transformation issue became much more urgent, more foregrounded in the findings and recommendations.

When asked what in the research process had turned out differently to what was expected, one CSAG staff member responded:

... not being able to do the faculty surveys was unexpected. There was considerable ambivalence in the senior ranks of faculties about that. The Deputy Vice Chancellor (DVC) writing a letter to support this

wasn't sufficient to make it happen. I think it goes back to the issue of autonomy. There needs to be an enabling environment for HCI, it shouldn't be an instruction from above. Beyond the champions there are different kinds of lecturers – like the cohort of lecturers who have some interest and might be persuaded, and then others who have no interest at all in HCI ... you can't force them.

The questionnaires had seemed to elicit a particularly poor response from lecturers at the University:

Yes, it is difficult for a structured survey to get the full picture ... If you measure everybody with the same rubric, you are going to miss a lot of things. It is a challenge for the higher education sector, since institutions are different, institutional cultures are different. And even for us, how do you measure nine faculties with the same sort of survey or survey instrument?

Different professional requirements for different qualifications were mentioned as a motivation for HCI:

What is interesting to think about is whether for certain disciplines, there is a requirement by certain external professional bodies that their graduates need to emerge with certain attributes and / or knowledges – Engineering is one example. That is also interesting in terms of the issue of autonomy, because the lecturer cohort has to think of balancing that imperative against ... our own [UP] internal graduate attributes, mission statements, values, that we set out as a goal for our institution.

The HCI component in Engineering was a good example of how this could work:

Using the Human and Social Sciences course in Engineering as a best practice, the course is not directly aimed at HIV, but it has an HIV component. It locates the student and their world-view – their understanding of South Africa – in a frame, where they have to think about politics, and the economy, and globalisation, the broad lens which they use for the course. HIV could be a thread that you could

draw through some of those topics, thinking of it more creatively, I suppose.

Enablers of the HCI process

When asked what had helped the HCI research process at UP to move forward, one of the team members responded:

... one of the enablers was our DVC, Norman Duncan, because the original communication to the faculties came through his office. So in the faculties you are more likely to get a response if it is an important person requesting the information. I think the Deputy Deans (Teaching and Learning) were also key to the process, to facilitate for the intra-faculty process. The TUKS AIDS Reference Group was also supportive.

A colleague added:

I would also say the persistence of the team – doggedness. Meetings were helpful, so we were collaborative in terms of trying to support each other. And I think once HEAIDS were on board, they were pretty supportive, in terms of the workshops: funding them, giving us some logistic support.

Another enabler was the extensive set of networks across UP, to which the CSAG had easy access:

A lot of the success of the project was because of the relationships we had built up over the years. If anyone on the project was stuck somewhere, somebody on the team knew somebody who could – sort of – unblock whatever was blocked, or what was being held back.

Possible benefit for students

Students did not stand to gain directly from the HCI research process. However, the team thought that the process probably had indirect benefits for students:

It may be useful for students but indirectly: getting the institution to reflect on what its role is in relation to students and in terms of the broader socio-political frame of South Africa. Do we just produce people who have narrow disciplinary knowledge, or do we prepare them for the world out there? And HIV is a part of that. Also I think it was useful getting the lecturers to think: what is my role in relation to the person in the classroom, not just as an empty vessel to receive knowledge, but who is there as a person with a sexuality and a gender and abilities and challenges? And maybe even living with HIV? Do I ignore all of those things and just teach them, or do I try and think about it differently?

I don't know whether we will bring about those changes at the level of lecturers, but we have started to have those conversations.

His colleague added some additional thoughts:

It is something I saw in the collegial conversations, and I think it comes in the form of some of the case-studies too. HCI was an organic process at UP, there was no top-down directive to do it. But lecturers who were involved in HCI found in many ways that they were in fact better lecturers because they included HIV. When they did teach about these issues, students would approach them with personal questions, HIV disclosure, HIV referral issues and similar things. So indirectly, it made them better lecturers, or at least led to a much better lecturing or teaching environment.

Innovation

An important innovative component of the CSAG's research process was the organising of 'collegial conversations'. This forum for discussion arose out of the CSAG's desire to include a capacity-building component for academics, which HEAIDS had stipulated should be part of the HCI research project:

... which for us was problematic ... because the easiest way to lose academics is to patronise them

“It was useful getting the lecturers to think: what is my role in relation to the person in the classroom, not just as an empty vessel to receive knowledge, but who is there as a person with a sexuality and a gender and abilities and challenges?”

– University of Pretoria FGD

or to tell them how to teach their subjects. That's why the name of the workshops we conducted was collegial conversations, not capacity building workshops – it was a negotiated process because HEAIDS wanted to insist that we should be teaching our lecturers how to teach ... and we tried to say well, research is a developmental process as well, and in the end they accepted that.

Participation in the collegial conversations exceeded expectations, with staff members from every faculty at UP becoming involved in the process:

In the collegial conversations all nine faculties participated. We clustered them into three faculties per collegial conversation. Many of academic staff who participated seemed to find the process to be a useful one, for engaging in discussion across and between faculties. Also for sharing best practice, and for learning from each other.

As the CSAG's publication describes, the collegial conversations brought to the fore some burning questions regarding HCI, such as:

- university policy and requirements versus academic autonomy and voluntarism
- disciplinary and departmental differences regarding the relevance of HIV and AIDS
- constraints on time and space in discipline-specific curricula: whether HCI should focus narrowly on HIV and AIDS and associated drivers, or adopt a broader approach where HIV and AIDS are seen as one of many contemporary challenges
- the possibility of a stand-alone first-year course complementing discipline-specific courses
- staff capacity and comfort in dealing with HIV and AIDS
- the value of HCI champions, and
- the need for capacity building and support for staff providing HCI.

Furthermore, another team member said:

It was also interesting in the collegial conversations to see how new networks were being formed – in the room – I remember for example, someone from Mining Engineering booking a lecturer from Social Work for classes [on teaching understanding of the impact of HIV] for the mining engineering

students – a month down the line – where it wasn't actually part of the curriculum prior to attending the collegial conversations.

Major learnings

When asked whether there had been any surprises in how the HCI research process had turned out, one team member thought that it hadn't been the case:

Not really. The one thing that struck me was that the people that were doing [HCI] were really happy. They enjoyed sharing with us, with someone who was genuinely interested in what they were doing, and it spoke to how isolated academics are generally. The people who were doing HCI were usually working on their own, without much support or recognition from their departments or colleagues. Which is really quite sad! Just the fact that we were people who were interested and prepared to spend time [talking to them], to write it up, show recognition of them, that was important.

She contrasted this experience with her encounters with academics who had not previously thought about doing HCI:

Those who weren't doing anything, what was striking was how easy it was, in the course of the interview, to hook people into thinking how they could integrate HIV into their courses. For example, the guy in Economic Management, a political scientist, who said, 'Our curriculum is so packed, you can't just add anything. There are all these rules and regulations. If you put something in, you have to take something out'. So I mentioned having spoken to the guy in Mining Engineering, and I said, 'For example, wouldn't it be possible in teaching Economics, just to use an HIV example, or to get your students to work out, what are the implications of HIV for South Africa, or for a particular company, of having staff who were HIV infected?' He would pick up that idea and sail off, develop a whole curricula in front of our eyes, and then he would say, 'But ... ' And that scenario happened more than once, where people would take the idea, and just run with it for a bit. But unless there was follow-up, nothing would come of it.

“There is a new generation of [HCI] champions we have managed to identify, and even people on the brink of retirement have got a new interest in it.”

– University of Pretoria FGD

Benefits

The CSAG team was asked whether the HCI research process had brought any unanticipated benefits. They agreed that it had certainly done so:

I think maybe linked to the idea of the champions, because we have lost lots of champions. I think the process revived some of the interest – like in Veterinary Science, Economic and Management Sciences. There is a new generation of champions we have managed to identify, and even people on the brink of retirement have got a new interest in it.

His colleague added further thoughts:

There were a number of people who said that they had found new ways to think about HIV. Beyond HIV. And I think it also enabled us to document our work [at the CSAG] and to cement our relationship with the University. I think there was very good feedback in general on the final report, saying, this is really a significant piece of work. So although we weren't primarily about developing a new HCI kind of thrust, I think that is actually a by-product. Also a general cementing of our reputation, of the quality of work we had brought together.

A public feedback session at UP about the HCI research process, which involved university academics from a range of disciplines, generated a very enthusiastic and lively response. Academics who attended had seemed energised by the interaction:

Also interesting for people from different spheres of the university and different faculties, to be hearing each other talk about HCI – that really is a cross-fertilisation, not often done in fact.

When asked about the long-term benefits of the project, one of the research team members argued:

I do think the fact that we have documented an almost two-decade long process, I think will have a lasting impact. The reports are all available in the

library too – there is a record. It is in some ways quite thought-provoking. We set out from conceptualisation to go beyond HIV, and think the report in many ways pre-empted the Fallist movement and the repercussions, so I think it was published at the right time. There are lessons to be learned.

A colleague had similar perceptions:

I would agree that it enhances our [the CSAG's] reputation – it is a really good record of our history and of our work.... Fees must Fall became a conversation about the nature of the university and how to decolonise it – the curriculum or the culture – it speaks very much to: What are we teaching? How are we teaching it? And whose knowledge matters? And why are we teaching it?

I think it allowed us to locate the discussion about HIV in the bigger discussion about the relevance of the institution. What is its bigger purpose? Who is included or excluded?

A key benefit accruing from the project had been the chance to acknowledge some seminal figures in HCI:

It was also an opportunity to acknowledge and note the contributions of some long-term champions. Two people that stand out for me would be Charlene Carbonatto and Anneliese Nienaber – they have been doing this for eons, and I think for them it was a little bit like soldiering on alone – and this made them feel like part of something bigger. They felt acknowledged.

A colleague added:

Anneliese Nienaber is so passionate about her HIV module, that when they wanted to cull it, she moved departments, rather move to another department so she could continue with her AIDS module. Charlene Carbonatto's teaching of HIV dates back to 1986-1987, long before anyone else was promoting HCI.

“I would say to other people wanting to do HIV curriculum integration: be patient, take a long-term view, build alliances and relationships with people, and give yourself time to reflect.”
– University of Pretoria FGD

Learning from the process

The CSAG research team was asked what learnings from the project they would want to share. One team member said:

Your strengths around HCI are your people ... which is, I think, why our organic approach worked. But you need some kind of support centre or department or function within the institution. We weren't necessarily directly mandated to help academic departments with their curricula, but we got involved. And it is all about in some way following the beat of your own drum, but also getting the intelligent, interested, critical thinkers, and not just 'Please just give us something and we can duplicate it'. And I think that is often where programmes go wrong. If it worked at university X, it should work at university Y, an umbrella approach doesn't always work.

Furthermore:

What I have seen in the process over the years is that curricula are supposed to be responsive. So if we take an overview, there were times when there were strong examples of HCI, and sometimes not, but it also reflected the political and social environment – for example, under Manto [Tshabalala-Msimang, minister of health] and [President] Mbeki a lot of money was coming in for AIDS. It isn't so easy now. Universities have started shutting down AIDS centres and programmes, and just shifting the task to Student Health, and I don't see Student Health being able to fulfil an HCI role.

Where to next?

The team members were asked for their thoughts regarding in which direction the HCI process at UP was leading. The team had various opinions:

Doing critical diversity literacy workshops, that is the next logical step, and it is interesting as well.

Seeing HIV no longer as a stand-alone issue, but as an entry point to talk about other issues confronting us, looking for linkages and synergies between the various issues.

Another view related to the value in sharing the experience with other institutions:

The report is out now and it has also been shared. There is also a whole curriculum transformation workstream – and it is important to share that resource with that workstream. Because it is local, it is more meaningful.

One team member wanted the process to be followed through in another way:

Impact often happens only after the conclusion of a project like this. Our contacts with academics were of necessity very brief and rushed. I wasn't part of the collegial conversations, and there was some discussion there. But I think, at a minimum, our DVC should ask for a representative from each department to get together, and say: 'So what are you doing now about HCI, now that the report has come out?' But it would need the CSAG to go to the DVC and ask him to follow up with the faculties ...

Conclusion

The HCI project which was undertaken by the Centre for Sexualities, AIDS and Gender on behalf of the University of Pretoria identified an extraordinary body of evidence of HCI engagement at the institution which enabled the Centre to, in effect, perform a meta-analysis of HCI at the university over an extensive period of 16 years. The publication that was a key output of this project describes the process and some of the most interesting examples of HIV curriculum integration in greater depth, and is a richly detailed and valuable resource for any group of academics contemplating HCI and wishing to learn how the process has taken place in other institutions⁷. There

are a number of important lessons to learn from the process which may be helpful to other institutions of higher learning. However, as the CSAG's team members caution, there is no one-size-fits-all solution to HCI for all institutions. Their summary document was never intended to be a universal prescription for how to engage with HCI.

Nevertheless, the prescience of the CSAG in terms of nurturing of collegial conversations at UP is

remarkable, since the contents of these discussions in many ways address concerns and themes foregrounded not long after in the nationwide student protests commonly known as *#FeesMustFall*.

Positioning HCI within broader notions of citizenship and civic responsibility, as UP is able to do, reveals foresight and a capacity to see the bigger picture which may be a useful *modus operandi* for other educational institutions too.

7 Centre for Sexualities, AIDS and Gender. 2016. *Integrating HIV and AIDS into the curriculum at the University of Pretoria: Time for transformation?* Pretoria: University of Pretoria.

MAPPING THE WAY FORWARD

The preceding project reports have provided clear evidence of the diversity represented by a wide range of experiences and the unique knowledge generated by 20 funded HCI projects. In some cases projects exceeded all expectations, and team members were able to develop strategies and to plan how to continue the work which had been begun in 2014–2016. In contrast, other projects experienced challenges related generally to the institutional context or to immediate project environment issues. However, even in such instances, HCI team members have been able to learn important lessons which would assist them in planning for more effective engagement in future curriculum integration.

An undoubted and significant benefit that arose from the HCI programme was the gradual and organic development of communities of practice, with such informal networks having the potential to persist well beyond the current funding cycle. The value of these networks is derived from the social and intellectual capital which they retained, and which can usefully inform any future institutional engagement with HIV or a host of associated burning societal issues.

HEAIDS recognises the need for further work to expand and / or continue the processes which were initiated through the individual HCI projects. Expansion or continuation could take a variety of different forms:

- A broader and more diverse range of learning programmes could in future collaborate on an interdisciplinary approach to curriculum integration.
- The focus of curriculum development could be broadened to include how best to develop selected graduate attributes (such as HIV and AIDS competency) and how to foster a sound understanding of citizenship, making use of the curriculum.
- Current engagement with intersectionality and HIV could continue. The concept of intersectionality could be usefully applied more generally to develop more nuanced understand of a wide range of current social problems, such as xenophobia, homophobia, gender-based violence, racism and classism.
- Future work could prioritise in-depth research which has the potential to open up further areas and issues which need addressing.

- Students, who were only indirect beneficiaries of the HCI projects, could be involved in future HCI endeavours and assume the role of direct participants. In addition, they could make a critical contribution to future programming through assuming the role of intended beneficiaries when the new curriculum is fully implemented.
- The induction of new academic staff members (in both the HEI and TVET sectors) could be enhanced to include becoming familiar with, and developing adept skills in using innovative methodologies within their teaching and assessment practices. These could include participatory pedagogies and associated experiential and reflective practices, which are of benefit to both teaching staff and students.
- Further inter-sectoral collaboration and innovative linkages could be considered, and
- Further communities of practice could be contemplated and where practical, nurtured. They could include regional associations and fostering of relationships between departments, faculties or institutions which involved the pooling of accumulated knowledge or being conscious of where mentoring might be useful.

If South Africa is to bring the HIV and AIDS epidemic under control by 2030 through meeting its 90-90-90 targets, i.e. ensure that by 2030, 90% of people in South Africa living with HIV know their HIV status, 90% of those who know their HIV status enjoy unfettered access to treatment, and 90% of individuals who are enrolled in treatment programmes achieve viral suppression to undetectable levels, then building and enhancing a fully effective response by the higher education sector – particularly with regard to conducting research, identifying programmatic interventions and developing more effective strategies in relation to HIV/AIDS – is not merely desirable; it is critical.

This programme has shown that the curriculum is a tool that may be used to address many societal issues. Higher education is about knowledge generation and about the creation of leaders and social change agents and thus the sector's contribution is indispensable if we are to ensure an end to the HIV/AIDS epidemic within our lifetimes.

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